

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

THE CAGE DOESN'T END

When You Turn 65 — It Changes Shape

A Lifecycle Analysis of Disability, Aging,
and Canadian Income Security

*Workers' Compensation · AISH · ADAP · the Cohabitation Rule
OAS · GIS · CPP · the Alberta Provincial Seniors Inventory
Indigenous · 2SLGBTQ+ · Immigrant · Caregiver Intersections*

The Alberta Disability System Breakdown — Advocate

St. Albert, Alberta | May 2026
albertadisabilitybreakdown@outlook.com

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Contents

Movement One — The Reality Check

Section 1.1	If You Get Hurt at Work Tomorrow	4
Section 1.2	The WCB System in Depth	9
Section 1.3	The AISH Application	14
Section 1.4	The AISH-to-ADAP Transition	20
Section 1.5	The Cohabitation Rule	24
Section 1.6	The Bridge to 65	29

Movement Two — The Architecture

Section 2.1	Old Age Security	32
Section 2.2	The Canada Pension Plan	37
Section 2.3	The CPP Disability Benefit	42
Section 2.4	GIS, the Allowance, and the Partnership Penalty	46
Section 2.5	The Alberta Provincial Seniors Inventory	51
Section 2.6	The Lifecycle Reassessment Thread	57
Section 2.7	Partnership Penalty Stage by Stage	62
Section 2.8	What You Put In vs What You Get Back	68
Section 2.9	The Gap Inventory	72

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Movement Three — The Intersection Chapters

Section 3.1	Indigenous Seniors	78
Section 3.2	2SLGBTQ+ Seniors	83
Section 3.3	Immigrant and Racialized Seniors	88
Section 3.4	Senior Caregivers	93

Movement Four — The Witness

Section 4.1	What Has Been Witnessed	99
Section 4.2	The Covenant: We See You	103

Reference

Source Index	Comprehensive citation of every source across the document	107
---------------------	--	-----

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The Cage Doesn't End When You Turn 65 — It Changes Shape

A Lifecycle Analysis of Disability, Aging, and Income Security in Canada

Movement One — The Reality Check

Section 1.1 — If You Get Hurt at Work Tomorrow

This document is written for several audiences. It is written for seniors who have been inside the system for years and have never seen the architecture they are inside of named in writing. It is written for advocates and policy researchers and elected officials who are looking for the documentary record. It is written for the Indigenous, 2SLGBTQ+, immigrant, and racialized communities for whom the system functions worst, and whose own work this document draws from rather than substitutes for.

But it is written first for the working-age reader who does not yet think this document is about them. It is written for the welder, the heavy-duty mechanic, the hospital cleaner, the warehouse picker, the long-haul driver, the meat-plant worker, the kindergarten teacher, the home-care worker, the pipefitter, the line cook, the construction labourer, the oil-patch tradesperson, the tradeshow rigger, the courier, and every other Canadian whose body is the instrument they earn with. It is written for the person who has never been on a disability program, has never thought of themselves as part of the disability community, and who believes — because they have been allowed to believe — that the system in this country will catch them if their body fails.

This is what happens if your body fails.

The cascade

You are forty-two years old. You have been working since you were seventeen. You earn \$95,000 a year. You have a mortgage, two kids, a spouse who works part-time and does most of the home labour, a paid-off vehicle, \$23,000 in an RRSP, \$8,000 in a TFSA, and a basement that you finished yourself last summer. You are not rich. You are doing what you were told to do. You vote. You file your taxes on time. You have paid the Canada Pension Plan, Employment Insurance, and federal and provincial income tax every year of your working life — somewhere in the range of \$700,000 in lifetime taxes, contributions, and premiums by the time the accident happens.

The accident happens on a Tuesday in November. You fall from a height. Or a load shifts. Or a vehicle hits yours on the way to a job site. Or your back, which has been telling you for years that something is wrong, finally tells you in a way that ends the work day. Or you are diagnosed with a disease that two years earlier had no symptoms. The mechanism does not matter. What matters is what comes next.

Weeks one through twelve: WCB

If the injury happened at work, the Workers' Compensation Board of Alberta opens a claim. WCB pays wage replacement at 90% of net earnings, capped at the 2026 Maximum Compensable Earnings of \$110,900 per year. (*Source: WCB-Alberta — Wage replacement; WCB-Alberta — Maximum Compensable and Assessable Earnings Fact Sheet, January 2026.*) If you earned more than that — and many trades workers, oil-patch workers, and skilled-trades workers do — your wage replacement is calculated against

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

the cap, not against what you actually lost. Between September 2018 and December 2020 there was no cap on wage loss benefits in Alberta. The cap was restored on January 1, 2021. The same provincial government that is now restructuring AISH into ADAP is the government that put the wage-replacement cap back in place after a brief period without one.

So if you earned \$125,000 a year, your WCB benefit is calculated as if you earned \$110,900. The \$14,100 of annual income above the cap simply disappears from the calculation. You paid income tax on every dollar of it. The WCB does not insure it.

Weeks thirteen through fifty-two: the cap, the deeming, the disputes

WCB benefits are not a guarantee. They are an adjudicated entitlement. The Board may require independent medical examinations to confirm your injury. It may dispute the connection between the injury and the workplace. It may apply a "deemed earning capacity" — a determination that you are capable of earning a certain amount in some hypothetical job, even if you cannot actually find that job. Your benefit is reduced by the deemed amount, regardless of whether the deemed earnings exist in reality.

If you are a high-income earner above the cap, the gap between what you lost and what WCB replaces grows every month. If you are a moderate-income earner inside the cap, you are receiving roughly two-thirds of your previous take-home pay (90% of net, less the cap effect, less any deemed earnings). Your mortgage did not adjust. Your kids' activities did not adjust. The cost of groceries did not adjust. The cost of the medication you now require did not adjust.

Year two: when WCB ends or denies

WCB benefits end when you can return to work, when a permanent partial impairment award is settled, when a permanent total disability pension begins, or — for most workers — when WCB and your treating physicians disagree about whether you are still disabled enough to qualify. If WCB closes your file and you cannot return to work, there is no automatic transfer to anything else. You apply for what comes next. You apply for AISH.

If your disability did not happen at work — if you developed multiple sclerosis, or had a stroke, or were diagnosed with cancer that became disabling — there was no WCB to begin with. You started at the AISH application.

What AISH is

The Assured Income for the Severely Handicapped pays \$1,940 per month. (*Source: Government of Alberta — AISH program page, 2026.*) That is \$23,280 per year, before any non-cash benefits. Average one-bedroom rent in Alberta in 2026 is between \$1,500 and \$1,900 per month. (*Source: Canada Mortgage and Housing Corporation rental market data; Inclusion Alberta Fact Sheet, December 2025.*) For a single parent of three children needing a three-bedroom unit, the average rent is \$2,138 per month. That is more than the entire AISH benefit, before a single grocery item is purchased.

To qualify for AISH, you must have a permanent disability that substantially limits your ability to earn a living, and you must meet the financial-eligibility tests. The financial tests are the part most working-age Canadians do not know about. Liquid assets — money in chequing, savings, TFSAs, RRSPs, GICs, non-registered investments, and most other savings vehicles — are capped at \$5,000 for a single applicant and \$10,000 for a couple with dependents. (*Source: Government of Alberta — AISH financial eligibility,*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

2026.) A vehicle is exempt up to a value the Board determines. A primary residence is exempt. Almost everything else you have built must be liquidated before AISH will pay you a dollar.

The \$23,000 in your RRSP — the retirement savings you built across twenty years of work — must be collapsed. You will pay withholding tax on the collapse, between 10% and 30% depending on the amount, plus the rest will be added to your income for the year and taxed at your marginal rate. The \$8,000 in your TFSA must be liquidated. The vehicle worth more than the exemption must be sold. The basement you finished yourself adds to the equity in your house, which is exempt from the asset cap, but you cannot eat the basement. The house equity does not pay your property taxes, your utilities, your insurance, or your maintenance — all of which still come due.

When you have spent down to the asset cap, AISH will begin to pay you \$1,940 per month.

What the cohabitation rule does

You are married. Your spouse works part-time and earns \$38,000 per year. AISH does not pay a married couple as if they are two individuals; it treats your spouse's income as household income for the purpose of determining your benefit. (*Source: Government of Alberta — AISH benefits and exemption rules; Inclusion Alberta — AISH Cohabitation Briefing, 2025.*) Your spouse's earnings reduce your AISH cheque. The exemption thresholds are calculated against the household, not against you. Above the partner-income exemption — which is being reduced from \$2,612 per month to \$1,500 per month under the post-July 2026 framework — every dollar your spouse earns reduces your AISH benefit by a percentage that approaches dollar-for-dollar at higher household incomes.

Your spouse cannot work less to preserve your benefit, because the household needs the income. Your spouse cannot work more to make up the gap, because every additional dollar earned reduces your benefit. The system has placed your spouse in a vise. The longer it lasts, the tighter the vise gets.

What this does to a marriage

This is the part the document does not need to argue. The pattern is documented in every disability-policy literature in this country. A spouse who is now the sole earner, the primary caregiver, the financial decision-maker, and the person responsible for managing a household whose income has dropped by 60% — while caring for a partner whose physical or cognitive condition has changed permanently, and often while still raising children — is a spouse under structural conditions that no marriage was designed to survive. Most do not. The literature on this is consistent across jurisdictions.

The decision to separate is rarely framed by the people inside it as a financial decision. It is framed as exhaustion, as resentment, as drift, as the loss of the person they married, as the cost of constant care without support, as the slow erosion of a partnership under conditions of permanent crisis. The financial mechanism that produced those conditions does not appear in the conversation. It does not need to. It already did its work.

The Canada Disability Benefit, and what Alberta does with it

The federal government created the Canada Disability Benefit in 2024 to lift working-age Canadians with disabilities above the poverty line. The CDB pays up to \$200 per month to eligible recipients. Alberta is the only province in Canada that claws this benefit back dollar-for-dollar from AISH. (*Source: Inclusion Alberta — Canada Disability Benefit in Alberta Fact Sheet, July 2025; Petit, G., Canadian Tax Observatory, April*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

2026.) The federal money intended to ease your situation is captured by the provincial calculation. You receive zero net benefit from the CDB. The federal investment is absorbed by the province.

The same pattern operates with the Canada Pension Plan Disability benefit. 16,161 Albertans on AISH — 20.4% of the entire AISH caseload as of September 2025 — receive CPP-D from contributions they made during their working years. (Source: *Government of Alberta — Assisted Living and Social Services Open Data, AISH Caseload September 2025*.) Every dollar of CPP-D is deducted from their AISH benefit. They paid into CPP for ten, fifteen, twenty years before becoming disabled, qualified for CPP-D when their disability ended their employment, and receive zero net benefit from the contributions they made. Their total income is identical to the income of a person who never worked a day in their life.

Five years in

Five years after the accident, you are forty-seven. You are no longer married. The basement you finished is in the house your former spouse and the children live in; you live in a one-bedroom apartment that takes most of your monthly AISH cheque. The RRSP is gone. The TFSA is gone. The vehicle is gone. The line of credit you used to bridge the gap between WCB ending and AISH starting is still being paid down. You have not held a job in five years and the gap on your résumé is growing in a way that means even if your disability allowed you to return to part of your previous work, no employer is now going to consider you.

You have learned the unwritten rules of the system. You have learned that any money in your account on the first of the month above \$5,000 puts your benefit at risk. You have learned that any new relationship triggers a reassessment. You have learned that the call centre that replaced your individual caseworker in 2024 cannot tell you anything that requires looking up your file. (Source: *Inclusion Alberta ADAP Fact Sheet, December 2025; AUPE — AISH Caseworker Caseload Statement, 2025*.) You have learned to call back the next day, get a different person, and try again.

You are eighteen years away from sixty-five. You are eighteen years away from the senior system.

What is waiting at sixty-five

This document exists because most Canadians believe — because they have been allowed to believe — that the senior system is qualitatively different. That the Canada Pension Plan, Old Age Security, and the Guaranteed Income Supplement constitute a different kind of program from AISH and ADAP — a program that is not stingy, not punitive, not built around the assumption that anyone receiving it is gaming it, not designed to extract the maximum from the federal contribution while delivering the minimum to the recipient.

Most Canadians believe this because nobody has shown them the architecture from end to end. The architecture is the same. The mechanism is the same. The cohabitation penalty is the same. The asset capture is the same. The contribution-versus-receipt mismatch is the same. The structural pressure to live alone, to remain unmarried, to declare separation in order to preserve income, is the same. The names of the programs change at sixty-five. The cage does not change. It changes shape.

Movement Two of this document walks the architecture, program by program, dollar by dollar, source by source. Movement Three documents how the architecture operates worst on Indigenous, 2SLGBTQ+, immigrant, racialized, and other intersectionally affected populations. Movement Four is the closing — written for the seniors who already live inside this cage, who have been single-on-paper for years to keep the Guaranteed Income Supplement, who lost a spouse and an income in the same week, who watched

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

their working-life savings vanish into long-term care accommodation costs, who never got back what they paid in.

The document is also written for the welder, the heavy-duty mechanic, the hospital cleaner, the warehouse picker, the long-haul driver, the meat-plant worker. If you are forty-two years old and reading this and your body still does what you ask of it, you are reading the document about what is waiting if it stops. The architecture exists. The numbers are published. The mechanism is documented. None of this is hidden. It has only been distributed in pieces, across different programs, in language designed to keep each piece separate from the others, so that the cage can only be seen from the inside.

This document assembles the cage. It exists so that the cage can be seen from the outside, by the people who do not yet know they are walking toward it.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.2 — Inside WCB: The Cap, the Deeming, the Disputes, the Cliff at 65

The Workers' Compensation Board of Alberta is the entry point to the cascade for most workers whose disability begins at work. It is also the system that working-age Canadians have the most public-facing trust in, because it is the system most working-age Canadians have heard of. Most have not been inside it. This section walks through what is actually inside it.

The Workers' Compensation Act of Alberta establishes a no-fault insurance system. Workers give up the right to sue their employer for workplace injury or illness, and in exchange the Board pays defined benefits regardless of who was at fault. Employers fund the system through premiums. Workers are not consulted on premium rates. The Board sets them. (*Source: Workers' Compensation Act, R.S.A. 2000, c. W-15; WCB-Alberta — Rate Setting page, 2026.*)

The Board operates at arm's length from government. The Government of Alberta appoints its Board of Directors. Policy changes flow from legislation, from Board decisions, and from Government direction through the Minister responsible. The Workers' Compensation Act has been amended repeatedly across successive governments. The amendments are not neutral. Each amendment shifts the balance of risk between worker and employer. The most recent significant amendment cycle — Bill 30 in 2017 under the New Democratic Party government, partial rollback under the United Conservative Party government beginning in 2019 — is documented and politically tracked. This section names that political history because the political history is what the worker reading this document will encounter as the operational rules of their claim.

The wage-replacement formula

WCB-Alberta calculates wage replacement at 90% of net earnings, where net earnings means gross employment earnings less federal income tax, provincial income tax, Canada Pension Plan contributions, and Employment Insurance premiums. (*Source: WCB-Alberta — Wage replacement page, 2026.*) The Board sets a Maximum Compensable Earnings amount each year — the ceiling above which earnings are not insurable for wage-replacement purposes. The Maximum Compensable Earnings amount for 2026 is \$110,900. (*Source: WCB-Alberta — Maximum Compensable and Assessable Earnings Fact Sheet, January 2026.*)

For the worker earning at or below the cap, the formula is straightforward: take gross earnings, subtract tax and CPP and EI, take 90% of the result. A worker earning \$80,000 per year in gross income receives a wage-replacement benefit calculated against approximately fifty-\$5,000 in net earnings, paid at 90% of that figure, which produces roughly \$49,500 per year — about \$9,500 below the original gross. The worker's actual cost of living, however, did not adjust by \$9,500 to match.

For the worker earning above the cap, the gap is larger and grows with income. A worker earning \$150,000 per year is treated by the Board as if they earned \$110,900 per year. The \$40,000 of annual income above the cap is uninsurable for wage-replacement purposes. The worker paid full federal and provincial income tax on that income. The Board does not insure it.

What the cap politics actually were

Between September 1, 2018, and December 31, 2020, there was no cap on wage-loss benefits in Alberta. (*Source: WCB-Alberta — Wage replacement, "Date of Accident" timeline; BCL Consulting — Maximum*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Insurable Earnings, 2018.) Workers injured during this 28-month window had their benefits calculated at 90% of actual net earnings, with no ceiling. This was a direct consequence of Bill 30, passed by the Alberta Legislature in late 2017 under the New Democratic Party government, on the recommendation of the WCB Review Panel established that year. The cap removal was one of the most significant pro-worker amendments in Alberta WCB history.

On January 1, 2021, the cap was restored. (*Source: WCB-Alberta — Wage replacement, 2026.*) The restoration was implemented under the United Conservative Party government, which had won the 2019 provincial election in part on a platform of reversing key elements of the previous government's labour reforms. The Maximum Compensable Earnings cap was reinstated and has been adjusted annually since. The cap restoration applies to all injury dates on or after January 1, 2021.

This means, as a documented matter of provincial policy, that two workers with identical injuries and identical earnings are entitled to different benefits depending on which date they were injured. A worker injured in October 2020, earning \$150,000 per year, has wage-replacement benefits calculated against the full one hundred and fifty thousand. A worker injured in October 2026, earning the same \$150,000 per year, has wage-replacement benefits capped at \$110,900. The same provincial government that reinstated the wage-replacement cap is the government that designed the AISH-to-ADAP transition documented elsewhere in the Alberta Disability System Breakdown campaign. The structural pattern is consistent. Workers receive less of what they paid into. Employers and the public treasury retain more.

Independent Medical Examinations

When a worker's claim is accepted by the Board, treatment proceeds under the supervision of the worker's treating physicians. The Board, however, retains the right to order Independent Medical Examinations — assessments conducted by physicians under contract to WCB-Alberta whose role is to provide the Board with a medical opinion independent of the treating relationship. (*Source: WCB-Alberta — Procedures Manual, Health Care, Application 1, 2025.*)

The IME is not, in operation, an independent process. The IME physician is paid by WCB. The IME report is provided to the Board. The Board uses the IME report to determine whether to continue, modify, or terminate benefits. If the IME report disagrees with the treating physician's assessment — which it frequently does, particularly on questions of work-readiness, recovery timeline, and ongoing impairment — the Board is institutionally positioned to weight the IME report more heavily. The worker's treating physicians may disagree in writing. The Worker may file a dispute. The dispute proceeds through internal Board review and, if not resolved, through the Appeals Commission. The dispute process can take 18 months to 3 years to complete. During that time, the worker's benefits are determined by the Board's most recent decision. If that decision was to terminate based on an IME, the worker is without benefits for the duration of the dispute.

The IME mechanism is the single most consequential operational reality inside WCB. It is the mechanism through which the Board exercises discretion over claim outcomes. It is documented in academic and legal literature on workers' compensation systems across Canadian jurisdictions. (*Source: Lippel, K., et al., "The mental health of workers' compensation claimants," Institute for Work & Health, 2016; Storey, R., "From the Pittston Strike to the U.S. Workers' Compensation Reform Movement," Labour/Le Travail, 2009.*) It is not unique to Alberta. It is consequential everywhere it operates.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Deemed earning capacity

For workers who cannot return to their pre-accident occupation but are determined by the Board to retain partial earning capacity, the Board applies a "deemed earning capacity" calculation. The worker's wage-replacement benefit is reduced by the amount the Board determines the worker is capable of earning in some post-injury occupation, regardless of whether the worker has actually obtained that employment.

A worker who earned \$80,000 per year as a heavy equipment operator and is determined to retain capacity for a forty-thousand-dollar-per-year sedentary administrative role has their wage-replacement benefit calculated against the difference between pre-injury and post-injury deemed earnings, not against actual wage loss. If the worker cannot find the deemed administrative role — because they have no administrative experience, because they live in a region without administrative jobs, because employers will not hire a worker with their injury history, because the disability is more limiting in practice than the IME report assessed — the deemed earning capacity stands. The benefit is reduced as if the deemed earnings were real.

The Board's policy permits review of deeming decisions. The review process is the same Board process that produced the original deeming decision. (Source: WCB-Alberta — Policy 04-04, *Permanent Disability; Advisor Office — Economic Loss Payments*, 2025.)

Maximum Medical Improvement

At a point determined by the Board's medical advisors, a worker is declared to have reached Maximum Medical Improvement, or MMI. The MMI determination is the gateway to permanent-disability adjudication. Before MMI, benefits flow as Temporary Total Disability (TTD) or Temporary Partial Disability (TPD) based on the Board's view of the worker's recovery progress. After MMI, benefits flow as Permanent Clinical Impairment (PCI) lump-sum awards or Economic Loss Payment (ELP) ongoing pensions, depending on the Board's assessment of the worker's permanent functional limitations and earnings impact.

The MMI determination is consequential. A worker declared to have reached MMI prematurely — before all available treatment has been attempted, before the full natural history of the condition has played out, before secondary conditions have been recognized — is locked into a permanent assessment based on incomplete clinical information. The PCI lump-sum award is calculated against a percentage-impairment rating assigned by a WCB-approved physician applying the American Medical Association's Guides to the Evaluation of Permanent Impairment. The rating produces a percentage. The percentage produces a dollar amount. (Source: WCB-Alberta — *Permanent Disability Compensation Fact Sheet WFS-463*, 2020; AMA *Guides to the Evaluation of Permanent Impairment*, 6th edition.) For a moderate impairment in the ten-to-20% range, the PCI lump sum may be in the range of twenty to \$40,000. For most workers, this lump sum is the only "settlement" they will ever receive. It is not designed to replace lost lifetime earnings. It does not.

The appeals architecture

A worker who disagrees with a Board decision has access to two layers of appeal. The first is the Dispute Resolution and Decision Review Body (DRDRB), which is internal to WCB. The DRDRB reviews the Board's decision and may uphold, modify, or reverse it. The second layer is the Appeals Commission, which is a separate quasi-judicial body. (Source: *Workers' Compensation Act*, R.S.A. 2000, c. W-15, Part 6 — *Appeals Commission for Alberta Workers' Compensation*; WCB-Alberta — *Appeals Process page*, 2026.)

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The Appeals Commission's decisions are binding on the Board. The Appeals Commission's decisions are subject to judicial review only on procedural grounds, not on the merits.

The appeals process can take 18 months to 3 years to complete. During the appeal, the worker continues to receive benefits at the level set by the most recent Board decision. If that decision was to terminate or reduce benefits, the worker is without those benefits during the appeal period. Workers who are financially unable to wait through the appeal often accept a reduced settlement to end the process. This is a documented operational outcome of long appeal timelines in workers' compensation systems generally. (Source: Lippel, K., "Compensation for Mental-Mental Claims under Canadian Workers' Compensation Law," *Health Law Journal*, 2002.)

When WCB ends

WCB benefits end in one of several ways. The worker may recover to the extent that the Board determines wage loss has resolved. The worker may settle a permanent partial impairment claim and exit the wage-loss system entirely. The worker may have benefits terminated based on an IME or deeming decision the worker disputes but does not win. The worker may exhaust appeals.

If the worker remains disabled and unable to work, and WCB benefits have ended through any of these mechanisms, there is no automatic transfer to anything else. The worker is now an applicant. They apply for AISH. They apply for the Canada Pension Plan Disability benefit if they have sufficient CPP contributions. They apply for Employment Insurance sickness benefits if they qualify. They wait. The wait, depending on the program, may be months. The wait for AISH, particularly when application volumes are high, may be longer than two years. (Source: AUPE — AISH Caseworker Caseload Statement, 2025; Inclusion Alberta ADAP Fact Sheet, December 2025.) During the wait, the worker has no income from the public system. Whatever savings they have are spent down. Whatever family supports they have are drawn upon.

This is the gap. It is structural. It is not the result of a single bad decision or a single underperforming office. It is the product of a system in which WCB-Alberta, AISH, ADAP, and CPP-D are administered separately, communicate poorly, and were not designed to operate as a continuous safety net for the worker who needs to move between them.

The cliff at 65

For workers receiving Economic Loss Payments — the WCB-Alberta wage-loss benefit for permanent earnings impact — the benefit is recalculated at age 65, on the assumption that a sixty-five-year-old worker would normally have retired. (Source: WCB-Alberta — Policy 04-04, Part II, Application 4: Economic Loss Payment; WCB-Alberta — Permanent Disability Compensation Fact Sheet WFS-463, 2020.) The recalculation is governed by a defined formula. The Board takes the total wage-loss compensation paid from the date of accident to the date of retirement adjustment and multiplies it by 2%. The result is the worker's annual ELP retirement benefit, paid for the rest of their life unless commuted to a lump sum.

WCB-Alberta's own published example, drawn from its worker fact sheet, illustrates the magnitude of this adjustment. A worker injured at age fifty-five in 2018, receiving Economic Loss Payments of \$48,300 per year, who retires at age 65 in 2028, has received \$483,000 in cumulative wage-loss benefits over the ten-year period. (Source: WCB-Alberta — Permanent Disability Compensation Fact Sheet WFS-463, June 2020, Worker Example.) At age 65, the ELP retirement benefit is calculated at 2% of that cumulative figure:

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

\$9,660 per year. That is \$805 per month, for the rest of the worker's life.

The worker's wage-loss benefit has dropped from approximately \$4,000 per month to approximately \$800 per month, on a single calendar day, on the worker's sixty-fifth birthday.

The worker has the right to argue that they would have continued to work past age 65 had they not been injured. (Source: WCB-Alberta — Advisor Office, *Economic Loss Payments After 65*, 2025.) The burden of proof is on the worker. Acceptable evidence includes: a written letter from a financial planner demonstrating that the worker's retirement savings are insufficient to retire at sixty-five; documentation of a mortgage extending past age 65; evidence of professional licensing or certification renewals indicating intent to continue working; employer statements confirming the worker had no planned retirement date. The Board reviews the evidence and decides. If the Board determines the worker would have retired at sixty-five, the ELP retirement adjustment proceeds. The decision can be appealed.

For most workers, the decision is not appealed successfully. The cliff arrives. The benefit drops. The worker, now sixty-five, transitions to Canada Pension Plan retirement, Old Age Security, and — if income is low enough — the Guaranteed Income Supplement. The remainder of this document walks the architecture of those programs.

The continuity argument, restated

The worker who fell from a height at age forty-two, in the cascade traced in Section 1.1, is now sixty-five. The intervening twenty-three years have included WCB benefits at reduced replacement rates, then disputes, then deeming, then a permanent partial impairment award, then a long-running Economic Loss Payment, then — at sixty-five — the 2% adjustment that reduces monthly income from approximately \$4,000 to approximately \$800 overnight. The worker has not been on AISH; the worker has remained inside the WCB system throughout. Even on the favourable side of the cascade, where AISH and the asset cap and the cohabitation rule were avoided, the worker arrives at sixty-five with a wage-loss benefit reduced to one-fifth of its previous value, no employer pension, no significant retirement savings (because for twenty-three years there was no surplus income to save), and a transition into the senior income system that begins at the floor.

The same worker, on the unfavourable side of the cascade — where WCB ended early, AISH became the primary income, and the asset cap forced liquidation of RRSPs, TFSAs, and vehicles — arrives at sixty-five with no savings to transition into the senior system at all. The senior system will not differentiate between these two workers in any meaningful way. CPP retirement is calculated on contribution history, and the worker's contribution history was interrupted at age forty-two. OAS is residency-based and pays the same to both workers. GIS is income-tested and pays the higher amount to the worker with less other income, which means the unfavourable-cascade worker actually receives slightly more from GIS — a documented inversion of the lifetime contribution principle, examined in Movement Two.

The cage that this document names is a continuous architecture from workplace injury to long-term care. The worker who falls at forty-two and the senior who divorces on paper at seventy-three to preserve the Guaranteed Income Supplement are in the same architecture, at different stages. Section 1.3 walks the AISH application — the unfavourable side of the cascade — in operational detail.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.3 — Inside AISH: The Application, the Spend-Down, the Wait, the Appeal You No Longer Have

Most workers do not arrive at AISH having read the program rules. Most workers arrive at AISH because WCB ended, or because the disability did not arise at work and was therefore never a WCB claim, or because Canada Pension Plan Disability has a contribution-history requirement they cannot meet, or because Employment Insurance sickness benefits have run out. Most workers arrive at AISH late, with their finances already substantially depleted, having relied on family supports and credit and the assumption that the next program would catch them. AISH is the program of last resort. It is administered as if applicants are gaming the system, not as if they have arrived after every other system has failed.

This section walks the AISH application in operational detail. It is a long section because the application is a long process and the operational reality of that process is the operational reality of the system the document is naming.

What you must establish

To qualify for the Assured Income for the Severely Handicapped program, an applicant must establish three things. First, that they have a permanent disability — defined under the Assured Income for the Severely Handicapped Act as a medical condition that is likely to remain with the applicant for the rest of their life and that substantially limits their ability to earn a livelihood. (*Source: Assured Income for the Severely Handicapped Act, R.S.A. 2000, c. A-45.1, section 3; Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007.*) Second, that they meet the financial-eligibility tests, which include both an income test and an asset test. Third, that they are an Alberta resident, eighteen or older, and not in a federal correctional facility serving a sentence of two years or more.

The medical determination is made on the strength of the Part B Medical Report, completed by the applicant's physician and submitted to the Board's adjudication staff. The financial determination is made on the strength of the Part A application, completed by the applicant, including disclosure of all bank accounts, all investment accounts, all real estate holdings, all vehicles, all life insurance with cash value, all employment income, all government benefit income from any other source, and all support payments received or paid. The Government of Alberta will request bank statements going back twelve months. (*Source: Government of Alberta — How to Apply for AISH, 2026; AISH Policy Manual.*)

The medical evidence problem

The Part B Medical Report requires a physician signature. Specifically, it requires the signature of a "regulated medical professional" whose qualifications are defined in regulation. For most components of the form, that means a physician — a medical doctor, either a general practitioner or a specialist with treating relationship to the applicant. Nurse practitioners are permitted to complete certain components of the Part B but, as of the 2026 program design, are not authorized to sign all sections of the form, despite NP scope of practice having expanded significantly under recent provincial reforms. (*Source: Edmonton Journal — "I can't sign the medical: Alberta nurse practitioner says AISH rules present barrier for patients," January 11, 2024; Government of Alberta — AISH application guidelines, 2026.*)

This requirement collides directly with the Alberta primary-care environment. As of 2024, 650,000 Albertans had no family physician. (*Source: Alberta Medical Association — AB's Family Medicine Crisis statement,*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

2024; CBC News — "Concerns grow as more and more Albertans can't find a family doctor," April 27, 2022; ThinkHQ survey commissioned by AMA, January 2024.) The number of family practices accepting new patients in Alberta dropped from 887 in 2020 to 164 in 2024. (Source: Medscape Medical News — "Alberta's Family Doctor Shortage Involves More Than Numbers," October 6, 2025.) Sixty-1% of Alberta family physicians, surveyed by the AMA in 2024, were considering leaving the provincial healthcare system. (Source: Global News — "61% of family physicians in Alberta considering leaving provincial health-care system: Report," January 24, 2024.)

The applicant who has no family physician must obtain one to complete the AISH Part B. This is not a quick task. Wait times for new patient acceptance, where any spots exist at all, may be six to twenty-four months. The applicant may pay out of pocket for a private clinic appointment to obtain a Part B signature, but private clinics are not always willing to complete government forms for patients without an established relationship. Walk-in clinics generally will not. The applicant may seek the signature of a physician with whom they have only a brief treating relationship — an emergency room physician, an urgent-care physician, a specialist seen once for a referral. Such physicians frequently decline to complete the Part B because they have not seen the applicant longitudinally and cannot attest to permanence. The Part B requires attestation to permanence. The applicant whose disability is real and permanent and who cannot find a physician to attest to it is, for the purpose of the program, ineligible.

The Government of Alberta covers a one-time supplemental payment of \$150 to compensate the physician for completing the Part B. (Source: Government of Alberta — "How to apply for AISH" — note: "your doctor may charge you for this service," 2026.) Current physician fees for completing complex disability forms range from \$250 to \$400, sometimes higher for specialist-level forms. (Source: Alberta Medical Association — *Schedule of Medical Benefits, uninsured services*, 2025.) The supplemental payment does not cover the actual cost. The difference is paid out of pocket by the applicant — at the moment in their life when out-of-pocket capacity is at its lowest.

The asset spend-down

The AISH financial-eligibility tests cap liquid assets at \$5,000 for a single applicant and \$10,000 for a household with dependents. (Source: *Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007, sections 4-7*; Government of Alberta — *AISH financial eligibility*, 2026.) Liquid assets are defined broadly. The category includes:

- Chequing and savings accounts at any bank, credit union, or other financial institution
- Tax-Free Savings Accounts (TFSAs)
- Registered Retirement Savings Plans (RRSPs)
- Registered Retirement Income Funds (RRIFs)
- Non-registered investment accounts
- Guaranteed Investment Certificates (GICs) and term deposits
- Cash on hand
- The cash surrender value of life insurance policies
- Stocks, bonds, mutual funds, ETFs, and similar securities
- Cryptocurrency holdings

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Accounts receivable, including loans owed to the applicant by family or third parties

A primary residence is exempt from the asset cap, though the equity in the residence is reviewed in some circumstances. A vehicle is exempt up to a value the Board determines. Household furnishings, personal effects, tools required for self-employment, and certain other categories are exempt. A Registered Disability Savings Plan (RDSP) is excluded from the asset test up to defined federal thresholds. Almost everything else must be liquidated.

The operational reality of this is what the working-age reader most needs to understand. An applicant with \$23,000 in an RRSP, accumulated across two decades of disciplined retirement saving, must collapse the RRSP. Collapsing an RRSP triggers withholding tax — 10% on amounts up to \$5,000, 20% on amounts between \$5,000 and \$15,000, 30% on amounts above \$15,000. (*Source: Canada Revenue Agency — RRSP withholding tax rates, 2026.*) The withheld amount goes to the Canada Revenue Agency. The remainder is added to the applicant's income for the year and taxed at the applicant's marginal rate at year-end. For an applicant who had any employment income earlier in the year before the disability, or who collapsed multiple registered accounts, the year-end tax bill can be substantial.

After tax, the applicant is left with a fraction of the saved amount. This is the amount the AISH cap requires them to spend down. Once they are below the cap, AISH will pay \$1,940 per month. The retirement savings are gone, the tax has been paid, the runway has been consumed, and the program — the program — pays subsistence.

A TFSA must be liquidated. There is no withholding tax on TFSA withdrawals, but the TFSA contribution room is permanently consumed for the year of withdrawal in the case of a TFSA that has been overcontributed; there are technical rules around recontribution that the applicant typically does not understand and does not have time to learn. The vehicle worth more than the exemption must be sold. The applicant who lives in a region of Alberta without functional public transit then has no transportation. The applicant who needed the vehicle to attend medical appointments now requires medical transportation services that AISH does not fully cover.

The home that is exempt and not exempt

The primary residence is exempt from the asset cap. This is sometimes presented in public-facing AISH summaries as if it means the applicant who owns a home is in a stable position. It does not mean that.

The home is exempt from being counted as a liquid asset. The home is not exempt from carrying costs. Property taxes continue. Property insurance continues. Utility bills continue. Maintenance, repairs, and replacement of failing systems — furnaces, roofs, water heaters, foundations — continues. Strata fees, condominium fees, and homeowners' association fees continue. None of these costs are reimbursed by AISH. All of them are paid out of the \$1,940 per month.

The applicant who owns a paid-off home in a moderate-cost region of Alberta may find that property tax alone consumes \$150 to \$400 per month, depending on assessed value and municipality. Insurance adds another \$75 to \$150. Heating, electricity, water, and waste add \$300 to \$500 per month depending on season. Maintenance averaged across the year — even at minimum — adds another hundred. Before food, before medications, before clothing, before any other expense, the homeowner on AISH may have spent \$600 to \$1,200 per month on home carrying costs alone. The remaining \$700 to \$1,300 is what is left for everything else.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The applicant who falls behind on property taxes or insurance can lose the home to municipal tax sale or to insurance lapse. This is documented in housing-counselling literature in Alberta and across Canadian provinces with similar program structures. (Source: *Canadian Centre for Policy Alternatives — "Disability and Housing Insecurity in Canada," 2023; Alberta Council of Disability Services — annual policy briefs, 2022-2025.*) The "exempt" home becomes the asset the applicant loses last, but loses nonetheless, when the program income proves inadequate to maintain it.

The wait

Once the application is submitted with complete medical and financial documentation, the wait begins. The Government of Alberta has not, as a matter of public policy, published current AISH application processing times. Disability advocacy organizations and applicants have documented wait times of six months to over two years, with significant variation by region and adjudication office. (Source: *Inclusion Alberta ADAP Fact Sheet, December 2025; AUPE — AISH Caseworker Caseload Statement, 2025.*)

During the wait, the applicant has no AISH income. If the applicant has exhausted Employment Insurance sickness benefits, terminated WCB benefits, and depleted personal savings to the AISH asset cap in anticipation of approval, the applicant has no income at all during the wait. Some applicants are eligible for Income Support (the general welfare program in Alberta), at substantially lower rates than AISH, while waiting. Income Support is itself asset-tested and income-tested. The applicant's family may step in. The applicant may use food banks. The applicant may run out of medications. The applicant may experience a mental-health crisis, hospitalization, eviction, or homelessness, while waiting for the program designed to support them.

This wait is not a glitch. It is the operational reality. It is documented across multiple advocacy reports and across the disability community's own organizational publications. (Source: *Inclusion Alberta — "Experiences with Family Support for Children with Disabilities," January 2025; Inclusion Alberta / ALIGN — "PDD Current State: Critical and Urgent," January 2025; Disability Action Hall — public statements 2024-2026.*)

The 2024 caseworker elimination

In 2024-25, the Government of Alberta eliminated individual AISH caseworkers province-wide and replaced them with a centralized call centre. (Source: *AUPE — AISH Caseworker Caseload Statement, 2025; Inclusion Alberta ADAP Fact Sheet, December 2025.*) Prior to this change, an AISH recipient had a named caseworker who was familiar with the file and reachable by direct phone or email. After the change, the recipient calls a general number, waits on hold, speaks to a different agent on each call, and may need to re-explain their situation each time. Caseworker caseloads, prior to elimination, had reached up to 200 files per worker — more than double the recommended maximum of 90 to 105. (Source: *AUPE — AISH Caseworker Caseload Statement, 2025.*)

The call centre model is documented as inaccessible to recipients with social anxiety, agoraphobia, severe communication-related disabilities, and brain injuries — categories of disability that are themselves AISH-qualifying. (Source: *Inclusion Alberta ADAP Fact Sheet, December 2025.*) The applicant who qualifies for AISH on the basis of a condition that makes phone calls difficult must, to access the program, navigate a phone-only contact system. The system was not designed around its users. It was designed around administrative cost containment.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The appeal you used to have

Until December 2025, an AISH applicant or recipient who disagreed with a Board decision had access to an independent Citizens Appeal Panel. The Panel was a quasi-judicial body composed of community members with relevant expertise, operating at arm's length from the Government and from the AISH adjudication staff. The Panel's decisions were binding on the program. Applicants who succeeded at the Panel had their decisions reversed and benefits reinstated, often with retroactive payment.

On December 9, 2025, the Alberta Legislature passed Bill 12, the *Disability Assistance Programs Act*. (Source: Government of Alberta — Bill 12, Disability Assistance Programs Act, Royal Assent December 9, 2025.) Bill 12 abolished the independent Citizens Appeal Panel. In its place, Bill 12 established a Medical Review Panel composed of medical professionals selected by the Government. The Medical Review Panel reviews disputed eligibility decisions. Its decisions are final. There is no independent appeal from a Medical Review Panel decision. Judicial review remains available only on procedural grounds, not on the merits.

Inclusion Alberta's November 26, 2025 statement on Bill 12 stated that the legislation "erodes rights and deepens poverty for Albertans with disabilities." (Source: Inclusion Alberta — Media Release, November 26, 2025.) The Alberta Medical Association, the Alberta Union of Provincial Employees, Disability Action Hall, and the People's Alliance for Disabled Albertans, among others, opposed Bill 12. No major Alberta disability advocacy organization publicly endorsed it. The municipal coalition documented in the Alberta Disability System Breakdown Coalition and UN CRPD Addendum — including the cities of Edmonton, Calgary, Lethbridge, Camrose, Claresholm, Red Deer, and Medicine Hat — has called for the AISH-to-ADAP transition to be paused pending meaningful consultation, in part because of the appeal-rights removal Bill 12 effected.

The applicant who, as of 2026, is denied AISH or has benefits terminated does not have the remedy that applicants had as recently as one year ago. Canada ratified the United Nations Convention on the Rights of Persons with Disabilities in 2010. (Source: United Nations General Assembly Resolution A/RES/61/106, 13 December 2006; Canada — instrument of ratification deposited March 11, 2010.) Article 13 of the Convention protects access to justice for persons with disabilities, including procedural accommodations and effective remedy. Bill 12 has, in operational effect, removed the effective remedy for AISH eligibility disputes in Alberta. The campaign's Coalition and UN CRPD Addendum documents the engagement of Articles 12, 13, 19, and 28 by the Bill 12 changes; that documentation is incorporated into this Section by reference.

What the applicant has by the end

The applicant who completes the AISH process — finds the physician, completes Part B, completes Part A, liquidates assets, waits the wait, receives approval — has by the end of the process a benefit of \$1,940 per month, an asset cap they must remain under for as long as they receive the benefit, a cohabitation calculation that ties their benefit to their spouse's income, a call centre as their primary point of contact with the program, and no independent appeal if the program's adjudicators err.

The applicant has also, in most cases, depleted retirement savings, personal savings, and family financial supports during the application and approval process. The runway is gone. The benefit is the floor. From this point forward, until age 65, the applicant is inside AISH. Section 1.4 walks the AISH-to-ADAP transition that begins July 1, 2026 — the next reassessment, the next reduction, the next cliff that a successful AISH

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

applicant in 2025 had no way of anticipating when they applied.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.4 — The Next Reassessment: The AISH-to-ADAP Transition, July 1, 2026

This document is being written and circulated in the months immediately preceding July 1, 2026 — the date on which the Government of Alberta's restructuring of the Assured Income for the Severely Handicapped program into the Alberta Disability Assistance Program takes effect. The transition is the immediate operational threat to 79,290 Albertans currently receiving AISH. (Source: *Government of Alberta — Assisted Living and Social Services Open Data, AISH Caseload September 2025*.) The structural mechanics of the transition are documented across the Alberta Disability System Breakdown Evidentiary Report Series, the Compound Failure Brief, the Consultation Gap Rebuttal, and the Coalition and UN CRPD Addendum. This Section integrates the central facts into the lifecycle architecture this document is naming, without restating the full evidentiary record.

The single most important lifecycle observation about the AISH-to-ADAP transition is this: it is a reassessment. The applicant who completed the AISH process in 2024, who found the physician, completed the Part B, liquidated retirement savings, depleted personal supports, and waited months or years for adjudication, is now subject to a reassessment that may move them onto a lower-paying program with reduced rights. The investment of time, money, medical effort, and emotional energy in the original application is not preserved. The reassessment can functionally undo it. This is the lifecycle pattern named throughout this document. AISH is a reassessment of FSCD and PDD eligibility. ADAP is a reassessment of AISH eligibility. CPP retirement at 65 is a reassessment of the working-age earnings record. Long-term care entry is a financial reassessment of the senior. The system reassesses continuously. Each reassessment is an opportunity for the system to reduce what the recipient receives.

What ADAP is

The Alberta Disability Assistance Program will replace AISH for most current recipients on July 1, 2026. (Source: *Government of Alberta — ADAP program page, 2026; Bill 12, Disability Assistance Programs Act, December 2025*.) The base monthly benefit under ADAP is \$1,740 — \$200 per month less than the current AISH benefit of \$1,940. (Source: *Government of Alberta — ADAP program page; Inclusion Alberta — ADAP Fact Sheet, December 2025*.) The two-hundred-dollar-per-month difference is offset, on a temporary basis, by a transition supplement that ends on December 31, 2027. After that date, the recipient receives \$1,740 per month with no supplement.

The cost-of-living indexing that applied to AISH benefits — legislated at 2% annually — has been removed under the ADAP framework. (Source: *Inclusion Alberta — ADAP Fact Sheet, December 2025*.) Future increases to the ADAP benefit are at the discretion of the Government, with no statutory floor.

The earnings exemption — the amount a recipient may earn from employment without it reducing their benefit — has been substantially reduced. Under current AISH rules, a single recipient may earn up to \$1,072 per month before exemption-reduction begins. Under the ADAP framework, the exemption is \$700 per month. Under the post-July-2026 AISH framework — for recipients who remain on AISH after the transition — the exemption is reduced to \$350. (Source: *Government of Alberta — ADAP program page; Inclusion Alberta — ADAP Fact Sheet, October and December 2025*.)

The Government's public framing of the seven-hundred-dollar ADAP exemption has been to describe it as "the highest employment income limit among comparable disability income assistance programs in Canada." This framing compares ADAP's seven-hundred-dollar exemption to the new AISH

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

three-hundred-and-fifty-dollar figure rather than to the current AISH one-thousand-and-seventy-two-dollar figure. The correct comparison is the current figure to the new figure. ADAP's exemption is a three-hundred-and-seventy-two-dollar reduction from what AISH recipients currently have. The new AISH exemption is a seven-hundred-and-twenty-two-dollar reduction. There is no version of these changes that is better than what currently exists.

Who chooses, and how

Under the ADAP framework, AISH recipients are not asked which program they would prefer to be on. The transition is automatic. On July 1, 2026, all current AISH recipients other than those who have successfully applied to remain on AISH through the new Medical Review Panel process are moved to ADAP. (Source: *Bill 12, Disability Assistance Programs Act; Government of Alberta — ADAP transition page, 2026*.) The successful application to remain on AISH requires a fresh Part B Medical Report from a treating physician — the same Part B that the original AISH application required, completed under the same primary-care conditions documented in Section 1.3.

79,290 Albertans seeking a fresh Part B medical signature in a province with 650,000 people without a family physician is not an operational plan. It is a policy structure that produces denials by attrition. Recipients who cannot obtain the Part B in time are moved to ADAP by default. The default is the lower-paying program with reduced rights.

What the Medical Review Panel is

Section 1.3 documented that Bill 12 abolished the independent Citizens Appeal Panel. The Medical Review Panel that replaces it is the body that adjudicates applications to remain on AISH after the July 1, 2026 transition. The Panel is composed of medical professionals selected by the Government. Its decisions are final. There is no independent appeal from a Medical Review Panel decision.

The lifecycle implication is direct. A recipient who successfully obtained AISH in 2018, was reassessed and approved for continued eligibility in 2021, and successfully appealed an adverse decision in 2023 through the then-existing independent Citizens Appeal Panel, faces, in 2026, a fresh adjudication by a panel that did not previously exist, on standards that have not been published in operational form, with no independent appeal available. The recipient's twelve-year history of qualifying for AISH does not bind the Medical Review Panel. The Panel adjudicates the application before it. If it denies the application to remain on AISH, the recipient transitions to ADAP at the lower benefit level with no further recourse on the merits.

This is the operational mechanism the United Nations Committee on the Rights of Persons with Disabilities has identified as a violation of CRPD Article 12 — equal recognition before the law. (Source: *United Nations Committee on the Rights of Persons with Disabilities — Concluding Observations on Canada, March 2025*.) 79,290 formal findings of permanent disability — the determinations on which AISH eligibility was granted across the existing caseload — are reversed without new evidence, without individual process, without explanation, by operation of statute. Canada's ratification of the CRPD commits it to protecting legal capacity for persons with disabilities. Bill 12 reverses that protection.

The CDB clawback

The Canada Disability Benefit, created under federal legislation in 2024 and paying up to \$200 per month to eligible disabled Canadians under the age of sixty-five, was designed to lift recipients above the federal

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

poverty line. (Source: Canada Disability Benefit Act, S.C. 2023, c. 17; *Employment and Social Development Canada — Canada Disability Benefit page*, 2025.) Alberta is the only province in Canada that claws back the CDB dollar-for-dollar from AISH benefits. (Source: *Inclusion Alberta — Canada Disability Benefit in Alberta Fact Sheet*, July 2025; Petit, G., *Canadian Tax Observatory*, April 2026; *Government of Alberta — AISH benefits*, 2026.) The recipient receives the federal payment as part of their bank deposit. The provincial system reduces the AISH or ADAP cheque by the same amount in the next payment cycle. The net benefit to the recipient is zero. The federal investment is captured by the province.

The Petit analysis published by the Canadian Tax Observatory in April 2026 documents this pattern at length and identifies it as a structural fiscal-transfer-capture mechanism unique to Alberta within the Canadian provincial system. (Source: Petit, G., "The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis," *Canadian Tax Observatory*, April 22, 2026.) The federal government has not, as of the date of this document, conditioned its disability transfer payments to Alberta on non-clawback compliance. The campaign's federal submission to the House of Commons Standing Committee on Human Resources, Skills and Social Development and the Status of Persons with Disabilities (HUMA) in April 2026 calls for the federal government to do exactly that.

The CPP-D clawback at the working-age stage

16,161 Albertans on AISH — 20.4% of the entire AISH caseload as of September 2025 — receive Canada Pension Plan Disability benefits from contributions they made during their working years. (Source: *Government of Alberta — Assisted Living and Social Services Open Data, AISH Caseload September 2025*.) Every dollar of CPP-D received is deducted dollar-for-dollar from the recipient's AISH benefit. The recipient's net total income is identical to the income of a person who never worked a day in their life and never made a CPP contribution.

The lifecycle observation is the central one for this Section. The blue-collar worker traced through Sections 1.1 and 1.2 — the welder, the heavy equipment operator, the long-haul driver — paid into CPP every working year. If their disability ends their employment and they qualify for CPP-D, they receive a federal benefit they earned through years of contribution. In Alberta, the province captures every dollar of that benefit through the AISH offset. The worker's working-life contribution is rendered worthless at the moment they need it. This is not an accident of program design. It is a deliberate provincial policy that has persisted across multiple Alberta governments, and it has not been reformed under the ADAP transition. Inclusion Alberta, the Public Interest Alberta, the Canadian Centre for Policy Alternatives, and the federal Liberal government's own analysis in the Canada Disability Benefit consultation papers have all identified the offset as inconsistent with the purpose of CPP-D. The offset continues. The province has not, in any public statement, committed to ending it under the ADAP framework. The default presumption — until the Government publishes different rules — is that the offset continues.

What the recipient experiences

The current AISH recipient, on July 1, 2026, will experience the following without acting:

- A two-hundred-dollar-per-month base benefit reduction (offset temporarily by a supplement ending December 31, 2027)
- A three-hundred-and-seventy-two-dollar reduction in the monthly employment income exemption
- The removal of statutory cost-of-living indexing

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- The continuation of the CDB clawback
- The continuation of the CPP-D dollar-for-dollar offset
- The replacement of the independent Citizens Appeal Panel with the Medical Review Panel
- The continuation of the call centre as the primary contact point with the program
- A new requirement, under ADAP, to participate in a personalized employment action plan with a case manager

The personalized employment action plan requirement is the single most consequential operational change for many recipients. It applies to all ADAP recipients except those who have been determined to be not employable. The determination of "not employable" rests with the Government's adjudication staff. Recipients who are determined to be employable — under criteria the Government has not published — must participate in a planning process, with consequences for benefit continuation if they do not. (*Source: Inclusion Alberta — ADAP Fact Sheet, December 2025.*) The case-manager capacity required to deliver seventy-nine thousand individual employment action plans does not exist. The funding to create it has not been announced. The plans, in operational reality, are likely to be administered through the same call centre that replaced the individual caseworkers in 2024.

This is the next reassessment. It is the proximate operational reality for current AISH recipients in Alberta as this document is written. Sections 1.5 and 1.6 walk the family-level operational reality of the cohabitation rule and the partnership penalty as they will operate under the ADAP framework, which is to say, as they have operated under AISH and continue to operate.

The lifecycle observation

Each program along the cascade — WCB, AISH, ADAP, CPP-D, CPP retirement, OAS, GIS, long-term care — has its own eligibility rules, its own benefit formula, its own appeal architecture, its own institutional culture, and its own pattern of capture and clawback. The rules are not coordinated across programs. The recipient navigates each one separately. The one consistent feature across all of them is that the recipient receives the minimum each program is structured to deliver, and the maximum extraction is performed on each transition between programs.

The recipient who survived the WCB cascade through the AISH application through the Bill 12 reassessment, and who arrives at ADAP on July 1, 2026 with reduced benefit and reduced rights, has eighteen years left until the senior system begins. The senior system's reassessment apparatus is the architecture Movement Two of this document begins to walk. Section 1.5 first walks what the cohabitation rule does to the recipient's family in the working-age years before that transition.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.5 — The Cohabitation Rule and the Family-Level Operational Reality

The cohabitation rule is the single most consequential operational feature of AISH for recipients who live with another adult. It is also the program feature most working-age Canadians do not know about, because it does not appear in public-facing program summaries and because it operates only after a recipient enters the program. A worker in the cascade traced through Sections 1.1 through 1.4 will encounter the cohabitation rule for the first time at the moment they apply, when the application form requires disclosure of all household members and their incomes. The recipient who succeeds at the application then encounters the rule in a different way: as the mechanism that structures their family life from that point forward.

What the rule says

Under AISH policy, the income of any adult sharing a household with the recipient may be assessed as part of the household income calculation and used to reduce the recipient's benefit. (*Source: Government of Alberta — AISH benefits and exemption rules; Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007; Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Sections 4.6 and 5.*) The policy applies regardless of the nature of the relationship. A married spouse, a common-law partner, a roommate sharing the rent, an adult child temporarily living at home, an elderly parent in a granny suite — all are treated under the same household-income rule. The policy contains some narrow exemptions, notably for paid live-in caregivers and certain documented support arrangements, but these are narrowly defined and require active disclosure and documentation. The default assumption is that any adult sharing the household contributes financially.

The exemption threshold — the amount of cohabiting-partner income that is excluded from the household calculation before reduction begins — is currently \$2,612 per month under AISH. (*Source: Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 3.2; Inclusion Alberta — ADAP Fact Sheet, December 2025.*) Under the post-July-2026 ADAP framework, the exemption is reduced to \$1,500 per month. Above the exemption, the recipient's benefit is reduced by a percentage of the additional household income, with the reduction reaching dollar-for-dollar at higher income levels.

What the rule does mathematically

The reduction structure is most clearly seen in worked examples drawn from published sources. The campaign's *AISH Evidentiary Report Series, April 2026* documents the following structure for current AISH and the post-July-2026 framework, modelled against publicly available program documentation in the absence of fully published rule tables. (*Source: AISH Evidentiary Report Series, Section 3.3; ADAP analysis by Petit, G., University of Calgary, cited in CBC News, August 21, 2025.*)

Under current AISH, the first \$1,072 of monthly employment income (for a single recipient) is fully exempt. Earnings between \$1,072 and \$2,144 per month are reduced at a 50% rate. Earnings above \$2,144 are reduced at 100%. Under the post-July-2026 AISH structure, the fully exempt amount drops to \$350 per month, and the reduction is modelled at 100% above that figure. Under the ADAP framework, the fully exempt amount is \$700 per month with reduction-rate structure expected to be similar in operational effect.

For a partnered recipient, the spouse-income exemption operates analogously but at the higher initial threshold described above. A spouse earning \$38,000 per year — three thousand one hundred and sixty-seven dollars per month — exceeds the current AISH partner-income exemption of two thousand six

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

hundred and twelve by five hundred and fifty-five dollars per month. Above the exemption, the recipient's AISH benefit is reduced at the rate prescribed by the regulation. By the time partner income reaches \$45,000 per year, the AISH benefit is reduced substantially. By the time partner income reaches \$60,000 per year, the AISH benefit is approaching the floor at which it is no longer worth applying for in the first place.

Under the post-July-2026 ADAP framework, the partner-income exemption drops from \$2,612 to \$1,500 per month. A spouse earning \$38,000 per year now exceeds the exemption by \$1,667 per month. The recipient's benefit is reduced from a lower starting point — \$1,740 rather than \$1,940 — by a larger amount, given the lower exemption. The same household income produces a substantially worse outcome under ADAP than it does under current AISH.

What the rule does to a working spouse

The mathematical structure produces a specific operational reality for the working spouse. A spouse who, prior to the recipient's disability, earned a market wage and was the family's secondary income earner now finds that:

- Working fewer hours reduces household income and is unsustainable, because the recipient's AISH benefit is already inadequate to cover household expenses.
- Working the same hours produces, under the post-July-2026 framework, a smaller net household income than was the case before the change, because the partner-income exemption has been reduced.
- Working more hours — taking a second job, accepting overtime, pursuing promotion — increases gross income but reduces the AISH benefit by a comparable amount, with the marginal household income gain approaching zero at moderate income levels.

This is the structural vise documented in Section 1.1. The spouse cannot work less without losing income the household needs. The spouse cannot work more without losing the AISH benefit at a rate that approaches dollar-for-dollar. The spouse is, in operational reality, financially trapped at a household income level the program rules permit, regardless of what the household actually needs.

The spouse is also, in most cases, the household's primary caregiver. The disabled partner's care needs, depending on the disability, may be substantial and continuous. The financial decision-maker, the household manager, the primary parent for any children in the household, and the person responsible for managing a permanent-crisis-stage marriage is the same spouse. The literature on caregiver burden under disability is consistent across jurisdictions and has been for decades. (*Source: Vitaliano, P.P., et al., "Is caregiving hazardous to one's physical health?" Psychological Bulletin, 2003; Schulz, R., Sherwood, P.R., "Physical and mental health effects of family caregiving," American Journal of Nursing, 2008; Statistics Canada — Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022.*) Caregiving is associated with increased rates of depression, anxiety, cardiovascular disease, immune dysfunction, and premature mortality in the caregiver. The longer caregiving continues, the more pronounced these effects.

The spouse in this position is not merely under financial pressure. They are under medical, psychological, social, and time-allocation pressure simultaneously. The literature on relationship dissolution under disability reflects this. Marriages in which one partner becomes permanently disabled have higher rates of separation and divorce than the general population, particularly when the disability is acquired in

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

mid-adulthood. (Source: Karraker, A., Latham, K., "In Sickness and in Health? Physical Illness as a Risk Factor for Marital Dissolution in Later Life," *Journal of Health and Social Behavior*, 2015; Yorgason, J.B., et al., "Health, disability, and marital quality," *Family Relations*, 2008.) The structural-economic pressure created by the AISH cohabitation rule operates in addition to the medical, psychological, and time-allocation pressures the literature already identifies.

What the rule does to non-romantic household arrangements

The cohabitation rule does not distinguish between romantic and non-romantic cohabitation. (Source: *Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 4.6.*) An AISH recipient who attempts to share a rental unit with a friend, an adult sibling, an aging parent, a co-parenting partner who is not a romantic partner, or a roommate found through a housing-share arrangement triggers a household-income assessment. The other adult's income is potentially counted against the recipient's benefit.

The operational consequence is that the most common cost-sharing strategy used by low-income people in unaffordable housing markets — pooling resources with another adult — is structurally unavailable to AISH recipients. In a province with no rent cap, in a market where average one-bedroom rent ranges from \$1,500 to \$1,900 per month and three-bedroom rent averages above \$2,138 per month, the recipient is required by the rule structure to maintain a single-occupant household at the AISH benefit level — which does not cover the cost of a single-occupant household.

The rule contains a documented exception for paid live-in caregivers under specific arrangements. The exception is narrow. It does not cover practical roommate arrangements. It does not cover co-parenting arrangements. It does not cover multi-generational household arrangements common in Indigenous, immigrant, and low-income communities. The exception covers a specific, formal, documented caregiving relationship and not the broad range of household-sharing arrangements that the rule otherwise reaches.

What the rule does to single parents

Single parents constitute approximately 6.8% of the AISH caseload — roughly 5,426 recipients as of September 2025, almost entirely women. (Source: *Government of Alberta — AISH Caseload Open Data, September 2025; Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 9.1.*) The cohabitation rule operates on this population in a particularly acute way. A single mother on AISH who attempts to share housing with another adult — a friend, a relative, a co-parent of any of her children, a partner — triggers household-income assessment. Her ability to share rent with another adult is structurally constrained. Her ability to enter into a stable adult relationship that progresses to cohabitation — which would, in most non-disabled circumstances, be a stabilizing factor for both her and her children — is structurally penalized.

The campaign's *AISH Evidentiary Report Series* identifies that the ADAP framework was "designed without a gender-disaggregated analysis of its impact." (Source: *AISH Evidentiary Report Series, Section 9.2.*) Programs that ignore gender and caregiving structure in their design consistently produce worse outcomes for women, and the cohabitation rule is one of the clearest examples of the pattern. The rule's effect on single mothers compounds with the post-July-2026 employment-exemption reduction, the housing-cost environment, and the limited availability of accessible community housing for families with children with complex needs.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

What the rule produces clinically

The structural pressure to live alone has clinical implications for a population whose qualifying conditions include depression, anxiety disorders, post-traumatic stress disorder, complex PTSD, borderline personality disorder, social anxiety disorder, and other mental-health conditions for which social isolation is a documented worsening factor. (Source: Holt-Lunstad, J., Smith, T.B., Layton, J.B., "Social Relationships and Mortality Risk: A Meta-analytic Review," *PLoS Medicine*, 2010; Cacioppo, J.T., Hawkley, L.C., "Social isolation and health, with an emphasis on underlying mechanisms," *Perspectives in Biology and Medicine*, 2003; Public Health Agency of Canada — "Social isolation of seniors," 2017; World Health Organization — *Social isolation and loneliness among older people*, 2021.)

The clinical literature is unambiguous. Social isolation is causally associated with worsened depression, increased anxiety, accelerated cognitive decline, increased cardiovascular disease, increased all-cause mortality, and reduced response to treatment for the underlying conditions that produce isolation as a symptom. A program rule that imposes social isolation as the financially safe default on a population whose qualifying conditions are themselves worsened by isolation is, in operational effect, a rule that worsens the conditions it is supposed to address.

The campaign's *AISH Evidentiary Report Series* documents that the cohabitation rule's clinical impact constitutes engagement of UN Convention on the Rights of Persons with Disabilities Article 19 — the right of persons with disabilities to live independently and be included in the community, with the explicit requirement that persons with disabilities have the opportunity to choose their living arrangements and not be obliged to live in a particular arrangement. (Source: *United Nations Convention on the Rights of Persons with Disabilities, Article 19; AISH Evidentiary Report Series, Section 4.6.*) Canada ratified the Convention in 2010. Alberta is bound by it. The cohabitation rule, by financially penalizing every shared living arrangement except solitude, does not give recipients the opportunity to choose. It removes the choice and replaces it with a structural incentive toward isolation.

The lifecycle observation

The cohabitation rule is the working-age expression of a partnership-penalty mechanism that operates across the lifecycle. The same mechanism, in a different program, in different language, with different exemption thresholds and different reduction rates, operates at age 65 through the Guaranteed Income Supplement. Movement Two of this document develops the GIS partnership penalty in detail, with the published Service Canada figures and the Service Canada Form ISP-3040 ("Statement — Spouses or Common-law Partners Living Apart for Reasons Beyond their Control"), which exists in the federal program structure for the same reason this Section identifies in the provincial program structure: because the penalty for partnership is large enough that the program needs an administrative mechanism to determine whether separation is "involuntary" enough to be rewarded.

The cage that this document names operates by the same principle at every stage of life. AISH penalizes cohabitation at the working-age stage. ADAP continues the penalty. Long-term care accommodation calculations apply spousal income tests. GIS reduces the couple-rate to a fraction of two single-rate equivalents. Each program operates the partnership penalty in its own form. None of them is coordinated with any of the others. The recipient navigates each one separately, with no acknowledgment from any of them that the cumulative effect of the same principle applied across forty years is a structural exclusion of the disabled and aging population from family life of any kind.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.6 closes Movement One by walking the worker — and the spouse, the children, the household — to age 65, where Movement Two takes over.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.6 — The Bridge to Sixty-Five

The worker traced through this Movement is now sixty-three years old. The accident, the long-running WCB claim, the AISH application, the AISH-to-ADAP transition, the marriage breakdown, the years on a fixed inadequate income, the cohabitation rule's structural pressure to remain alone — all of it is two decades behind. The worker's surviving social network is small. Their savings are exhausted. Their physical health has deteriorated under twenty years of poverty-line existence with limited access to non-insured medical services. Their mental health has been managed at whatever level the public system has been able to deliver. They have done what was asked of them at every stage. The system has done what its rules required at every stage. The two outcomes are not the same.

This Section closes Movement One by walking the final two years before the senior system begins. It is the shortest Section of Movement One because its function is bridging — to name what is approaching, to set up the program-by-program walk that Movement Two performs, and to make explicit the lifecycle observation that the document has been building toward since Section 1.1.

What turns over at sixty-five

At age 65, several things turn over at once for the worker on AISH or ADAP:

- AISH and ADAP eligibility ends. The provincial disability income program assumes the recipient transitions to federal seniors programs and ceases provincial payment. (*Source: Government of Alberta — AISH program rules; Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007; Government of Alberta — ADAP transition page, 2026.*) The recipient is no longer a disability program recipient. They are a senior.
- Old Age Security begins, if residency requirements are met. The current OAS rate (January–March 2026) for recipients aged 65–74 is \$742.31 per month. (*Source: Service Canada — Old Age Security Benefit Payment Rates, January–March 2026.*)
- Canada Pension Plan retirement begins, if not already begun, at the formula amount calculated against the recipient's lifetime contribution history. The recipient's contribution history was interrupted at the time of the disability — in the worker's case, age forty-two. Twenty-three years of zero or near-zero CPP contributions reduce the retirement-pension calculation substantially. The maximum CPP retirement pension for new beneficiaries beginning in January 2026 is \$1,507.65 per month. (*Source: Service Canada — CPP Maximum Benefit Amounts, January 2026.*) The average new CPP retirement pension at age 65 for new beneficiaries (April–June 2026) is \$925.35 per month. (*Source: Service Canada — CPP Statistics, April–June 2026.*) For a recipient whose contribution history was interrupted at age forty-two, the actual retirement-pension amount is materially below the average — typically in the range of \$300 to \$600 per month, depending on the contribution history before the disability and the operation of the disability dropout provision.
- Guaranteed Income Supplement eligibility begins, subject to income testing. For the worker arriving at sixty-five with no other income beyond OAS and a small CPP, GIS will be the largest single component of monthly income. The current single-recipient GIS maximum (January–March 2026) is \$1,109.85 per month at incomes below approximately \$22,440 per year. (*Source: Service Canada — Guaranteed Income Supplement Payment Rates, January–March 2026.*)

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- For the worker who continued to receive WCB Economic Loss Payments throughout the working years, the ELP retirement adjustment described in Section 1.2 takes effect. The wage-loss benefit is recalculated at 2% of cumulative wage-loss compensation paid from the date of accident to age 65, payable for the rest of the worker's life. The adjustment typically reduces monthly WCB income by seventy to 90%.

The transitions are not coordinated. The worker is responsible for applying for OAS, applying for CPP retirement, applying for GIS, and notifying Alberta Assisted Living and Social Services that they are aging out of AISH or ADAP. Each application has its own form, its own processing time, and its own potential for error or delay. There is no single transition office, no single transition application, and no single point of accountability if the transitions do not align. Recipients with cognitive disabilities, communication-related disabilities, or limited literacy navigate these transitions with limited support. Some do not complete the transitions on time and experience income gaps in the months around their sixty-fifth birthday.

What the worker actually receives at sixty-five

For the worker traced through this Movement — interrupted CPP contributions at age forty-two, full forty-year Canadian residency for OAS, no employer pension, no significant retirement savings — the monthly income at age 65 is approximately as follows:

- OAS (age 65–74): \$742.31
- CPP retirement (interrupted contributions): \$400 to \$600 (illustrative; actual amount depends on individual contribution history)
- GIS (income-tested at low income levels): typically \$800 to \$1,000 (the GIS amount declines as CPP and other income rise; for a recipient with \$500 of CPP, GIS is reduced from the maximum but remains the largest single component)

Total monthly income at sixty-five, for the worker on the unfavourable side of the cascade: approximately \$2,000 to \$2,300. The 2026 Canadian after-tax Market Basket Measure poverty threshold for a single person in Edmonton is approximately \$25,000 to \$27,000 per year. (*Source: Statistics Canada — Market Basket Measure thresholds, Canadian Income Survey, 2024 and 2025 reference periods, projected to 2026.*) The worker's senior income is at or marginally above the poverty line for a single person. It is below the threshold for a couple living together. If the worker reconciles with their former spouse — for emotional support, for shared housing, for caregiving in either direction — the GIS income test reduces both partners' GIS amounts and the household total declines, in some scenarios below where it would be if the partners lived separately.

This is the partnership penalty at the senior stage. It is the same mechanism the cohabitation rule operates at the working-age stage, in a different program, with different numbers, producing the same structural pressure. The worker who finally has the option to be in a household with another person again is structurally penalized for choosing it. Movement Two walks this in detail.

The cumulative observation

The worker has, by the age of sixty-five:

- Paid in lifetime contributions, taxes, and premiums in the range of \$700,000 (Section 1.1)

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Received, across the disability years, a fraction of those contributions back through WCB and AISH/ADAP, with the bulk of the contribution captured by the cap structures, the asset spend-down requirements, and the dollar-for-dollar offsets of CPP-D and CDB (Sections 1.1, 1.4)
- Lost the marriage and the household and most of the social network the working years built (Section 1.5)
- Lost the retirement savings, the home equity that could not be maintained, the vehicle, the investments (Sections 1.1, 1.3)
- Arrived at sixty-five with monthly income at the poverty floor and structural pressure to remain alone (this Section)

The structural pattern has been documented at every stage. The mechanisms are published. The numbers are public. The lifecycle of capture and exclusion that the worker has traced through this Movement is not a sequence of unfortunate individual decisions or of program failures at specific points. It is the operating reality of an architecture that was built, program by program, across decades, with each program designed to deliver the minimum and to capture the maximum, and with no part of the architecture coordinated against the cumulative experience of the recipient who passes through all of them.

What Movement One has named

This Movement has named the cascade for the working-age person whose body fails. It has walked the entry into WCB, the operational reality of WCB benefits and disputes and the age-65 cliff. It has walked the AISH application, the asset spend-down, the wait, the appeal that no longer exists. It has walked the AISH-to-ADAP transition as the next reassessment in a continuous reassessment architecture. It has walked the cohabitation rule and the family-level operational reality of the partnership penalty at the working-age stage. It has bridged to age 65 and named the senior system the worker is about to enter.

Movement One has been written for the working-age reader who did not believe this document was about them. It has been written so that reader can see the cascade in operational detail, in primary-source-grounded numbers, in the actual rules and programs that will be applied to them if their body fails or their work ends. The cascade is documented. The mechanisms are published. The reader who finishes Movement One has been given the cage, end to end, at the working-age stage.

Movement Two now walks the senior system the worker is about to enter. It documents Old Age Security, Canada Pension Plan, the Guaranteed Income Supplement, the Allowance, the Allowance for the Survivor, CPP Survivor Benefits, CPP credit splitting, CPP pension sharing, Alberta Seniors Benefit, Special Needs Assistance for Seniors, the Alberta Seniors Drug Benefit, and long-term care accommodation calculations. It walks the lifecycle reassessment thread that Movement One has begun to name. It walks the partnership penalty, stage by stage, with the published math. It walks the "what you put in versus what you get back" question, in eight layers, with the Service Canada figures and the Government of Alberta open data. It walks the gap inventory of what is and is not covered for seniors in this country.

The worker has reached sixty-five. The senior system begins.

The cage does not end. It changes shape.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Movement Two — The Architecture

Section 2.1 — Old Age Security: The Foundation, the Clawback, the Political Vulnerability

The Old Age Security pension is the foundation of the Canadian senior income system. It is the most universal of the senior programs, the most widely understood by the public, and the most widely misunderstood. Movement Two of this document begins with OAS because OAS is the single program that most Canadians believe they have an entitlement to in old age. The structural truth is that OAS is an entitlement only in the sense that the Government of Canada has chosen to make it one, that the Government has chosen otherwise in the past, and that the Government can choose otherwise again. This Section walks the operational reality of OAS — how it is funded, who qualifies, what they receive, how it is reduced, and what its political history reveals about the durability of the entitlement.

What OAS is and how it is funded

The Old Age Security pension is a monthly taxable payment available to most Canadians aged sixty-five and older. (Source: *Old Age Security Act, R.S.C. 1985, c. O-9*; *Service Canada — Old Age Security overview page, 2026*.) It is funded from the general tax revenues of the Government of Canada. It is not funded from individual contributions. There is no OAS payroll deduction during a worker's working years. There is no OAS account in a worker's name that grows over time. The OAS pension is paid by Parliament from the federal consolidated revenue fund in the year the pension is paid, on the same basis that Parliament pays for highways, military procurement, federal courts, and Indigenous services.

This funding structure is not a technical detail. It is the central operational fact about OAS that working-age Canadians most need to understand. The OAS pension cannot be claimed by a senior on the basis that they paid for it through their working life, because they did not. The OAS pension is paid because Parliament has chosen to pay it, in the amounts Parliament has chosen, to the recipients Parliament has determined eligible. Each of these decisions has been made differently at different times in Canadian history, and each can be made differently again.

Eligibility: residency, age, and the sponsorship exclusion

The Old Age Security Act establishes eligibility on three primary axes: age, residency, and (since 2025) sponsorship status.

The age threshold is sixty-five. (Source: *Old Age Security Act, R.S.C. 1985, c. O-9, section 3*.) Pension payments may begin the month after the recipient's sixty-fifth birthday. The recipient may defer the start of OAS up to age 70 in exchange for an increased monthly amount, discussed below.

The residency threshold has two tiers. To qualify for the full OAS pension, the applicant must have resided in Canada for at least forty years after the age of eighteen. To qualify for a partial OAS pension, the applicant must have resided in Canada for at least ten years after the age of eighteen. (Source: *Old Age Security Act, sections 3 and 4*; *Service Canada — Old Age Security eligibility page, 2026*.) The partial pension is calculated proportionally: a recipient who has lived in Canada for twenty years after age eighteen receives twenty-fortieths, or 50%, of the full OAS pension.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The sponsorship exclusion was implemented effective October 1, 2025. (Source: Service Canada — *Old Age Security: Eligibility, sponsorship clause, 2025*; Government of Canada — *Immigration, Refugees and Citizenship Canada policy update, 2025*.) Individuals under a sponsorship agreement are not eligible to receive the Guaranteed Income Supplement, the Allowance, or the Allowance for the Survivor for the duration of their sponsorship period. Under family-class sponsorship of parents and grandparents, that period is twenty years. The sponsored senior is, in operational effect, dependent on the sponsor for the duration of the sponsorship. The exclusion does not affect the basic OAS pension itself, where residency requirements are met, but it removes the income-tested top-up that would otherwise lift the recipient from the OAS-only floor to a more livable income. The 2025 sponsorship-rule change expanded the categories of sponsored persons excluded from these benefits.

Current OAS amounts

The OAS pension has, since July 2022, been paid at two different rates: a base rate for recipients aged sixty-five through seventy-four, and a higher rate for recipients aged 75 and over. The differential was introduced under the Liberal government's *Bill C-12, An Act to amend the Old Age Security Act*, with the stated purpose of providing additional support to older seniors whose savings were more likely to be depleted. (Source: *Bill C-12, An Act to amend the Old Age Security Act (Guaranteed Income Supplement)*, S.C. 2022, c. 5; Department of Finance Canada — *Budget 2021 — Strengthening Old Age Security, 2021*.)

For the period January through March 2026, the maximum OAS pension was \$742.31 per month for recipients aged 65 to 74, and \$816.54 per month for recipients aged 75 and over. (Source: Service Canada — *Old Age Security Benefit Payment Rates, January–March 2026*.) For the period April through June 2026, the maximum was adjusted to \$743.05 per month (65 to 74) and \$817.36 per month (75 and over). (Source: Service Canada — *Maximum Benefit Amounts and Related Figures – Old Age Security, April–June 2026*.) OAS amounts are reviewed quarterly and adjusted for cost of living using the Consumer Price Index. The annualized maximum at the April–June 2026 rate is approximately \$8,917 per year for ages 65 to 74 and approximately \$9,808 per year for ages 75 and over.

These amounts are not, by any standard Canadian measure, sufficient to live on alone. The 2026 Market Basket Measure poverty threshold for a single person in a major Canadian metropolitan area is approximately \$25,000 to \$27,000 per year. (Source: Statistics Canada — *Market Basket Measure thresholds, Canadian Income Survey, 2024 and 2025 reference periods*.) OAS alone, at the maximum rate, provides approximately one-third of the income required to reach the poverty line for a single senior in a Canadian city. The Guaranteed Income Supplement is the program designed to address the gap, for those whose income is low enough to qualify; the GIS is documented in Section 2.4.

The deferral option and its math

A recipient may defer the start of OAS up to age 70. (Source: *Old Age Security Act, R.S.C. 1985, c. O-9, section 7.1*; Service Canada — *When to start your retirement pension page, 2026*.) Each month of deferral past age 65 increases the monthly OAS amount by 0.6% — equivalent to 7.2% per year of deferral, or 36% total at age 70 compared to age 65. There is no further increase for deferral past age 70. A recipient who has not applied by age 70 is not retroactively credited with deferral past that point.

The deferral option is, in policy intent, a mechanism that allows higher-income seniors to optimize their retirement income by delaying OAS until they are past the recovery-tax threshold or past their

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

highest-earning years. In operational reality, the deferral option produces a higher long-term income stream for recipients with the financial capacity to wait, and produces nothing for recipients without that capacity. A senior without other significant retirement income cannot defer OAS because they need it now. A senior with significant retirement income — who is least in need of the OAS lift — can defer to optimize their lifetime benefit. The deferral structure rewards those who least need it. This is consistent with the structural pattern documented elsewhere in this document.

A senior who defers and dies before reaching the breakeven age — typically around age eighty, depending on inflation assumptions and the recipient's specific circumstances — receives less in lifetime OAS than they would have received by starting at sixty-five. The deferral decision is functionally a longevity bet. Recipients with shorter life expectancies, including recipients with disabilities, recipients in lower-income communities, Indigenous recipients, and racialized recipients whose life expectancies are statistically lower than the Canadian average, are structurally penalized by the deferral option being framed as a financial-optimization tool. The optimization framing assumes a longer life expectancy than these populations have.

The recovery tax (clawback)

OAS is income-tested at the upper end through the OAS recovery tax, commonly referred to as the OAS clawback. (Source: *Old Age Security Act, sections 33-37*; *Service Canada — Old Age Security pension recovery tax, 2026*.) For recipients whose net income exceeds a threshold set annually, 15 cents of each additional dollar of income is repaid through the recovery tax. The recovery tax continues to apply to additional income until the recipient's OAS pension is fully clawed back at the upper threshold.

For the period July 2026 through June 2027, the recovery-tax thresholds are as follows. The clawback begins at net income of \$93,454 (based on 2025 net world income for that recipient). The clawback fully eliminates OAS at net income of approximately \$151,668 for recipients aged 65 to 74, and at approximately \$160,696 for recipients aged 75 and over. (Source: *Service Canada — Maximum Benefit Amounts and Related Figures – Old Age Security, April–June 2026*; *Wealthsimple — OAS Clawback Explained, January 2026*.) These figures are adjusted annually for inflation.

The recovery-tax mechanism is, in policy intent, a means-test on the upper end of OAS eligibility. In operational reality, it produces several structural outcomes that are often surprising to the recipient encountering them for the first time:

- A senior with a modest workplace pension, a modest CPP retirement pension, and modest investment income can find themselves above the recovery-tax threshold without considering themselves wealthy. Net income of \$93,454 is well below the income at which most Canadians would self-identify as having significant resources.
- The recovery tax is calculated against net income, which includes most retirement income but not Tax-Free Savings Account withdrawals. Seniors who saved primarily through TFSAs face less clawback than seniors who saved primarily through Registered Retirement Savings Plans, even at identical effective income levels. The structure rewards savers who used the right vehicles.
- The recovery tax does not differentiate between earned income, pension income, investment income, and other income types. A senior who continues to work past sixty-five — for income, for purpose, for engagement — has their OAS clawed back at the same rate as a senior whose income is from passive

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

investments.

- The recovery tax is calculated on individual income, not household income. Two seniors each with \$90,000 of income face no recovery tax. One senior with \$180,000 of income (the same combined household amount) faces full clawback. Income splitting between spouses is therefore a significant tax-planning consideration for senior couples — a consideration that requires either a workplace pension that permits splitting, or proactive use of spousal RRSPPs during the working years, or sophisticated retirement-income planning that most seniors do not have access to.

What OAS does not include

OAS does not include a survivor benefit. (Source: *Old Age Security Act*; *ATB Financial — When should I start collecting OAS? 2026*.) When an OAS recipient dies, the OAS pension ends. The surviving spouse does not inherit any portion of the deceased spouse's OAS. The only OAS-related provision for survivors is the Allowance for the Survivor, available to surviving spouses or common-law partners aged 60 to 64 with annual income below approximately \$30,312 (2026 figure). (Source: *ATB Financial — OAS Allowance and GIS, 2026*; *Service Canada — Allowance for the Survivor page, 2026*.) The Allowance for the Survivor ends the month following the survivor's sixty-fifth birthday, at which point the survivor's own OAS, GIS, and CPP retirement (if any) become the income.

This design choice — the absence of a survivor benefit in OAS — has substantial impact on the income of surviving spouses, particularly women, who statistically outlive their male partners. A senior couple receiving two OAS pensions plus one or two CPP retirement pensions plus, where applicable, GIS, sees a substantial drop in household income at the death of one partner. The surviving partner continues to face the full cost of housing, utilities, food, transportation, and care, but with one OAS gone, often with the larger CPP gone (if the deceased partner was the higher earner), and with a different GIS calculation that may or may not produce more income depending on circumstances. The surviving partner's income drop at widowhood is documented across senior-poverty literature as a major driver of senior-women's poverty in Canada. (Source: *Statistics Canada — Income of Senior Women in Canada, 2017 and 2022 reference periods*; *Public Health Agency of Canada — Health Inequalities in Canada, women and aging report, 2023*.)

The political vulnerability of OAS

OAS has been amended repeatedly across successive federal governments. The amendments have not been neutral, and the political history is consequential for the working-age reader who assumes OAS will be paid at the current rate, on the current eligibility terms, when they reach sixty-five.

In 2012, the federal Conservative government under Prime Minister Stephen Harper announced changes to OAS that would gradually increase the age of eligibility from sixty-five to sixty-seven, phased in between 2023 and 2029. (Source: *Government of Canada — Budget 2012, Old Age Security changes*; *Library of Parliament — Bill C-38, Jobs, Growth and Long-Term Prosperity Act, 2012*.) The change was passed as part of an omnibus budget bill. It was justified on the basis of demographic sustainability, pointing to the projected increase in the OAS-eligible population as the baby-boom cohort reached retirement age.

In 2016, the federal Liberal government under Prime Minister Justin Trudeau reversed the age-of-eligibility increase, restoring the eligibility age to sixty-five. (Source: *Government of Canada — Budget 2016 — A New Chapter for OAS*; *Library of Parliament — Bill C-15, Budget Implementation Act, 2016, No. 1*.) The reversal was justified on the basis that the previous government's analysis had not accounted adequately

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

for the impact on low-income seniors, who are most reliant on OAS and least able to delay retirement.

The same program — the same statute — has been amended in opposite directions by successive federal governments within a four-year window. The OAS eligibility age that a Canadian forty-five years old today plans for is the eligibility age the current government has chosen. The eligibility age for that same Canadian when they reach sixty-five depends on what the government in power at that time chooses, on what subsequent governments choose, on the budget and demographic context at each decision point, and on the political pressure that low-income seniors and their advocates can bring at each decision point.

The OAS recovery-tax thresholds have been adjusted repeatedly. The 75-and-over differential was introduced in 2022 and could be modified or removed by future legislation. The residency requirements have been the subject of repeated review. The funding source — general revenue — means that OAS is in direct competition with every other federal expenditure for the available fiscal capacity. There is no dedicated OAS reserve, no OAS-specific funding stream, no protection from being reduced or restructured if a future government chooses to do so.

The lifecycle observation

OAS, more than any other program in this Movement, depends on the political choices of the government in power. The worker arriving at sixty-five in 2046 — twenty years from the date of this document — will receive the OAS that the federal government of 2046 has chosen to provide, on the eligibility terms and at the indexing structure that the federal government of 2046 has chosen. Nothing about the current OAS program is structurally guaranteed for that worker. The program is statutory. The statute can be changed.

This is the structural fragility of OAS. It is, in current operational reality, the most universal and most reliable component of the senior income system. It is also the most exposed to political change. The worker who has built their retirement assumptions around the current OAS structure is making an assumption about the political durability of a program that has been changed twice within the last decade.

Section 2.2 walks the Canada Pension Plan — the program that, unlike OAS, is contributory. The contribution structure of CPP introduces a different set of problems, documented next.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.2 — The Canada Pension Plan: Contributions, Formula, the Average vs the Maximum

The Canada Pension Plan is the contributory program. Unlike Old Age Security, which is paid from general revenue regardless of work history, CPP is funded by mandatory payroll contributions deducted from every working Canadian's pay (and from a corresponding employer match) over their entire working life. Most Canadians know they pay into CPP. Most Canadians believe, on the basis of having paid in, that they are entitled to receive back what they put in. This section walks the operational reality of CPP — what the contribution structure is, how the benefit is calculated, what the average recipient actually receives, and why the gap between contribution and receipt is the single most important fact about CPP that working-age Canadians have not been told plainly.

What CPP is

The Canada Pension Plan is a compulsory contributory social insurance program established by federal legislation in 1965, operating in every Canadian province and territory except Quebec, where the parallel Quebec Pension Plan (QPP) provides comparable benefits. (Source: *Canada Pension Plan Act, R.S.C. 1985, c. C-8*; *Quebec Pension Plan Act, CQLR c R-9*.) CPP is administered by Employment and Social Development Canada and the Canada Revenue Agency, with investment management of the contributory fund handled by the Canada Pension Plan Investment Board (CPPIB).

The CPPIB manages contributions on behalf of all current and future beneficiaries collectively. There is no individual CPP account. There is no record at the CPPIB of "your" contributions or "your" balance. The contributions of every Canadian payer are pooled into a single fund. The fund pays the benefits of every Canadian recipient, on the basis of a benefit formula established by statute. The formula determines what each recipient receives. The contributions and the benefits are linked through the formula, but they are not linked through any individual account structure.

This is the central operational fact about CPP that working-age Canadians most need to understand, and the one most consistently misrepresented in popular discussion of the program. CPP contributions are not savings. They are mandatory payroll deductions that fund a defined-benefit social insurance program, returned to the contributor — if the contributor lives long enough to collect — through a formula the contributor does not control. The contributor does not own the contributions. The contributor does not have a claim on the contributions. The contributor has a claim on the formula, and the formula is what it is.

The contribution structure

CPP contributions are calculated as a percentage of pensionable earnings between the basic exemption and the Year's Maximum Pensionable Earnings (YMPE) ceiling. (Source: *Canada Revenue Agency — CPP contribution rates, maximums and exemptions, 2026*; *Service Canada — Canada Pension Plan: Contributions and Earnings, 2026*.) The basic exemption — the amount of earnings on which no CPP is collected — is \$3,500 per year. The 2026 YMPE is \$74,600. The employee contribution rate on earnings between the exemption and the YMPE is 5.95%. The employer matches at the same rate. Self-employed Canadians pay both halves, totalling 11.9% on the relevant earnings band.

A worker earning at or above the 2026 YMPE pays the maximum employee CPP contribution of approximately \$4,421 per year (calculated against pensionable earnings of \$71,100, the YMPE less the

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

basic exemption, at 5.95%), with the employer matching. Total employee-plus-employer contribution at maximum: approximately \$8,842 per year. (Source: *Canada Revenue Agency — CPP contribution rates and maximums, 2026*.)

Since 2024, an enhanced second-tier contribution (CPP2) has applied to earnings between the YMPE and a Year's Additional Maximum Pensionable Earnings (YAMPE). For 2026, the YAMPE is \$85,000. The CPP2 rate on earnings in this band is 4% (employee), matched by the employer. (Source: *Canada Revenue Agency — CPP enhancement and CPP2, 2026*.) The CPP2 enhancement was introduced under federal legislation in 2019 with the stated purpose of increasing the income-replacement rate of CPP from 25% of pre-retirement earnings to 33% of pre-retirement earnings, phased in over decades.

Above the YAMPE, no CPP is collected on additional earnings. The income above \$85,000 per year (in 2026) is not insurable for CPP purposes. The high-earning worker pays CPP on the first eighty-five thousand and pays nothing further. The CPP benefit they will receive at retirement is calculated against the contributions they made within the YAMPE cap, not against their actual earnings.

What the worker pays over a career

A worker who earned at or above the YMPE for forty years from age twenty-five to sixty-five — that is, a worker who maxed out CPP every year of a forty-year working life — paid into CPP, in current-year-equivalent dollars, in the range of \$150,000 to \$170,000 across the career (employee portion only; employer match is approximately the same again). The exact figure depends on contribution-rate adjustments over time, the YMPE's growth across the career, and inflation indexing.

A worker who earned consistently below the YMPE — for example, at the median Canadian individual income, around fifty-\$5,000 per year — paid in proportionally less. A worker with intermittent earnings, parental leaves, periods of unemployment, periods of caregiving, or periods of disability paid in less again, depending on the shape of the interruption and the dropout provisions described below.

What the worker receives at retirement depends on the formula, not directly on the contribution amount.

The benefit formula

The CPP retirement benefit is calculated by averaging the worker's pensionable earnings over the contributory period (from age eighteen to the year of CPP application or age 65, whichever is earlier), expressed as a percentage of the YMPE in each year, and applied to the formula's replacement-rate parameter. (Source: *Canada Pension Plan Act, R.S.C. 1985, c. C-8, sections 44-50; Service Canada — How much CPP could you receive, 2026*.) The replacement-rate parameter is 25% for the original CPP base benefit and an additional 8% (phased in) for the post-2019 CPP enhancement, eventually reaching 33% for workers who pay enhanced CPP for full careers.

The formula includes drop-out provisions that exclude the worker's lowest-earning years from the calculation. The general dropout provision excludes up to 17% of the worker's lowest-earning months (a maximum of approximately ninety-six months, or eight years). The child-rearing dropout excludes years in which the worker had primary responsibility for a child under age seven and had reduced earnings as a result. The disability dropout excludes months in which the worker received CPP-D benefits. (Source: *Service Canada — How much you could receive, dropout provisions, 2026*.) The dropout provisions protect the worker's benefit calculation from being dragged down by years of zero or near-zero income that were

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

the result of life events the worker did not control.

The dropout provisions protect partially. They do not eliminate the impact of long-term contribution interruption. A worker who became disabled at age forty-two and stopped working entirely has twenty-three years of zero or near-zero earnings before reaching age 65. Even with the disability dropout (which excludes the months on CPP-D specifically) and the general dropout (which excludes the lowest-earning eight years), a worker with twenty-three years of interruption sees substantial benefit reduction relative to a worker who contributed continuously for forty years.

What the recipient actually receives

The maximum new CPP retirement pension at age 65 for benefits beginning in January 2026 is \$1,507.65 per month. (*Source: Service Canada — Maximum Benefit Amounts and Related Figures – Canada Pension Plan, January 2026.*) That is approximately eighteen thousand ninety-two dollars per year.

The average new CPP retirement pension at age 65 for new beneficiaries during the period April through June 2026 was \$925.35 per month. (*Source: Service Canada — CPP retirement pension page, statistics for new beneficiaries, April–June 2026.*) That is approximately eleven thousand one hundred and four dollars per year — roughly 61% of the maximum.

Put differently: the average new CPP recipient in 2026 receives about three-fifths of what the maximum recipient receives. The gap is not because the average recipient was negligent or unlucky. The gap is because the formula favours workers who maxed out CPP contributions every year of a forty-year career. Most workers do not. Most workers had years of below-YMPE earnings, years of unemployment, years of caregiving, years of part-time work, years of self-employment with intermittent income, years of disability — every one of which reduces the lifetime contribution and therefore the benefit calculation.

The maximum is the published headline. The average is what most recipients actually receive.

For a worker traced through Movement One — disability at age forty-two, twenty-three years of zero or near-zero earnings before reaching sixty-five — the actual CPP retirement pension is below the average. The exact figure depends on the worker's contribution history before age forty-two and the operation of the disability dropout. For the worker who contributed at the YMPE for the seventeen years from age twenty-five to age forty-two, with full disability dropout for the years on CPP-D and the eight-year general dropout, the resulting CPP retirement pension is approximately \$350 to \$600 per month at age 65. This is the figure used in Section 1.6's calculation of the worker's senior income.

What the high earner does not get

The flip side of the average-versus-maximum gap is what the high earner does not get back. A worker who earned \$150,000 per year for twenty-five years paid the maximum CPP contribution every year. Their contributions over those twenty-five years totaled approximately \$95,000 to \$110,000 (employee portion). They paid full federal and provincial income tax on the additional \$65,000 per year above the YMPE — generating, depending on tax bracket and province, approximately \$20,000 per year in additional income tax that funded general government revenue rather than CPP. Total above-YMPE income tax over twenty-five years: approximately \$500,000.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Their CPP retirement pension at age 65 is calculated against the YMPE-capped earnings, not against the actual earnings. The pension is approximately the same as for any other worker who maxed out CPP for twenty-five years — perhaps slightly less than the maximum, because they did not contribute for the full forty-year reference period required for the maximum.

The \$500,000 in above-YMPE income tax produces no CPP entitlement. The ninety-five to \$110,000 dollars in CPP contribution produces approximately twelve hundred to \$1,500 per month in retirement income. Calculated as a return on contribution, this is approximately what a defined-benefit pension would produce — except that, unlike a defined-benefit workplace pension, CPP is not vested in the contributor. It is the formula benefit, paid as long as the worker lives. If the worker dies the year after starting CPP, the benefit ends (with a small survivor pension to a spouse, calculated against a separate formula). If the worker lives to ninety, the benefit continues.

The longevity bet built into CPP retirement is the same longevity bet built into OAS deferral. Both reward longer lives. Both penalize shorter lives. Populations with shorter life expectancies — Indigenous Canadians, lower-income Canadians, racialized Canadians, Canadians with disabilities — are statistically penalized by the lifetime-payment structure. The contributor with the shorter life expectancy receives less of their contribution back.

The CPP-D-to-CPP-retirement conversion at sixty-five

A recipient of CPP Disability — the contributory disability benefit administered by Service Canada — has CPP-D converted to CPP retirement at age 65. (*Source: Service Canada — Canada Pension Plan disability benefit, 2026.*) The conversion is automatic. The recipient does not apply. CPP-D ends and CPP retirement begins on the recipient's sixty-fifth birthday.

The conversion is consequential in two ways. First, the CPP retirement pension after conversion is typically less than the CPP-D pension was. CPP-D includes a flat-rate component above the contributory portion, and the flat-rate component does not carry into the retirement pension. The recipient's monthly income from this stream drops at age 65. Second, the conversion does not eliminate the dollar-for-dollar AISH offset described elsewhere in this document. For a recipient who was on AISH at the working-age stage with CPP-D being clawed back to zero net benefit, the transition to CPP retirement at sixty-five produces a similar or worse offset experience under the senior system if the recipient is also receiving GIS — discussed in Section 2.4. The federal CPP retirement income reduces the GIS amount, just as the federal CPP-D income reduced the AISH amount, on a per-dollar basis defined by each program's clawback structure.

The lifetime CPP contribution made by the worker who became disabled at age forty-two is therefore captured twice: once at the working-age stage by the AISH/ADAP offset, and again at the senior stage by the GIS reduction. The contribution funds federal benefits that are, in operational reality, redirected away from the contributor at every life stage at which they would otherwise have provided meaningful additional income.

The lifecycle observation

CPP is the program working-age Canadians most associate with "what I paid in." It is the program in which the gap between contribution and receipt is the most consequential and the most concealed. The maximum that public discussion focuses on is received by approximately the top decile of recipients. The average —

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

what most recipients actually receive — is approximately 60% of that maximum. For workers with disability-interrupted contribution histories, the actual amount is below the average. For workers with cohabitation-rule, AISH-offset, and GIS-clawback exposure, the federal CPP contribution is captured by provincial programs at multiple life stages.

The "what you put in" question, posed by a working-age reader who has been paying CPP every year of their working life, has a structural answer. The amount you put in does not determine the amount you get back. The formula determines it. The formula is capped, the average is well below the maximum, and provincial programs reduce the federal benefit for the populations whose need is greatest. CPP is a tax-funded social insurance program. It is not a savings account. The insurance pays out on a basis the insured does not control.

Section 2.3 walks CPP Disability — the contributory benefit for working-age Canadians whose disability ends their employment, and the program through which the working-age stage of the cage most directly captures the federal CPP contribution.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.3 — CPP Disability: The Contribution That Was Captured

The Canada Pension Plan Disability benefit is the federal contributory benefit for working-age Canadians whose severe and prolonged disability ends their ability to work. It is the program that, more than any other in this document, demonstrates the structural pattern of federal-contribution capture by provincial programs. This Section walks CPP-D — what it is, who qualifies, what it pays, and what happens to the benefit when the recipient is also receiving provincial disability income assistance.

What CPP-D is

CPP Disability is a monthly federal benefit paid to contributors under age 65 who have a severe and prolonged disability that prevents them from working at any substantially gainful occupation. (Source: *Canada Pension Plan Act, R.S.C. 1985, c. C-8, sections 42-44; Service Canada — Canada Pension Plan disability benefit, 2026*.) The benefit consists of a flat-rate component plus a contributory component calculated against the recipient's CPP contribution history. The contributory component varies by individual contribution history; the flat-rate component is uniform across recipients.

For 2026, the maximum CPP-D benefit is approximately one thousand seven hundred and fifty-six dollars per month for a recipient with the maximum contributory history and meeting all eligibility criteria. (Source: *Service Canada — CPP Maximum Benefit Amounts, 2026; CPP Fact Sheet #4, April 2026*.) The average CPP-D benefit for new beneficiaries is substantially lower, in the range of one thousand one hundred to one thousand \$300 per month, depending on the contribution profile of the new-beneficiary cohort in the relevant quarter.

A CPP-D recipient may also receive a CPP Children's Benefit for each dependent child under eighteen (or under twenty-five if in full-time post-secondary). The 2026 Children's Benefit is approximately three hundred and one dollars per month per child. The Children's Benefit is paid through the recipient and is intended to support the costs of dependent children.

Who qualifies

To qualify for CPP-D, an applicant must meet both medical and contribution criteria. (Source: *Canada Pension Plan Act, sections 42 and 44; Service Canada — Canada Pension Plan disability benefit eligibility, 2026*.)

The medical criteria require that the applicant have a "severe and prolonged" disability. "Severe" means the applicant cannot work regularly at any substantially gainful occupation. "Prolonged" means the disability is long-term, of indefinite duration, or likely to result in death. Both criteria must be met. The Service Canada adjudication process applies these criteria with substantial discretion. The denial rate for first-application CPP-D claims has historically been in the range of fifty to 60% across the program's history. (Source: *Office of the Auditor General of Canada — Canada Pension Plan Disability program audits, multiple years; Council of Canadians with Disabilities — CPP-D Reform Submissions, 2018-2024*.)

The contribution criteria require that the applicant have made valid CPP contributions in four of the last six calendar years (the "contribution requirement"). Some flexibility exists for late applicants and for applicants with longer contribution histories, but the four-of-six rule is the standard test. (Source: *Service Canada — CPP disability eligibility, 2026*.) The contribution requirement is the rule that excludes many people from CPP-D — including workers with intermittent earnings, parents who took extended leave for caregiving,

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

gig-economy workers with sporadic CPP-pensionable income, and workers whose disability emerged gradually over years during which their work declined before formally ending.

A worker who became disabled at age forty-two — the worker traced through Movement One — typically meets the contribution requirement, having worked steadily through the years before the disability. A worker who became disabled at age fifty-five after a decade of part-time, irregular, or self-employment income may not meet it. A parent who took five years of full-time childcare leave around the time their disability emerged may not meet it. The contribution requirement is structural. It does not consider the reasons for the contribution gap.

The application and the wait

The CPP-D application process requires medical documentation, contribution history confirmation, and personal narrative of the disability and its impact on work capacity. The application is reviewed by Service Canada adjudication staff, with medical input from internal physicians. (*Source: Service Canada — How to apply for CPP disability benefits, 2026.*) Initial decisions can take several months. Reconsideration of denied applications can take additional months. Appeals to the Social Security Tribunal can take more than a year.

For applicants who succeed, CPP-D is paid retroactively to the determined date of disability onset (subject to a maximum retroactivity period of fifteen months for most applicants). For applicants who are denied at the initial decision and pursue reconsideration and appeal, the multi-year process produces no income during the wait. Many applicants who would qualify on the merits do not survive the application process financially. They settle for less, withdraw the application, or accept provincial disability assistance at a lower benefit level rather than pursuing CPP-D through to the end of the appeals process.

The relationship to AISH

The structural significance of CPP-D, for the population traced through Movement One of this document, is its interaction with the provincial AISH program. Most working-age Albertans whose disability has ended their employment apply for both CPP-D (federal) and AISH (provincial), because the eligibility criteria for the two programs are non-identical and an applicant may qualify for one and not the other, or for both with different effective dates.

When an applicant qualifies for both, the operational reality is that AISH treats CPP-D as income and reduces the AISH benefit dollar-for-dollar by the amount of CPP-D received. (*Source: Government of Alberta — AISH benefits and exemption rules; Inclusion Alberta — AISH/CPP-D Briefing, 2025; Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 7.*) An AISH recipient who qualifies for CPP-D at one thousand \$100 per month sees their AISH benefit reduced by one thousand \$100 per month. Their total monthly income is identical to the AISH-only amount. The CPP-D contribution they made through their working years produces no net income increase.

As of September 2025, 16,161 Albertans on AISH were receiving CPP-D. (*Source: Government of Alberta — Assisted Living and Social Services Open Data, AISH Caseload September 2025.*) That is 20.4% of the AISH caseload — more than one in five recipients. Every one of these recipients made CPP contributions during their working years. Every one of these recipients qualified for CPP-D on the basis of those contributions and the medical eligibility test. Every one of these recipients receives zero net benefit from

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

those contributions because of the provincial offset.

The structural pattern is clearest when stated this way: a person who paid into CPP for fifteen years before becoming disabled receives the same total monthly income as a person who never worked a day in their life and never made a CPP contribution. The fifteen years of contributions produce no advantage in the income the recipient actually receives. The provincial system captures the federal payment in full.

Why this is not reformed

The CPP-D-to-AISH offset has been documented and challenged for decades. Disability advocacy organizations have called for its reform repeatedly. Inclusion Alberta, the Council of Canadians with Disabilities, and the Canadian Centre for Policy Alternatives have produced analyses identifying the offset as inconsistent with the purpose of CPP-D. (Source: *Inclusion Alberta — Canada Disability Benefit in Alberta Fact Sheet, July 2025*; *Council of Canadians with Disabilities — CPP-D Reform Submissions, multiple years*; *Canadian Centre for Policy Alternatives — Income Security for People with Disabilities, 2022*.) The federal government's analysis during the development of the Canada Disability Benefit explicitly identified provincial offsets as a structural problem that would undermine the CDB's intended impact.

The offset has not been reformed because, in operational fiscal terms, it benefits the province at the expense of the federal government and at no cost to the province's payments to the recipient. The federal CPP-D payment arrives in the recipient's account. The provincial AISH cheque is reduced by the equivalent amount. The province's net outlay decreases by the amount of the federal payment. The federal government has paid CPP-D as if to support the recipient; the province has captured that payment as if to support its own balance sheet. The recipient is the unaffected party in the provincial balance, but the provincial balance has improved because of the federal contribution.

This is what the campaign's documents call "fiscal-transfer capture." It is not unique to CPP-D. The same pattern operates for the federal Canada Disability Benefit, which Alberta also claws back dollar-for-dollar. (Source: *Inclusion Alberta — Canada Disability Benefit in Alberta Fact Sheet, July 2025*; Petit, G., "The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis," *Canadian Tax Observatory, April 22, 2026*.) Alberta is the only province in Canada that applies the dollar-for-dollar CDB clawback. Other provinces apply partial offsets or no offset. The Government of Alberta has not, in any public statement, committed to ending the CPP-D offset under the post-July-2026 ADAP framework.

The federal government has not, in any public statement, committed to conditioning federal disability transfer payments on non-clawback compliance. The campaign's federal submission to the House of Commons Standing Committee on Human Resources, Skills and Social Development and the Status of Persons with Disabilities (HUMA) in April 2026 calls for the federal government to do exactly that. The submission argues that federal transfer payments designed to lift disabled Canadians above the poverty line should not be capturable by provincial program structures.

The conversion at sixty-five

CPP-D ends on the recipient's sixty-fifth birthday and is replaced by CPP retirement. (Source: *Service Canada — Canada Pension Plan disability benefit, conversion at retirement, 2026*.) The conversion is automatic. The recipient does not apply.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The conversion typically reduces the monthly amount. CPP-D includes a flat-rate component above the contributory portion; the flat-rate component does not carry into CPP retirement. CPP retirement is calculated against the full contributory record, with the disability dropout provision excluding the months on CPP-D from the calculation. The result, for most recipients, is a CPP retirement pension lower than the CPP-D pension was — typically several hundred dollars per month lower, depending on the recipient's working-years contribution history.

For the recipient who was on AISH throughout their working-age disability years, the conversion at sixty-five coincides with the end of AISH eligibility (which terminates at sixty-five for all recipients). The recipient transitions to CPP retirement plus OAS plus, where eligible, GIS. The CPP-D dollar-for-dollar offset that AISH applied no longer operates, because AISH no longer pays the recipient. But the GIS calculation, documented in Section 2.4, applies its own reduction structure. Each dollar of CPP retirement income reduces the GIS amount, depending on the recipient's marital status and total income, at a rate that approaches but does not always reach the dollar-for-dollar offset of the AISH-stage clawback. The federal CPP contribution is captured at a different rate by a different program. The capture continues.

The lifecycle observation

CPP-D is the working-age expression of the same federal-contribution-capture pattern that operates at the senior stage through GIS. The contributor who paid into CPP through their working years sees the federal disability benefit captured by the provincial AISH program, then sees the federal retirement benefit reduced by the federal GIS structure. At every stage, the contribution is in the system. At every stage, the recipient does not receive its full value.

This is the structural answer to the "what did I put in for?" question, posed at the working-age stage by an AISH recipient on CPP-D, and posed at the senior stage by a CPP retirement pensioner on GIS. The answer in both cases is the same. The contribution funded the program. The program operated on the contribution. The benefit was paid. The benefit was captured by the next program in the cascade. The recipient's net income was determined by the floor of the lowest-paying program in the cascade, not by the contribution that funded the highest-paying.

Section 2.4 walks the Guaranteed Income Supplement, the Allowance, and the Allowance for the Survivor — the income-tested federal programs that produce the largest single dollar capture of CPP retirement income at the senior stage, and that contain the partnership penalty mechanism in its most fully developed form.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.4 — The Guaranteed Income Supplement, the Allowance, and the Partnership Penalty

The Guaranteed Income Supplement is the federal program designed to lift low-income seniors above the floor that OAS alone would leave them on. It is also the program through which the partnership penalty operates most clearly at the senior stage, and the program in which the campaign canon's central observation about the structural pressure to live alone receives its most complete documentary expression. This Section walks GIS, the related Allowance and Allowance for the Survivor benefits, the published 2026 dollar amounts, and the partnership penalty math that the federal benefit table produces.

What GIS is

The Guaranteed Income Supplement is a non-taxable monthly federal benefit paid to OAS recipients with low income. (*Source: Old Age Security Act, R.S.C. 1985, c. O-9, Part II; Service Canada — Guaranteed Income Supplement page, 2026.*) It is funded from the same general revenue source as OAS. It is income-tested: the amount of GIS payable is reduced as the recipient's other income increases. To qualify, the recipient must be sixty-five or older, must be receiving OAS, must be resident in Canada, and must have annual income (or combined annual income, for couples) below the applicable threshold for their marital and household situation.

GIS was introduced in 1967 — two years after the establishment of OAS — to address senior poverty among Canadians whose work history had not produced sufficient retirement income to live above the poverty line on OAS alone. (*Source: Library of Parliament — Old Age Security Act and Guaranteed Income Supplement: Background and History, 2018.*) It was designed as an anti-poverty program. It was set, deliberately, at a level near but generally below the Canadian poverty line, on the basis that a benefit set above the poverty line might disincentivize work or savings during the working years. The poverty-floor design choice has been documented in policy literature for decades and has been the subject of academic and advocacy critique consistently. (*Source: Institute for Research on Public Policy — "Unfinished Business: Pension Reform in Canada," 2018; National Council of Welfare — Welfare Incomes reports, multiple years; Caledon Institute of Social Policy — Income Security for Seniors, multiple papers.*)

Current GIS amounts

For the period January through March 2026, the maximum monthly GIS amount for a single, divorced, or widowed senior was \$1,109.85 per month, payable at annual income (excluding OAS) below approximately \$22,440. (*Source: Service Canada — Old Age Security Benefit Payment Rates, January–March 2026; Spring Financial — Guaranteed Income Supplement page, 2026.*)

For a senior who is married or in a common-law relationship and whose spouse also receives OAS, the maximum monthly GIS amount per spouse was \$668.08 per month, with combined annual income (excluding OAS for both partners) below approximately \$29,712. (*Source: Service Canada — Old Age Security Benefit Payment Rates, January–March 2026; ATB Financial — OAS Allowance and GIS for Low Income Seniors, 2026.*)

For a senior whose spouse does not receive OAS or the Allowance, the maximum monthly GIS was approximately \$1,409.72 per month, with combined annual income below approximately \$53,904. (*Source: ATB Financial — OAS Allowance and GIS, 2026 figures table.*)

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

For a senior whose spouse receives the Allowance (the spouse being aged 60 to 64), the maximum monthly GIS was \$667.41 per month, with combined annual income below approximately \$41,616.

These figures are adjusted quarterly for inflation.

What the partnership-penalty math actually shows

The single most consequential operational fact about GIS, for the central argument of this document, is the size of the partnership penalty built into the benefit table. The published Service Canada amounts produce the following comparison:

- Two single seniors, each at low income, each receiving the maximum single GIS, total combined GIS: \$2,219.70 per month. (Two times \$1,109.85.)
- The same two people, married or common-law, both receiving OAS, each at the same low income, each receiving the maximum couple GIS, total combined GIS: \$1,336.16 per month. (Two times \$668.08.)
- Difference: \$883.54 per month. Annualized: \$10,602.48 per year.

The same two people, with the same total income, in the same housing market, with the same expenses, receive \$883.54 per month less in GIS for being together than they would receive for being apart. The penalty is built into the benefit table. It is not a calculation requiring assumptions, modeling, or interpretation. It is the published Service Canada figure.

The justification for the difference, in policy intent, is that two people sharing a household have lower per-person living costs than two people maintaining separate households — economies of scale on rent, utilities, groceries, transportation, and other shared expenses. The justification has merit in the abstract. It does not have merit at the dollar amounts the benefit table actually produces. Two single seniors each receive enough OAS plus GIS to live just below or at the Market Basket Measure poverty threshold for a single person. The same two people coupled receive substantially less than the poverty threshold for a couple. The economies-of-scale justification, applied at the dollar level the program actually pays, produces a couple-floor that is below the cost of a couple's living, in current Canadian housing markets in particular.

Form ISP-3040 and what it admits

Service Canada's Form ISP-3040 is titled "Statement — Spouses or Common-law Partners Living Apart for Reasons Beyond their Control." (*Source: Service Canada — Form ISP-3040, "Statement – Spouses or Common-law Partners Living Apart for Reasons Beyond their Control," 2017 and current.*) The form's purpose is to allow GIS to be calculated on individual income rather than combined income when separation is involuntary — for example, when one spouse is in long-term care, in hospital, or otherwise unable to maintain the shared household. The form's text includes the phrase: GIS payments may be calculated on individual income rather than combined income "if it is to the advantage of the couple."

The existence of Form ISP-3040 is the Government of Canada acknowledging, in its own paperwork, that being apart yields more benefits than being together. The "for reasons beyond their control" qualifier is the policing mechanism. Voluntary separation for benefits is not supposed to qualify for the higher individual-income calculation. The form requires the couple to demonstrate the involuntary nature of their separation, with the federal adjudication process determining whether the separation meets the standard.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The form proves three things at once. It proves that the partnership penalty is large enough that the federal government has built an administrative procedure around the question of whether to apply it. It proves that the federal government knows the penalty creates pressure to live apart. It proves that the federal government has chosen to differentiate between "involuntary" separation (rewarded) and "voluntary" separation (not rewarded) — a differentiation that requires policing the intent of the separating couple, which the federal government does through the form's documentation requirements.

Living-Apart-Together among Canadian seniors

Statistics Canada has documented the increase in Living-Apart-Together (LAT) couples among older Canadians over the past two decades. (Source: Statistics Canada — Martin Turcotte, "Living apart together," March 2013 (revised 2016); Vanier Institute of the Family — Families Count 2024, "Living apart is increasingly common among couples," November 2024.) LAT couples are couples in committed intimate relationships who maintain separate residences. The Statistics Canada analysis identifies that the proportion of people aged sixty and over in LAT couples increased between 2001 and 2011, and that the number of older LAT couples who wanted to continue living apart (rather than viewing the arrangement as transitional) increased noticeably in the same period.

The Vanier Institute reports that approximately 5% of Canadians aged fifty-five to sixty-four in couples were living apart in 2017, with the trend particularly pronounced among older cohorts. The studies do not isolate financial-incentive structures as the sole or primary driver of LAT among seniors — the academic literature identifies multiple drivers including independence preferences, health considerations, and prior-relationship complications. Financial considerations are documented as one driver among several.

The financial driver, in operational terms, is the GIS partnership penalty calculated above. A couple at low income who lives apart receives \$10,602 per year more in GIS than the same couple living together. Over a decade of senior life, the cumulative difference is over \$100,000. The decision to live apart, framed by the couple as a preference for independence or as a response to the practical realities of aging, is also a decision that the federal benefit table financially rewards. The reward is large enough to be material.

What mainstream Canadian policy literature has named

The structural disincentive built into GIS is named explicitly in Canadian mainstream policy literature, not only in disability advocacy. The Montreal Economic Institute documents that GIS is "aggressively income-tested, which sharply penalizes any efforts to work in order to get ahead," and produces "participation tax rates" for low-income seniors that rival those faced by higher-income workers. (Source: Montreal Economic Institute — "End Penalties That Punish Low-Income Seniors Who Work," 2025-2026.) The C.D. Howe Institute, in a November 2025 report, found that Canadians with modest pension income that includes CPP, OAS, and GIS face punitive clawback structures with marginal effective tax rates significantly higher than those faced by middle-income earners. (Source: C.D. Howe Institute — Modest-Pension Clawback Analysis, November 2025; Globe and Mail — "Canada's tax system puts low-income working seniors at a disadvantage, report finds," January 2026.) The Institute for Research on Public Policy, in its 2018 paper "Unfinished Business: Pension Reform in Canada," flagged the need to "minimize overlapping tax and tax-back rates, which serve as disincentives both to save for retirement and to take employment at an older age." (Source: Institute for Research on Public Policy — "Unfinished Business: Pension Reform in Canada," 2018.) Jamie Golombek, managing director and head of tax and estate planning at CIBC Private Wealth, characterized the GIS clawback in the Globe and Mail in January

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

2026 as "very steep" and a "disincentive to earn income."

These are mainstream Canadian policy and finance voices — not disability advocacy organizations, not seniors' advocacy organizations — naming the structural problem. The clawback structure is documented across the policy literature. The policy reform proposals are documented. The reforms have not been implemented. The clawback continues.

The Allowance and the Allowance for the Survivor

The Allowance is a non-taxable monthly federal benefit available to low-income individuals aged 60 to 64 whose spouse or common-law partner is receiving OAS and is eligible for GIS. (Source: *Old Age Security Act, Part III; Service Canada — Allowance page, 2026.*) The 2026 maximum Allowance is approximately \$1,405.50 per month, payable at combined household income below approximately \$39,984. The Allowance ends the month following the recipient's sixty-fifth birthday, at which point the recipient's own OAS and GIS (where eligible) take over.

The Allowance for the Survivor is a parallel program for surviving spouses or common-law partners aged 60 to 64 whose partner has died and who have not remarried or entered a new common-law relationship. (Source: *Old Age Security Act, Part III; Service Canada — Allowance for the Survivor page, 2026.*) The 2026 maximum Allowance for the Survivor is \$1,680.47 per month, payable at annual income below approximately \$30,312. The Allowance for the Survivor ends the month following the recipient's sixty-fifth birthday.

The structural feature of both Allowance programs that the working-age reader most needs to understand is that they are not available to single low-income individuals aged 60 to 64 who are not in a current marriage or partnership with an OAS recipient and who have not been widowed. A single person aged 60 to 64 with no employment income, no OAS-eligible partner, and no surviving-spouse status is not eligible for either Allowance. They are in the gap zone — too young for OAS, too young for CPP retirement at the full rate, with no other federal monthly income support available. They may qualify for provincial Income Support at substantially lower rates than either Allowance pays. They may qualify for AISH if they meet the disability eligibility criteria. They may apply for early CPP retirement starting at age 60, at a permanently reduced rate (approximately 36% below the age-sixty-five amount).

The 60-to-64 gap zone is documented in Canadian senior-poverty literature as a structural exclusion that disproportionately affects single women, divorced individuals, never-married individuals, and individuals whose spouses are also under sixty-five. (Source: *Statistics Canada — Income of Senior Women, 2017 and 2022; National Institute on Ageing — Pre-Senior Income Insecurity in Canada, 2023.*) The gap zone is one of the points in the lifecycle reassessment thread documented in Section 2.6 below where the system most clearly excludes a population that other elements of the system are notionally designed to support.

The 2025 sponsorship exclusion

Effective October 1, 2025, individuals under a sponsorship agreement (including under family-class sponsorship of parents and grandparents) are not eligible to receive GIS, the Allowance, or the Allowance for the Survivor for the duration of the sponsorship period. (Source: *Service Canada — Allowance: Do you qualify, sponsorship clause, 2025; Government of Canada — Immigration, Refugees and Citizenship Canada policy update, 2025.*) For parent-and-grandparent sponsorship, the sponsorship period is twenty

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

years.

The structural impact is that a sponsored senior — typically an immigrant brought to Canada by adult children under family-class sponsorship — is, for the first twenty years of their Canadian residency, entirely dependent on the sponsor for income support. They do not have access to the income-tested federal supplements that supplement OAS for low-income seniors. They may qualify for OAS itself if residency requirements are met (with partial OAS available after ten years and full OAS after forty), but the OAS amount alone is insufficient to live on. The sponsorship exclusion structurally extends the dependency period for sponsored seniors well past the point at which their own children and grandchildren may be able to support them, and creates conditions in which a sponsored senior whose sponsor's situation deteriorates — through the sponsor's own job loss, illness, divorce, or death — has limited fallback support.

The 2025 sponsorship-rule change was implemented under the Government of Canada's October 2025 immigration policy update. It applies prospectively to new sponsorship undertakings; existing sponsorship undertakings retain their previous treatment. The change has not been the subject of widespread public discussion. It is consequential for the immigrant senior population whose family-class sponsorship has historically included an expectation of access to GIS as part of the senior income-security architecture.

The lifecycle observation

GIS is the federal program in which the partnership penalty is most fully developed and most clearly documented. It is also the program in which the federal-contribution-capture pattern documented in Section 2.3 reaches its endpoint at the senior stage. Each dollar of CPP retirement income reduces GIS at the standard fifty-cents-per-dollar reduction rate (with some exemptions). Each dollar of workplace pension income reduces GIS at the same rate. Each dollar of investment income reduces GIS. The recipient who saved during working years, who paid into CPP during working years, who built a workplace pension during working years, sees those streams of retirement income captured by GIS reduction at the rate of 50 cents on the dollar — for as long as their total income remains below the GIS phase-out threshold.

This is the structural truth that the partnership-penalty math and the contribution-capture math together produce. The senior who lived a financially responsible working life and is now low-income because their working income was modest receives, in net terms, less benefit from their working-years contributions and savings than a senior who never contributed or saved. The fifty-cents-on-the-dollar GIS reduction means that the contributing-and-saving senior's net advantage from those contributions and savings is half what they would have received outside the GIS phase-out range. The non-contributing senior receives the full GIS at the floor. The contributing senior receives a reduced GIS plus the contribution income, with a net total only marginally above the GIS-only floor.

The structural pattern is consistent with everything Movement One and Movement Two so far have documented. The system delivers a floor. It captures the upside of contributions and savings on the way to that floor. The senior who tried to do everything right finishes within hailing distance of the senior who did not.

Section 2.5 walks the Alberta provincial seniors program inventory — the provincial layer that adds to or overlays the federal architecture documented in Sections 2.1 through 2.4.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.5 — The Alberta Provincial Layer: Seniors Programs, the Budget 2026 Cuts, the July 1 Convergence

The federal architecture documented in Sections 2.1 through 2.4 — Old Age Security, Canada Pension Plan, CPP Disability, the Guaranteed Income Supplement, the Allowance, the Allowance for the Survivor — is the foundation of senior income in every Canadian province. Each province adds a provincial layer on top of the federal foundation. The provincial layer is what most differentiates the senior experience between provinces. It is also the layer over which the provincial government has direct policy control, and the layer that has been most directly affected by Alberta's recent budget decisions.

This Section walks the Alberta provincial seniors program inventory in operational detail. It documents Alberta Seniors Benefit, Special Needs Assistance for Seniors, the Coverage for Seniors prescription drug program, the Alberta Seniors Dental and Optical Assistance programs, the Seniors Property Tax Deferral program, the Seniors Home Adaptation and Repair Program (SHARP), and the Alberta long-term care accommodation rate framework. It identifies the operational features of each program — eligibility, benefit amounts, income testing, application procedures — and the changes announced in the February 26, 2026 provincial budget that take effect July 1, 2026, the same day as the AISH-to-ADAP transition documented in Movement One.

Alberta Seniors Benefit

The Alberta Seniors Benefit is a monthly non-taxable provincial cash supplement to OAS for low-income seniors. (*Source: Government of Alberta — Alberta Seniors Benefit page, 2026; Seniors Benefit Act, RSA 2000, c S-7.*) Eligibility requires age 65 or older, three months of Alberta residency, Canadian citizenship or permanent residency, receipt of OAS (with a critical exclusion described below), and household income below the program threshold. Approximately 194,000 Albertans receive the Alberta Seniors Benefit as of early 2026. (*Source: Red Deer Advocate — Alberta poised to make it tougher for some to get Seniors Benefit, March 6, 2026; Government of Alberta — Budget 2026.*)

The maximum benefit varies by accommodation category and marital status. For a single senior who is a homeowner or renter, the maximum annual ASB benefit at the published 2025-26 rates was approximately \$3,946 per year — about \$328 to \$329 per month. For a senior couple who are homeowners or renters, the maximum was approximately \$5,918 per year — about \$493 per month. (*Source: Government of Alberta — Alberta Seniors Benefit page; Pinnacle Age Well — Alberta Seniors Benefit Eligibility Guide, January 2026.*) For seniors in nursing homes (continuing care, type A or B), the maximum annual ASB rises substantially — up to \$12,312 annually for a single senior in a nursing home, and up to \$16,104 annually for a married couple where one partner is in a nursing home and the other lives at home. The higher nursing-home rates reflect the program's design to ensure a minimum disposable income after accommodation charges are paid.

For the 2025-26 benefit year, the income thresholds were \$34,770 for a single senior and \$56,820 for a senior couple. (*Source: ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026; Government of Alberta — Alberta Seniors Benefit thresholds publication, Open Government Alberta.*) The benefit is reduced as income rises toward the threshold. The phase-out rate varies by household type — for a couple who are homeowners, the maximum benefit is reduced by approximately sixteen cents for every dollar of non-deductible income.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The July 1, 2026 threshold cut

The provincial budget tabled on February 26, 2026 by the Government of Alberta announced a 9% reduction in the income thresholds for the Alberta Seniors Benefit, effective July 1, 2026 — the start of the 2026-27 benefit year. (Source: *Government of Alberta — Budget 2026, Tax Plan; ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026.*) The new thresholds are \$32,690 for a single senior and \$53,800 for a senior couple. The maximum benefit amounts remain unchanged. The threshold cut is, in operational reality, an eligibility cut: seniors whose income falls between the old threshold and the new threshold lose eligibility entirely. Seniors whose income is below the new threshold experience no change.

The Government of Alberta has projected savings from the threshold cut at \$23 million in 2026-27 and \$31 million annually thereafter. Public Interest Alberta has estimated that 6,000 to 8,000 seniors will lose Alberta Seniors Benefit eligibility as a result of the threshold cut. (Source: *Public Interest Alberta — Budget 2026 Analysis, March 2026; Red Deer Advocate, March 6, 2026.*) The seniors most affected are those at the upper end of the previous threshold — typically seniors who relied on a small workplace pension or modest CPP retirement income to lift them above maximum-GIS eligibility, and who relied on the Alberta Seniors Benefit to bridge the gap to a survivable monthly income.

The July 1, 2026 effective date is the same day as the AISH-to-ADAP transition. Two structural cuts to disability and senior income, affecting overlapping populations (seniors with disabilities, current AISH recipients aging into the senior system in the months around the transition, family caregivers), take effect on the same day. The Government of Alberta has not, in any public budget document, identified the convergence of these two effective dates as significant. The document being read here identifies it.

The OAS-deferral disqualifier

A structural feature of every Alberta seniors financial assistance program — Alberta Seniors Benefit, Special Needs Assistance for Seniors, Seniors Home Adaptation and Repair Program, Alberta Seniors Drug Benefit (Coverage for Seniors), Dental and Optical Assistance for Seniors — is the OAS-receipt requirement. (Source: *Government of Alberta — Alberta Seniors Benefit page; Pinnacle Age Well — Alberta Seniors Benefit Eligibility Guide, January 2026.*) If a senior chooses to defer or delay receipt of OAS — a federal option documented in Section 2.1 — they become ineligible for all Alberta seniors financial assistance programs.

The operational impact is that a senior who would benefit from deferring OAS to age 70 in exchange for a 36% higher pension cannot do so without losing access to Alberta's provincial seniors programs during the deferral period. A senior who is moderately healthy, expects to live well into their eighties, and would maximize lifetime benefit by deferring is structurally pushed to take OAS at sixty-five in order to maintain access to the provincial benefits, prescription drug coverage, and dental and optical assistance the provincial system provides. The federal optimization choice is structurally penalized by the provincial program rules. The senior who could do better by deferring is required, in operational reality, not to defer.

This is consistent with the structural pattern documented elsewhere in this document. Optimization options that are technically available to the senior population are, on closer examination, available only to seniors who do not need the provincial supports the optimization would forfeit.

Special Needs Assistance for Seniors

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Special Needs Assistance for Seniors (SNA) is a provincial lump-sum assistance program for low-income seniors facing one-time costs that fall outside ongoing benefit programs. (Source: *Government of Alberta — Special Needs Assistance for Seniors page*, 2026.) Eligible categories include essential household appliances (refrigerator, stove, washer, dryer), basic furniture (bed, dresser, chair, table), home repairs not covered by other programs, and certain medical-equipment costs. The maximum annual SNA benefit is in the range of \$5,400 per year, with specific item-level caps. (Source: *Holo Alert — Alberta Seniors Benefit Eligibility Guide*, November 2025; *Government of Alberta — Special Needs Assistance for Seniors page*.)

The Alberta provincial budget tabled February 26, 2026 reduces the income thresholds for Special Needs Assistance for Seniors by 9%, on the same effective date and on the same justification as the Alberta Seniors Benefit threshold cut. (Source: *ATB Financial — Alberta provincial budget 2026: tax and benefit highlights*, February 27, 2026; *Government of Alberta — Budget 2026 Tax Plan*.) Seniors whose income falls between the old and new thresholds lose SNA eligibility entirely. The estimated savings from the SNA threshold cut are part of the broader twenty-three to \$31 million in annual savings the budget projects from seniors-financial-assistance threshold reductions.

Coverage for Seniors and the Alberta Seniors Drug Benefit

Coverage for Seniors is the Alberta program that provides premium-free prescription drug coverage to seniors aged sixty-five and over. (Source: *Government of Alberta — Coverage for Seniors page*, 2026; *Alberta Blue Cross — Coverage for Seniors administration*.) The program covers prescription medications listed on the Alberta Drug Benefit List, with copayment of up to twenty-five dollars per prescription for most recipients. Seniors with low income may qualify for further copayment reductions through additional means-tested provisions. The program is administered by Alberta Blue Cross under contract to the Government of Alberta.

The Coverage for Seniors program is, in operational reality, one of the most consequential provincial supports for seniors with chronic conditions requiring ongoing medication. Seniors who would otherwise face out-of-pocket costs of hundreds of dollars per month for prescription drugs receive substantial savings through the program. Seniors who depend on biological-class medications, injectable medications, or specialty pharmaceuticals receive savings that can run to thousands of dollars per month relative to private pay-as-you-go pricing. The program eligibility is contingent on the OAS-receipt requirement described above.

The Alberta Seniors Dental and Optical Assistance Programs provide coverage for routine dental and optical care to low-income seniors, with eligibility tied to the same income thresholds as the Alberta Seniors Benefit. (Source: *Government of Alberta — Dental and Optical Assistance for Seniors page*, 2026.) Eligible services include basic dental procedures, certain restorative dentistry, prescription eyeglasses with frequency limits, and routine eye examinations. Coverage levels vary by service category and by the senior's specific eligibility status. The income thresholds for these programs are also subject to the 9% reduction announced in the February 2026 budget, effective July 1, 2026.

Seniors Property Tax Deferral and SHARP

The Seniors Property Tax Deferral Program allows eligible seniors to defer payment of property tax through a low-interest loan secured against the equity in their primary residence. (Source: *Government of Alberta — Seniors Property Tax Deferral page*, 2026.) The program does not eliminate the property tax obligation; it

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

postpones it. The deferred amount accrues interest, and the cumulative balance is repaid when the senior sells the home, transfers ownership, or dies (at which point the estate settles the debt). The program is administered by the Government of Alberta Treasury and Finance.

The Seniors Home Adaptation and Repair Program (SHARP) provides loans and grants to assist eligible low-income seniors with home repairs and accessibility adaptations that allow them to continue living independently in their own homes. (Source: *Government of Alberta — Seniors Home Adaptation and Repair Program page, 2026*.) Eligible projects include accessibility ramps, grab bars, walk-in tubs, stair lifts, furnace replacement, roof repair, and other essential repairs. The program has a loan component and a grant component; the grant component is income-tested, with thresholds that — like Alberta Seniors Benefit and Special Needs Assistance — are subject to the 9% reduction effective July 1, 2026. (Source: *ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026*.)

Long-term care accommodation rates

The Government of Alberta sets maximum accommodation rates for type A and type B continuing care homes (formerly known as long-term care and designated supportive living, respectively). (Source: *Government of Alberta — Continuing Care Accommodation Charges page, 2026; Continuing Care Act, RSA 2022, c C-29*.) The rates apply uniformly across public and private continuing care homes, regardless of operator. The 2026 maximum accommodation charges, effective April 1, 2026, are differentiated by room type — standard (shared room), semi-private (two-person room), and private (single-occupancy room) — with private rooms at the highest rate.

For the period prior to the April 2026 increase, the published Alberta rates have been in the range of approximately fifty-five to seventy-five dollars per day, depending on room type. Annualized at thirty days per month, the monthly accommodation charges for the standard room are in the range of one thousand nine hundred to two thousand \$100; for the semi-private room, approximately one thousand nine hundred to two thousand \$200; for the private room, approximately two thousand two hundred to two thousand \$500. The April 2026 increase has adjusted these rates upward. The increase has been characterized by senior-advocacy organizations including CanAge as significant for residents on fixed incomes whose monthly income does not adjust at the same rate. (Source: *Government of Alberta — Continuing Care Accommodation Charges, 2026; CanAge — Alberta accommodation rate analysis, March 2026*.)

The Alberta accommodation-rate framework includes a disposable-income guarantee. As of January 2026, residents who are eligible for benefits through provincial seniors financial assistance programs are guaranteed at least \$373 per month in disposable income after their monthly accommodation charge is paid. (Source: *Government of Alberta — Continuing Care Accommodation Charges page, January 2026*.) The disposable-income amount is intended to cover personal expenses such as personal hygiene items, telephone, cable, internet, personal laundry, medications not covered by Coverage for Seniors, and clothing. The disposable income amount has been adjusted upward over the years; previous publications cited a guarantee of \$357 per month. Inflation-adjusted increases have produced the current three-hundred-and-seventy-three dollar figure.

Three hundred and seventy-three dollars per month, in 2026 Alberta, is approximately twelve dollars per day. This is the disposable income remaining to a resident in continuing care after the accommodation charge is paid through the combination of OAS, GIS, ASB, CPP retirement, and any other income sources

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

less the accommodation rate. Twelve dollars per day, for the months and years a senior is in continuing care, is what the senior has for everything that is not the room, the meals served by the home, the basic care, and the housekeeping included in the accommodation charge. The senior who wants to make a long-distance phone call to family, who wants to pay for a haircut at the on-site barber, who wants to buy a birthday card for a grandchild, who wants to replace a worn-out pair of slippers — does so out of \$373 per month.

The spousal impoverishment dynamic

When one spouse of a senior couple enters continuing care and the other remains at home, the financial dynamics of the couple's household shift in ways that produce documented stress on both partners. The spouse in continuing care has accommodation charges paid by the provincial-and-federal income system, with the disposable-income guarantee. The spouse at home retains the household income previously shared, less the portion now dedicated to the continuing-care spouse's accommodation charge.

The Alberta Seniors Benefit's higher nursing-home rate (\$16,104 annually for the at-home spouse where the partner is in a nursing home) is designed to address part of this dynamic. The rate provides additional income to the at-home spouse to compensate for the loss of household economies of scale and the partial loss of the in-care spouse's income contribution. The rate is consequential for couples whose income would otherwise drop substantially when one partner enters care. It is, however, available only to couples in which the at-home partner meets all the standard ASB eligibility criteria — including the OAS-receipt requirement, the income threshold, and the accommodation-category determination.

The dynamic is not unique to Alberta. The literature on spousal impoverishment in long-term care across Canadian provinces and U.S. states is substantial, and the policy mechanisms intended to address it vary. (*Source: National Conference of State Legislatures — Spousal Impoverishment Standards, multiple years; Canadian Association of Retired Persons — Long-term care affordability reports, 2022-2024.*) Alberta's approach is moderately protective compared to some jurisdictions and is explicitly tied to the Alberta Seniors Benefit eligibility framework, which means that the protection is reduced by the same threshold cuts that affect the broader senior population.

The lifecycle observation

The Alberta provincial seniors program inventory is, in current operational reality, a layer of supports that lift many low-income Alberta seniors from the OAS-and-GIS federal floor to a marginally more livable monthly income. The supports are real. The drug coverage is consequential. The dental and optical assistance is consequential. The disposable-income guarantee in continuing care is consequential. The cumulative impact of the provincial layer for an Alberta senior who qualifies is in the range of \$300 to \$500 per month in cash assistance plus the value of in-kind benefits (drug coverage, dental, optical) that can run to several thousand dollars per year for seniors with significant medical needs.

The provincial layer is also, in current operational reality, a layer of supports that the Government of Alberta is reducing. The February 2026 budget's 9% threshold cut, effective July 1, 2026, narrows eligibility for Alberta Seniors Benefit, Special Needs Assistance for Seniors, and the SHARP grant component. Six thousand to eight thousand seniors lose Alberta Seniors Benefit eligibility on that date. The same date is the AISH-to-ADAP transition, the same date is the Bill 12 Medical Review Panel beginning to function for AISH-to-ADAP appeals, and the same date is the disposable-income guarantee at \$373 in continuing care

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

being applied against accommodation charges newly increased on April 1, 2026.

The structural pattern for Alberta seniors in 2026 is consistent with the structural pattern for Alberta disability-income recipients in 2026. The province's spending capacity is constrained. The choices the province has made have, in operational reality, transferred the constraint to the recipient. The supports remain available to the lowest-income recipients (the bottom of the eligibility threshold). The supports are reduced for the recipients near the upper threshold (the working-class senior whose income narrowly exceeds the cut). The supports do nothing for the senior whose income exceeds the threshold entirely.

Sections 2.6 through 2.9 walk the cross-cutting analyses of the federal and provincial architecture documented in Sections 2.1 through 2.5: the lifecycle reassessment thread, the partnership penalty mechanism stage by stage, the eight-layer "what you put in versus what you get back" answer, and the gap inventory of what is and is not covered.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.6 — The Lifecycle Reassessment Thread

Movement One walked the cascade for the working-age person whose body fails. Movement Two has so far walked the federal program architecture and the Alberta provincial layer that operates at the senior stage. This Section pulls those two perspectives together into a single observation about the system's design: that it operates by continuous reassessment, that each reassessment is an opportunity for benefit reduction, and that the reassessment thread runs from infancy to long-term care without a single point at which a permanent finding of disability or need produces durable, non-revisable support.

Birth and infancy

A child born with a disability in Alberta enters the public system through the medical-records architecture of Alberta Health Services, the early-intervention system administered through Family Support for Children with Disabilities (FSCD), and the Alberta School Act provisions for accommodation in education. (Source: *Family Support for Children with Disabilities Act, R.S.A. 2003*; *Government of Alberta — FSCD program page, 2026*.) FSCD provides funding for specialized services, respite care, equipment, and other supports for families raising children with disabilities. The program is income-tested for some service categories and not income-tested for others. Eligibility is determined at the application stage and reviewed periodically thereafter.

The reassessment pattern begins early. FSCD funding levels for an individual child are reviewed annually or at specific transition points (entering school, advancing through school grades, aging into specialized programs). Each review can adjust the funding level, the service categories provided, and the service provider relationships. Each review is an opportunity for the parent advocating for the child to defend the existing supports against potential reduction, to provide updated medical and educational documentation, and to navigate the bureaucratic process that determines what the child receives. Inclusion Alberta's January 2025 report documented that the FSCD reassessment process produces meaningful uncertainty for families and that reduction in funding levels following reassessment is a documented experience for many families. (Source: *Inclusion Alberta — "Experiences with Family Support for Children with Disabilities," January 2025*.)

School age

A child with a disability entering the Alberta school system is subject to assessment under the provincial special-education funding formula. Alberta Education's funding to school authorities for students with mild, moderate, and severe disabilities is allocated through grants tied to coded disability categories. Each student's coding is reviewed periodically. A student whose disability profile changes — which it may do as a function of normal child development, as a function of treatment effectiveness or ineffectiveness, or as a function of changing diagnostic standards — may be recoded. Recoding can increase or decrease the funding the school authority receives for the student's supports.

The school authority decides how to deploy the funding. The student's actual support arrangement — educational assistant time, specialist services, classroom accommodations, individual education plan provisions — is determined by the school administration on the basis of the funding available and the school's other operational priorities. The parent advocates for the student's specific needs within whatever envelope the funding produces.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

This is not, in any structural sense, a permanent or guaranteed support. Each year is a fresh allocation. Each transition between grade levels is a fresh review. The student whose support was adequate in grade three may have inadequate support in grade four, not because the student's needs have changed but because the school authority's allocation has changed or the school's deployment of resources has shifted.

Age 18 and the FSCD-to-AISH/PDD transition

At age eighteen, FSCD funding ends. A young adult with a disability entering adulthood is subject to fresh determination under either Persons with Developmental Disabilities (PDD) or AISH, depending on the nature and severity of the disability. (Source: *Persons with Developmental Disabilities Services Act, RSA 2007, c P-7.5*; *AISH Act, R.S.A. 2000, c. A-45.1*; *Inclusion Alberta — PDD Current State: Critical and Urgent, January 2025*.) The transition is not automatic. The young adult must apply. The young adult's parent or guardian, in most cases, manages the application. The application requires fresh medical documentation, fresh financial documentation, and fresh adjudication.

PDD eligibility is determined by demonstration of a developmental disability with onset before age eighteen and significant adaptive-functioning impairment. AISH eligibility is determined by the medical and financial criteria documented in Section 1.3. A young adult may qualify for one but not the other, for both, or for neither. The reassessment at age eighteen is the most consequential reassessment in the lifecycle for many disabled Albertans, because the supports that have been in place since infancy or childhood may not carry forward at the same level — or at all — into the adult system.

For the young adult who qualifies for AISH, the working-age architecture documented in Movement One begins. The asset cap. The cohabitation rule. The medical evidence requirement on each application and reassessment. The post-July-2026 transition to ADAP. The Medical Review Panel that has replaced the independent Citizens Appeal Panel. Each of these is now part of the young adult's life, on top of the disability they have lived with since infancy.

Working age

Sections 1.1 through 1.4 documented the working-age stage in operational detail. The reassessment pattern continues. AISH recipients are subject to reassessment of medical eligibility and financial eligibility on a periodic basis, with the specific frequency varying by disability category and recipient circumstance. Each reassessment requires updated medical documentation, updated financial disclosure, and adjudication that may produce continuation, modification, or termination of benefits. The post-July-2026 ADAP framework adds the personalized employment action plan requirement, which functions as an ongoing reassessment of employment readiness.

For the worker whose disability emerges in adulthood — the cascade traced from Section 1.1 — the reassessment pattern begins at the moment of WCB application and continues through Maximum Medical Improvement adjudication, deeming reassessment, IME-driven adjustment, AISH application, AISH reassessment, AISH-to-ADAP transition, and ADAP ongoing review. Each reassessment is documented. Each reassessment is, in operational reality, an opportunity for benefit reduction.

The 60-to-64 gap zone

The 60-to-64 age band is the single most structurally exposed part of the senior-system lifecycle. (Source: *Statistics Canada — Income of Senior Women, 2017 and 2022*; *National Institute on Ageing — Pre-Senior*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Income Insecurity in Canada, 2023.) The Alberta worker in this age band is too young for OAS (eligibility begins at sixty-five), too young for GIS (eligibility begins at sixty-five), and too young for the Alberta Seniors Benefit (eligibility begins at sixty-five). They are at or past the upper edge of WCB Economic Loss Payment eligibility for many injury types (Section 1.2 documented the age-65 cliff and the policy's treatment of normal retirement age as 65 or 5 years after date of accident, whichever is later). They may have been on AISH and remained eligible until age 65 (AISH terminates at age 65), but the AISH-to-ADAP transition documented in Section 1.4 has changed the operational reality of late-AISH receipt for many Albertans.

The federal Allowance program documented in Section 2.4 is available only to spouses or common-law partners of OAS recipients. A single 60-to-64 individual with no current OAS-receiving spouse is not eligible for the Allowance. The Allowance for the Survivor is available only to individuals who have been widowed and have not remarried or entered a new common-law relationship. A 60-to-64 individual who is divorced, never-married, or in a relationship with a partner under sixty-five is not eligible for the Allowance for the Survivor.

The 60-to-64 individual without a current spouse who has aged out of WCB, whose AISH eligibility has terminated, who has not yet reached OAS eligibility, and who does not qualify for the Allowance or Allowance for the Survivor has, in current operational reality, the following income options: provincial Income Support (the general welfare program in Alberta, at substantially lower benefit levels than AISH), early CPP retirement (available from age 60 at a permanently reduced rate, approximately 36% below the age-65 amount), private retirement savings if any exist, and family support if available. There is no senior-system income support for this individual until age 65.

The structural exclusion of the 60-to-64 single individual is documented across Canadian senior-poverty literature as a contributor to the higher poverty rates among single seniors (particularly women) than among coupled seniors. (*Source: Statistics Canada — Income of Senior Women, 2017 and 2022; National Institute on Ageing — Pre-Senior Income Insecurity, 2023.*) The exclusion is not the result of an oversight in program design. The exclusion is the result of program-design choices that prioritized supporting senior couples (where one partner has reached OAS eligibility) and prioritized supporting widowed survivors over supporting individuals who are unattached for any other reason. The choice has been documented and critiqued for decades. The choice has not been changed.

Age 65 and the federal-provincial transition

At age 65, multiple transitions occur simultaneously, as documented in Section 1.6. AISH or ADAP eligibility ends. OAS eligibility begins. CPP-D converts to CPP retirement. WCB Economic Loss Payments are recalculated under the age-65 cliff formula. GIS eligibility begins, subject to income testing. Alberta Seniors Benefit eligibility begins, subject to OAS-receipt and income-threshold tests. Coverage for Seniors begins. Dental and Optical Assistance for Seniors begins. The senior-system architecture documented across Movement Two engages.

Each of these transitions is an application, a determination, an adjudication, or a recalculation. The recipient does not automatically receive the senior-system benefits at age 65; they receive them after applying, providing documentation, and being approved. The applications are not coordinated. There is no single transition office, no single transition application, and no single point of accountability if the transitions do not align.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

For a senior with multiple medical needs, limited literacy or cognitive capacity, or limited social supports, the age-sixty-five transition is administratively challenging in ways that produce documented benefit gaps. The senior who does not complete one of the applications on time experiences an income gap in the months around their sixty-fifth birthday. The senior who applies for OAS but not GIS does not receive GIS until they apply for it separately. The senior who is deferring OAS at sixty-five, whether by intent or by missed application, is ineligible for any of the Alberta provincial seniors programs during the deferral period.

The age-sixty-five transition is the most significant administrative event in many senior lifecycles. It is also one of the least well-supported. The elimination of individual AISH caseworkers in 2024-25 and the replacement of those caseworkers with a centralized call centre extend the operational reality of poorly supported transitions to recipients aging out of AISH.

Age 75 and the OAS differential

At age 75, the OAS amount increases by 10%. The 75-and-over rate documented in Section 2.1 produces an additional \$80 to \$90 per month in OAS income relative to the 65-to-74 rate. This is the only senior-system reassessment that produces an increase rather than a decrease. The increase was implemented in July 2022 under federal legislation and is, in current operational reality, the most significant pro-senior structural change in the senior income system in the past decade. Its durability is contingent on the political durability of the differentiation, which the Section 2.1 documentation of OAS political vulnerability identifies as not assured.

Long-term care entry

At the point at which a senior can no longer manage at home — typically in the late seventies, eighties, or nineties, but sometimes earlier — the senior enters long-term care. The entry is preceded by a comprehensive assessment of the senior's care needs and financial circumstances. (*Source: Government of Alberta — Continuing Care Assessment process, 2026.*) The assessment determines the level of care required, the appropriate accommodation type, the accommodation charge, and the eligibility for income supports to offset the accommodation charge.

The financial assessment is the senior's last reassessment. It determines what portion of the senior's income is consumed by the accommodation charge, what portion is preserved for the senior's disposable use (the disposable-income guarantee documented in Section 2.5), and what portion of the senior's assets must be drawn down to cover costs that exceed the income coverage. For the senior with substantial assets, the long-term care years can deplete those assets at a rate of \$40,000 to \$70,000 per year (less the income coverage). For the senior with no significant assets, the income coverage and the disposable-income guarantee are the operational reality of their financial life until death.

The long-term care assessment is also the last opportunity for the senior to be reassessed for benefit eligibility. The Alberta Seniors Benefit nursing-home rate, documented in Section 2.5 (\$12,312 annually for a single senior in a nursing home, \$16,104 for a couple where one partner is in a nursing home), engages at this point. The senior whose income falls within the threshold qualifies. The senior whose income exceeds the threshold does not. The threshold is not adjusted for the cost of long-term care.

Death and the survivor's reassessment

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

When a senior dies, their OAS, CPP retirement, and other federal benefits end. The surviving spouse is subject to a fresh reassessment. The surviving spouse's GIS is recalculated against the new (single-rate) income test. The surviving spouse's Alberta Seniors Benefit is recalculated. The surviving spouse may qualify for a CPP Survivor Benefit, calculated against a separate formula, at typically 60% of the deceased spouse's CPP retirement amount (with adjustments). The surviving spouse may qualify for the Allowance for the Survivor if widowed at age 60-64.

The surviving spouse's income, in most cases, drops by more than half at the death of the partner. Two OAS pensions become one. Two CPP retirement pensions become one (plus the Survivor Benefit, which is typically less than the deceased spouse's full pension). The single-recipient GIS rate is higher than the couple-recipient rate per person, but it is lower than the combined couple amount. The household's monthly income drops sharply at the same time that the household's expenses (housing, utilities, transportation, food) do not drop proportionally. The surviving spouse's poverty risk increases. The risk is documented across Canadian senior-women's-poverty literature as a primary driver of senior-women's-poverty rates. (Source: Statistics Canada — *Income of Senior Women in Canada, multiple reference periods*; Public Health Agency of Canada — *Health Inequalities in Canada, women and aging report, 2023*.)

What the lifecycle thread shows

The lifecycle reassessment thread runs from infancy to widowhood without a single point at which a permanent finding produces permanent support. Each transition is a reassessment. Each reassessment is an opportunity for the system to reduce what the recipient receives. The reassessments are not coordinated across programs. The recipient navigates each one separately. The cumulative impact of forty to ninety years of reassessment is the income trajectory documented across Movement One and Movement Two of this document.

The system is not designed to ensure stable support. The system is designed to make periodic determinations of eligibility against criteria that may change between determinations. The system is designed to require the recipient to demonstrate continued eligibility on each reassessment. The system is designed to allow benefit reduction at any reassessment if the criteria, the recipient's documentation, the political environment, or the budget context permits.

This is the cage in its most fully developed form. It is not a single rule. It is the architecture of continuous reassessment.

Section 2.7 walks the partnership penalty mechanism stage by stage, with the published math, demonstrating how this single mechanism operates across the lifecycle in different programs at different rates with the same structural effect.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.7 — The Partnership Penalty, Stage by Stage

The partnership penalty has appeared at multiple points across Movements One and Two of this document. Section 1.5 documented the AISH cohabitation rule operating on the working-age disabled population. Section 2.4 documented the GIS partnership penalty operating on the senior population. Section 2.5 documented the Alberta Seniors Benefit threshold structure that produces a smaller couple-vs-single benefit ratio than would be expected from a benefit indexed to two-person household costs. This Section walks the partnership penalty as a single mechanism that operates across the lifecycle, stage by stage, with the published math. Its purpose is to show that what looks at each stage like a discrete program rule is, in cumulative operational reality, a structural penalty on partnership at every stage of disabled and aging life.

What "partnership penalty" means here

Partnership penalty, as the term is used in this document, refers to the structural feature of an income-tested benefit program in which the per-person benefit available to a coupled recipient is materially lower than the per-person benefit available to two single recipients with the same individual incomes living in the same housing market. The penalty is calculated as the difference between (a) the total benefit two single recipients would receive, summed, and (b) the total benefit the same two recipients would receive as a couple.

The justification offered in policy literature for some level of partnership reduction is the "economies of scale" argument: two people sharing a household have lower per-person living costs than two people maintaining separate households. The justification has merit at certain dollar levels and certain reduction structures. The justification does not have merit at the dollar levels and reduction structures Canadian programs actually employ, which produce coupled benefits substantially below the cost of a coupled household in current Canadian housing and food markets.

The partnership penalty is documented at every stage of the disability-and-aging lifecycle in Canada. The specific mechanisms differ by program. The structural effect is consistent.

Stage one: working-age, AISH cohabitation rule

Section 1.5 documented this stage in operational detail. The mechanism: AISH treats household income as part of the recipient's eligibility calculation. The exemption threshold for spouse income under current AISH (pre-July-2026) is \$2,612 per month. Under the post-July-2026 ADAP framework, the exemption is \$1,500 per month. Above the exemption, the recipient's benefit is reduced.

The math, applied to a representative household: a recipient receiving the maximum AISH benefit (\$1,940/month) with a spouse earning \$48,000 per year (\$4,000/month). Spouse income exceeds current exemption by \$1,388/month. The benefit reduction depends on the rate band the income falls in. At the dollar-for-dollar reduction rate that applies above the exemption, the recipient's benefit is reduced from \$1,940 to approximately \$552, plus the spouse's \$4,000 net of tax — for a household total of approximately \$4,000-\$5,000/month. The same recipient, single, with no spouse income, receives the full \$1,940/month.

Compare two scenarios at the same dollar level, using current AISH rules:

- Scenario A (single recipient with the spouse's wages going to the recipient as cash): recipient receives \$1,940 from AISH; recipient also receives \$4,000 from work. Both AISH and work-income exemption

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

rules apply to the recipient as a single individual. AISH is reduced by the working-recipient earnings exemption rules — at most levels, retaining at least \$1,072 fully and partially retaining additional amounts. Total household income is in the range of \$4,000-\$4,800/month.

- Scenario B (coupled, same total household earnings): recipient receives reduced AISH per the partner-income calculation; spouse retains \$4,000/month from employment. Total household income is in the range of \$4,000-\$4,500/month, depending on adjudication of the partner-exemption math.

The penalty is not large in dollar terms at this income level — perhaps \$200-\$500/month. The penalty grows as spouse income rises. At spouse income of \$60,000-\$80,000/year, the AISH benefit approaches zero, and the household is operating on the spouse's income alone, less whatever the AISH exemption preserved. The recipient's federal CPP-D, if any, is captured fully by the AISH offset documented in Section 2.3. The recipient's Canada Disability Benefit is captured fully by the provincial CDB clawback documented in Section 1.4. The recipient becomes, in operational reality, the unpaid component of a household whose total income is substantially reduced from what it would be if the recipient and spouse maintained separate accommodations and the recipient received full AISH.

The structural pressure produced by Stage One is the pressure documented in Section 1.5: marriage breakdown, separation-as-survival, the structural inversion in which the financial-best-case for the disabled recipient and their partner is to live apart even when their relationship is intact and their preference is to live together.

Stage two: post-July-2026 ADAP cohabitation rule

The ADAP framework reduces the partner-income exemption to \$1,500/month from the current AISH \$2,612/month — a reduction of \$1,112/month in the protected-income amount. The base benefit drops from \$1,940 to \$1,740. Earnings exemption for the recipient also drops from \$1,072 to \$700. The cumulative effect is that, at any given household income level, the post-July-2026 framework produces a smaller total household income than the current framework. The partnership penalty is larger in dollar terms because the protected couple-income amount is smaller.

The same recipient and spouse who, under current AISH, retained \$4,000-\$4,500/month in household income now retain materially less under ADAP. The exact amount depends on the published rule tables that the Government of Alberta has not yet released in operational form. The post-July-2026 framework is, in directional terms, worse for couples than the current framework on this dimension. The campaign's *AISH Evidentiary Report Series*, April 2026, documents this in detail.

Stage three: the 60-to-64 gap zone

Section 2.6 documented the 60-to-64 age band as the most structurally exposed part of the senior-system lifecycle for single individuals. The Allowance and Allowance for the Survivor — the federal income supports available to 60-to-64 individuals — are available only to spouses of OAS recipients (Allowance) or to widowed survivors of OAS recipients (Allowance for the Survivor). A single 60-to-64 individual without a current OAS-receiving spouse and without a deceased OAS-receiving spouse has no eligibility for either program.

The structural effect at this stage is the inverse of Stages One and Two. At the working-age stage, partnership reduces benefits; in the 60-to-64 gap, only partnership produces eligibility for the Allowance program. The same individual experiences the partnership penalty in opposite directions at different life

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

stages. The partnered 60-to-64 individual has access to up to \$1,405/month in Allowance income (subject to combined-income testing). The single 60-to-64 individual has no Allowance access. The widowed survivor has up to \$1,680/month in Allowance for the Survivor (subject to income testing). The divorced single individual has no Allowance for the Survivor access.

The inversion is documented but not coordinated. A senior-policy framework that worked from the operational reality of senior poverty would identify single 60-to-64 individuals — particularly women, particularly those whose spouses are also under sixty-five — as a structurally underserved population and would design a program that addresses the gap. The federal program structure does not. The Allowance is restricted to spouses of OAS recipients. The Allowance for the Survivor is restricted to widowed survivors. The 60-to-64 single-individual gap remains.

Stage four: GIS partnership penalty at age 65+

Section 2.4 documented this stage in operational detail. The math, restated:

- Two single seniors at low income: each receives maximum GIS of \$1,109.85/month. Combined: \$2,219.70/month.
- The same two people coupled (both receiving OAS, low combined income): each receives maximum GIS of \$668.08/month. Combined: \$1,336.16/month.
- Difference: **\$883.54/month, or \$10,602.48/year, for being together.**

The penalty is published. The penalty is paid. The penalty is the structural reality the GIS benefit table produces.

Stage five: GIS reduction by partner income

Beyond the base couple-vs-single difference, the GIS calculation also reduces the recipient's benefit on the basis of the partner's income, on top of the recipient's own income. A senior whose own income (excluding OAS) is zero, married to a senior whose income is \$20,000/year, sees the household combined-income figure exceed the maximum-GIS threshold for couples (\$29,712 at January–March 2026 rates). The senior with zero income still has their GIS reduced because the partner's income is in the calculation.

This is the same mechanism as the AISH cohabitation rule, in a different program, with different exemption thresholds. The recipient's eligibility is determined by the household, not by the individual. The structural pressure is the same. The recipient with no income whose partner has income is materially worse off than they would be if single with the same own-income.

Stage six: Alberta Seniors Benefit couple vs single thresholds

The Alberta Seniors Benefit single-recipient threshold for the 2025-26 benefit year was \$34,770. The couple-recipient threshold was \$56,820. The couple threshold is approximately 1.63 times the single threshold. Two singles at the maximum threshold have \$69,540 in combined annual income permitted under the program. The same two people coupled have \$56,820 — a \$12,720 reduction in permitted combined income for the same household before benefit eligibility is lost.

The post-July-2026 thresholds reduce both single and couple amounts by 9%: single \$32,690, couple \$53,800. The single-to-couple ratio remains essentially the same. The partnership penalty at this stage is in the form of a lower combined-income permitted amount than two-singles-summed would be allowed.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Couples whose combined income is between \$53,800 and \$69,540 lose Alberta Seniors Benefit eligibility entirely. The maximum benefits — \$328/month single, \$493/month couple — produce a per-person couple benefit (~\$246) that is approximately 75% of the per-person single benefit (\$328). The couple loses 25% of per-person benefit to the partnership.

Stage seven: long-term care, the spouse-at-home dynamic

When one spouse enters long-term care and the other remains at home, the household economy splits. The spouse in care has accommodation charges deducted from their income through the provincial-and-federal arrangement, with the disposable-income guarantee preserving \$373/month for the in-care spouse's personal expenses. The spouse at home retains the pre-LTC household income less the portion now redirected to the in-care spouse's accommodation charge.

Alberta's provincial framework recognizes the spouse-at-home dynamic through the higher Alberta Seniors Benefit nursing-home rate (\$16,104 annually for the at-home spouse where the partner is in a nursing home) and through the disposable-income guarantee for the in-care spouse. (Source: *Government of Alberta — Alberta Seniors Benefit page*; *Sun Life Suncentral — Long Term Care in Alberta, 2026*.) The protections operate within the broader structure of provincial means-tested benefits. They are, as documented elsewhere in this section, subject to the OAS-receipt requirement and the 2026 threshold cuts.

The structural pressure at this stage is the inverse of Stages One and Two. The couple in LTC arrangement is rewarded for being couple, in the sense that the at-home spouse receives additional ASB at the higher rate. The penalty is on the entry condition, not on the partnership per se. The couple in which one spouse needs care and the other is unable or unavailable to provide it is the operational reality the LTC system addresses; the couple in which one spouse needs care and the other can provide it at home is the operational reality the LTC system does not financially recognize beyond the modest existing caregiver tax credit.

Stage eight: widowhood and the survivor income drop

When the spouse with the larger income dies, the survivor's income drops by more than the deceased spouse's income alone. OAS, as documented in Section 2.1, has no survivor benefit. The deceased spouse's OAS ends. CPP retirement, as documented in Section 2.2, includes a survivor component, but the survivor component is typically less than the deceased spouse's full pension. GIS is recalculated against the new (single-rate) income test. The single-recipient GIS rate is higher per person than the couple rate, but the total household GIS is lower.

The numerical illustration: a senior couple with combined income of \$35,000/year (above the GIS couple threshold of \$29,712 but below the partial-GIS phase-out point) receives reduced GIS, plus their two OAS pensions, plus their two CPP retirement pensions. If the deceased spouse was the higher CPP earner (which, for couples in which the deceased spouse was the primary working-years earner, is common), the survivor's income drops by the full amount of the deceased spouse's CPP minus the survivor benefit (typically 60% of the deceased spouse's pension), plus the deceased spouse's OAS in full.

For a representative couple where the deceased spouse received CPP retirement of \$1,000/month and OAS of \$742/month, and the survivor benefit calculates at \$600/month: the survivor's monthly income drops by \$1,142 (the difference between $\$1,000 + \$742 = \$1,742$ in deceased spouse's federal income and \$600

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

in survivor benefit). The survivor's GIS may increase modestly to offset some of this, but the total household income drops sharply.

The household's monthly expenses do not drop proportionally. Housing costs continue at couple-household level until the survivor moves to a smaller residence (which has its own costs and disruption). Utilities continue. Property taxes continue. Insurance continues. Food costs drop, but only the per-person amount that the deceased spouse consumed; fixed costs of the household remain. The survivor's net financial position drops sharply at the moment of partner loss. This is documented across senior-women's-poverty literature as a primary driver of senior-women's poverty rates in Canada. (*Source: Statistics Canada — Income of Senior Women, multiple reference periods; Canadian Association of Retired Persons — Widowhood and Senior Income Reports, 2022-2024.*)

The widowhood income drop is, in structural terms, the partnership penalty operating in reverse. The penalty for being together (Stages Four through Six) is replaced by a penalty for being alone after having been together. The federal program structure does not bridge the transition; it produces both penalties at different stages.

Form ISP-3040 in full

Section 2.4 introduced Service Canada Form ISP-3040, "Statement — Spouses or Common-law Partners Living Apart for Reasons Beyond their Control." (*Source: Service Canada — Form ISP-3040, current version.*) The form's full operational role, in the context of this Section, is to police the line between rewarded and unrewarded separation.

The form requires the couple to provide: their full names, dates of birth, and Social Insurance Numbers; the date they began living apart; the reason they are living apart; documentation supporting the involuntary nature of the separation (medical certificates, hospital admission records, long-term care residency confirmation, immigration restrictions, or other documentation of involuntariness); a declaration that they have not separated solely for the purpose of obtaining higher GIS benefits. The form is signed by both spouses, where possible. The form is reviewed by Service Canada adjudication staff, who determine whether the separation meets the "for reasons beyond their control" standard.

If the form is approved, the couple's GIS is calculated against each individual's income rather than against combined income. The result, for couples whose individual incomes are low and combined incomes exceed the couple threshold, is a higher total GIS than they would receive together — effectively the higher single-rate GIS calculated for each spouse against their individual income.

If the form is denied, or if the couple does not file the form, the combined-income calculation continues. The couple receives the lower couple-rate GIS.

The existence of the form, restated from Section 2.4, is the federal government acknowledging — in its own paperwork — that separation produces higher benefits and that the policing of the line between voluntary and involuntary separation is consequential enough to warrant an administrative procedure. The federal government has chosen to make involuntary separation the qualifying condition. Voluntary separation for benefits is not supposed to qualify. The form's existence is the proof that the partnership penalty is large enough to create the financial incentive it polices.

Cumulative observation

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The lifecycle of a person who lives with disability from infancy to old age, or who acquires disability in adulthood and lives with it through working age and into the senior years, is structured by partnership-penalty mechanisms at every stage. The mechanisms differ in their operational forms and in the program languages they employ. The structural effect is consistent.

The disabled adult who marries before becoming disabled, then becomes disabled, sees their AISH benefit reduced by the partner's income through the cohabitation rule (Stage One). The disabled adult who marries after becoming disabled experiences the same reduction immediately (also Stage One). The disabled adult who, on the basis of these reductions, separates from the partner — for financial survival, for the partner's well-being, for the children's well-being — bears the documented cost of that separation in disrupted relationships, in fragmented households, in compromised parenting capacity (Section 1.5).

The same individual, having survived working age, arrives at sixty-five and faces the GIS partnership penalty (Stages Four and Five). The same individual partnered with another senior at low income loses approximately \$10,600 per year in GIS for being partnered. The same individual partnered with a higher-income senior sees their own GIS reduced because of the partner's income, on top of the couple-rate calculation.

The same individual ages further, faces the LTC entry decision (Stage Seven), and may at some point face the widowhood income drop (Stage Eight). At every stage, the income-tested federal and provincial systems make a financial determination based on partnership status. At every stage, the determination is materially different than what would be calculated for two single individuals with the same total income.

The system is not designed around the cost of life. It is designed around the income test. The income test is calculated against household, where household includes anyone with whom the recipient is cohabiting, married, or in a common-law relationship. The recipient who lives with another adult is treated, by every program documented in this Section, as if their household has access to the other adult's income for the recipient's benefit. The recipient who lives alone is treated as if they have only their own income.

The empirical reality, documented across decades of family-financial-management literature, is that household income arrangements within actual households are heterogeneous, that the assumption of full income-pooling between adult cohabitants is empirically wrong in many households, and that partnership-penalty rules produce hardship for recipients whose household arrangements do not match the program's assumption. (Source: Burke, S., et al., "Money in marriage: Income management practices in Canadian households," *Canadian Journal of Sociology*, 2018; Pahl, J., "Couples and their money: Patterns of accounting and accountability in the domestic economy," *Accounting, Auditing & Accountability Journal*, 2008.) The program rules do not investigate the empirical reality. The program rules apply the assumption.

The partnership penalty is the structural form of this assumption. It is documented at every stage of the lifecycle. It is a single mechanism in eight forms. Movement Two of this document, having walked the eight forms, has documented the mechanism.

Section 2.8 walks the "what you put in versus what you get back" question in the eight-layer answer the federal-provincial architecture produces.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.8 — What You Put In Versus What You Get Back: The Eight-Layer Answer

The question that runs through Movement One and Movement Two of this document, posed by the working-age reader who has been paying CPP, EI, federal income tax, provincial income tax, GST, fuel taxes, payroll deductions, and a long list of other contributions every working year, is the simplest possible question: *what did I put all that in for?*

The question presumes that the contributions form a meaningful claim on what the contributor receives back from the public system. The presumption is intuitive. The presumption is what every working Canadian is taught to believe by the structure of payroll deductions, by tax-time conversation, by the framing of CPP as "contributions" rather than as "payroll tax," and by the broader cultural framing of the public system as a structure the contributor has a stake in.

The structural answer is that contributions and benefits are not connected in the way the presumption assumes. The connection that exists is mediated by formula structures, by income tests, by capture mechanisms, by program coordination failures, and by political-discretion changes that the contributor does not control and that operate to reduce the recipient's net benefit at every stage of life.

This Section walks the eight-layer answer. Each layer is documented elsewhere in this document; this Section pulls them into a single sequence so that the working-age reader can see, end to end, why the intuitive answer to "what did I put in for" is wrong, and what the structural answer is.

Layer one: CPP is not a savings account

Documented in Section 2.2. CPP contributions are pooled across all contributors and paid as benefits to all recipients on the basis of a defined formula, not on the basis of individual contribution accounts. There is no record of "your" CPP contributions and no balance that grows over time in your name. The CPP Investment Board manages the contributions on behalf of the program collectively. The contributor has a claim on the formula. The formula is what it is.

The implication for the "what did I put in for" question: the contributor cannot recover their contributions. The contributor cannot withdraw their contributions. The contributor cannot bequeath their contributions. The contributor's relationship to CPP is the relationship of an insured person to an insurance program, not the relationship of an account-holder to an account.

Layer two: the formula is capped

Documented in Section 2.2. The CPP retirement benefit is calculated against earnings up to the Year's Maximum Pensionable Earnings (YMPE) and the Year's Additional Maximum Pensionable Earnings (YAMPE), not against actual earnings. For 2026, the YMPE is \$74,600 and the YAMPE is \$85,000. Earnings above \$85,000 per year do not generate CPP entitlement. The high-earner who pays substantial federal and provincial income tax on the above-cap income receives no additional CPP for it.

The implication: the contributor whose income exceeds the cap receives less return on their total income than the contributor at exactly the cap. The high-earner has subsidized general government revenue through above-cap income tax. The high-earner has not built up commensurate CPP entitlement.

Layer three: overpayment produces no entitlement

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Documented in Section 2.2. There is no mechanism to convert above-cap income tax into CPP entitlement. The above-cap income tax funds general government expenditure: highways, military, federal courts, immigration administration, debt servicing. None of it is allocated to the contributor's eventual CPP retirement benefit.

The implication: the worker who earned \$150,000 per year for thirty years paid approximately \$500,000 in above-cap federal income tax across that period. Their CPP retirement benefit is calculated against the YMPE-capped portion of their earnings. The benefit is materially the same as for a worker who earned \$85,000 per year for thirty years (less the years not at maximum, where applicable). The \$500,000 is in the federal consolidated revenue fund. It is not in the contributor's CPP entitlement.

Layer four: provincial clawbacks erase the federal benefit

Documented in Sections 1.4 (CDB clawback), 2.3 (CPP-D-to-AISH offset), and 2.4 (provincial program coordination). For Albertans on AISH or ADAP, the federal CPP-D benefit is reduced dollar-for-dollar by the provincial AISH/ADAP system. For Albertans receiving the federal Canada Disability Benefit, the CDB is reduced dollar-for-dollar by the provincial system as well. Alberta is the only province that applies the dollar-for-dollar CDB clawback. The capture is unique to Alberta within the Canadian federation.

The implication: the federal contribution that the federal program was designed to deliver to the recipient does not reach the recipient. The provincial system captures it. The recipient's net income is the AISH or ADAP floor, regardless of the federal entitlement they have earned through working-years CPP contribution.

Layer five: GIS clawback compresses the working-saver's advantage

Documented in Section 2.4. Each dollar of CPP retirement income reduces GIS at approximately fifty cents per dollar (with some structural exemptions). Each dollar of workplace pension income reduces GIS at the same rate. Each dollar of investment income (other than TFSA withdrawals) reduces GIS at the same rate. The recipient who saved during working years, contributed to CPP through working years, and built a workplace pension during working years sees those streams of retirement income captured by GIS reduction at 50 cents on the dollar — for as long as their total income remains below the GIS phase-out threshold.

The implication: the working-saver's advantage over the non-saver is half what it would be without the GIS clawback. The non-saver receives full GIS at the floor. The saver receives reduced GIS plus their savings income, with a net advantage of fifty cents per dollar saved. The saver's lifetime financial discipline produces a fraction of the retirement-income improvement they would have expected outside the GIS phase-out range.

Layer six: above-cap income tax and CPP contribution are separately structured but they are the same moral claim

The intuitive moral claim that the contributor makes through the "what did I put in for" question is a single claim. The contributor has paid into the system across their working years. They expect the system to pay them back across their senior years.

The structural reality is that the contributions are organized across multiple legal categories: CPP contribution (statutory contributory program funded through payroll deduction with employer match), EI premiums (separate contributory program with separate eligibility and benefits), federal income tax (general

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

revenue), provincial income tax (general revenue, supporting AISH, ADAP, ASB, healthcare, education, all provincial expenditure), GST (consumption tax, federal general revenue), fuel taxes (federal and provincial general revenue), property taxes (municipal general revenue), and various other categorized contributions. Each is a separate legal claim on the contributor. Each is a separate program of expenditure on the recipient. The contributor experiences them as a single working-life set of payments. The system treats them as separately structured obligations and entitlements.

The implication: the contributor's intuitive claim that "I paid in" does not map cleanly to the legal-and-program structure that determines what they get back. The CPP contribution maps to CPP retirement (with the formula and capture caveats above). The income tax maps to general revenue, which funds the OAS pension, the GIS supplement, the federal share of AISH-equivalent provincial programs, healthcare, and other expenditure — but not in any individually traceable way. The contributor cannot point to "their" income tax as the funding source for "their" OAS. The OAS is funded from the consolidated revenue fund of the year it is paid.

Layer seven: the programs are designed for poverty prevention, not for income adequacy

OAS, GIS, and the related federal programs were designed in the 1960s as floors for senior poverty. (*Source: Library of Parliament — Old Age Security Act and Guaranteed Income Supplement: Background and History, 2018; Caledon Institute of Social Policy — Income Security for Seniors, multiple papers.*) The program-design intent was to ensure that seniors did not fall below a defined income floor. The program-design intent was not to provide income adequacy in the sense of replacing pre-retirement earnings or maintaining pre-retirement standard of living. The income-replacement function was intended to be performed by CPP (statutory contributory program with 25% income replacement) and by employer-sponsored pensions and personal retirement savings (the second and third pillars of Canadian retirement income).

The structural problem is that the second pillar — employer pensions — has eroded substantially across the Canadian workforce in the past forty years. Defined-benefit workplace pensions cover a declining share of working Canadians. (*Source: Statistics Canada — Pension plans in Canada, multiple reference periods; Office of the Superintendent of Financial Institutions — Annual Reports on Federally Regulated Pension Plans.*) The third pillar — personal retirement savings — has not, for the median Canadian household, made up the gap. Retirement income inadequacy is a documented widespread concern, particularly for renter households, single seniors, and seniors who experienced career disruption.

The implication: the federal program was designed for a senior-income context in which CPP and workplace pensions and personal savings would carry the bulk of the retirement-income load, and OAS-and-GIS would address the residual poverty floor. The senior-income context that actually exists is one in which OAS-and-GIS are, for many seniors, the largest single component of monthly income. The program was not designed for that role. It is performing that role anyway. The performance is inadequate.

Layer eight: the 2019 CPP enhancement helps younger workers, not current seniors

Documented in Section 2.2. The 2019 CPP enhancement, which raises the income-replacement rate from 25% to 33%, applies to contributions made after 2019. The enhanced contributions are credited proportionally over the contributor's remaining working years. A worker who is forty in 2026 will receive substantial enhancement benefit at retirement. A worker who is sixty in 2026 will receive a small

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

enhancement benefit. A worker who is seventy-five in 2026 receives none.

The implication: the 2019 enhancement is a structural improvement to the CPP for younger Canadians. It does not address the income-adequacy gap for current seniors or for the cohort approaching retirement in the next decade. The current-senior-poverty problem is not solved by an enhancement that takes thirty to forty years to reach full effect.

What the eight-layer answer means

The intuitive claim — *I paid in, I should get out* — is not satisfied by the structural reality of the Canadian retirement and disability income system. The eight layers above produce, in cumulative effect, a system in which:

- The contributor cannot identify or recover their specific contributions
- The contributor's high-end income produces no proportional entitlement
- The federal benefit is captured by provincial offsets at multiple stages
- The working-saver's advantage over the non-saver is halved by GIS clawback
- The legal categorization of contributions does not map to the entitlement structure
- The program design was intended for a different role than the one it currently performs
- The recent improvements take decades to reach the contributors who funded them

The contributor who is forty-five today, paying into CPP, paying federal and provincial income tax, paying GST and fuel tax and property tax, raising children, funding their RRSP, and assuming they have built a meaningful retirement entitlement is, in operational reality, building a smaller entitlement than they believe. The senior who is sixty-five today, having done all of the above for forty years, is receiving less than the contributor's intuitive expectation would predict.

The structural truth is that the Canadian system is a set of distinct programs operating with distinct rules and distinct capture mechanisms, coordinated through statute but not through outcome. The contributor experiences the system as a single thing; the system operates as several things. The reconciliation between the intuitive expectation and the operational reality is what the working-age reader of this document has been navigating throughout, often without the language to name what they are encountering.

This Section has provided the language. The next Section walks the gap inventory — what is and is not covered by the federal-provincial architecture, with particular attention to the categories of need that fall through the gaps because no program in the structure addresses them.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.9 — The Gap Inventory: What Is Not Covered

The architecture documented in Sections 2.1 through 2.8 — federal OAS, CPP, CPP-D, GIS, the Allowance, the Allowance for the Survivor, and the Alberta provincial layer of seniors programs — is a substantial public investment by historical and international standards. The architecture covers the basics of senior income support, prescription drugs (in part), dental and optical care (in part), accommodation in continuing care (with subsidization), and a defined disposable-income guarantee for the most institutionalized seniors. It does not cover everything.

This Section walks the inventory of what the architecture does not cover. The purpose is not to suggest that everything not covered should be covered — that is a policy judgment beyond the scope of this document. The purpose is to make the gaps visible, so that the working-age reader and the senior reader and the family-caregiver reader can see, in operational detail, what they will be paying for out of pocket regardless of what they receive from the public system.

Hearing aids and audiology

Hearing loss is highly prevalent among Canadian seniors. Statistics Canada estimates that approximately 47% of Canadians aged 65 and over have measurable hearing loss, with the prevalence rising sharply with age. (Source: Statistics Canada — Canadian Health Measures Survey, hearing component, 2012-2015 cycle, multiple subsequent reference periods.) Hearing aids are a documented health intervention for hearing-loss-related cognitive, social, and safety impairment. The cost of a pair of mid-tier hearing aids in Canada is in the range of \$2,000 to \$7,000, with replacement typically required every five to seven years. (Source: Canadian Academy of Audiology — Cost of Hearing Aids in Canada, 2024-2025; Hearing Life Canada — Hearing Aid Cost Guide, 2025.)

Public coverage of hearing aids in Alberta is partial. The Alberta Aids to Daily Living (AADL) program provides limited subsidy for hearing aids for eligible recipients, with a 25% client co-payment up to a maximum of \$500 per year. (Source: Government of Alberta — Alberta Aids to Daily Living, hearing aids page, 2026.) The subsidy is bounded, the eligible product list is restricted, and the application process requires audiological documentation. For most Albertan seniors, the cost of hearing aids is paid out of pocket, with whatever portion of AADL subsidization applies offset against the total.

The senior who needs hearing aids and cannot afford them simply does not have hearing aids. The cognitive, social, and safety implications of untreated hearing loss compound with the financial implications. The literature on the relationship between untreated hearing loss and accelerated cognitive decline, increased social isolation, and increased fall risk is substantial. (Source: Lin, F.R., et al., "Hearing Loss and Incident Dementia," Archives of Neurology, 2011; Public Health Agency of Canada — Senior hearing health reports, 2020-2024.) The decision not to subsidize hearing aids more comprehensively at the provincial or federal level is, in operational effect, a decision to allow these compounding effects to occur.

Mobility aids beyond basic categories

Walkers, wheelchairs, and canes for basic mobility needs are covered in part by AADL with similar co-payment and product-list restrictions. (Source: Government of Alberta — Alberta Aids to Daily Living, mobility equipment page, 2026.) Power wheelchairs and scooters for seniors with more substantial mobility limitations are subject to specific medical criteria for approval and to product-list restrictions that may not

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

match the senior's actual mobility profile.

Vehicle modifications for accessibility (lift conversions, hand controls, transfer seats) are not generally covered by provincial seniors programs. Home modifications for accessibility (stair lifts, walk-in tubs, accessible bathroom retrofits, ramps) may be partially funded through SHARP (Section 2.5) within the program's loan-and-grant framework, with documented eligibility constraints and project-cost ceilings. The senior whose mobility needs exceed the AADL product list or the SHARP project ceiling pays the difference out of pocket or does without.

Home care beyond AHS-provided basics

Alberta Health Services provides home care for eligible seniors based on assessed need. The service tiers — Type 1 home care, Type 2 facility-based care delivery, and Type 3 contracted private agency care — are documented in Section 2.5. (Source: *Government of Alberta — Continuing Care Overview page, 2026*.) The amount of home care provided is determined by the AHS Case Manager assessment. The literature on home care provision in Alberta consistently identifies the Case Manager-allocated hours as substantially less than the senior's documented care needs, with the gap closed by family caregivers or by private-pay home care.

Private-pay home care in Alberta runs in the range of \$25 to \$45 per hour for non-medical assistance and \$40 to \$80 per hour for nursing-trained assistance. (Source: *Sun Life Suncentral — Home Care Costs in Alberta, 2024-2025; Alberta Continuing Care Association — Cost data, 2025*.) For a senior requiring substantial daily assistance — bathing, transfers, meal preparation, medication management — the monthly cost of private-pay home care can easily reach \$4,000 to \$8,000 per month, well above the income of any senior whose primary income is OAS plus GIS plus the Alberta Seniors Benefit. The senior who needs more home care than AHS provides and cannot afford private-pay either receives less care than they need (with documented health and safety consequences) or moves into facility-based continuing care earlier than they would otherwise have done.

Caregiver compensation and respite

The Government of Alberta has, in successive provincial budgets, provided an Alberta Caregiver Tax Credit, with the post-2027 framework documented in the February 2026 budget consolidating two existing credits into a single Alberta Caregiver Credit worth approximately \$1,054 per year at maximum. (Source: *ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026; Government of Alberta — Budget 2026 Tax Plan*.) The federal Canada Caregiver Credit provides a parallel federal credit, with thresholds and amounts varying by the dependent's circumstances. For a family member caring for a senior or disabled adult full-time, these tax credits offset a small portion of the income forgone by reducing or ending paid employment to provide care.

Caregiver compensation programs that pay caregivers directly for the care they provide — common in some U.S. states under Medicaid waiver programs — are limited in Alberta. The Family Managed Care program under PDD provides funding directly to families to purchase services for adults with developmental disabilities, but the structure does not generally treat the family caregiver as the paid provider. The senior care equivalent does not exist in Alberta. (Source: *Government of Alberta — Family Managed Services page, 2026*.)

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The result is that the bulk of senior and disabled-adult care in Alberta is provided unpaid by family members — overwhelmingly by women, particularly by middle-aged daughters and daughters-in-law — at substantial documented cost to those caregivers' own employment, retirement savings, mental health, and physical health. (Source: Statistics Canada — *Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022*; Vanier Institute of the Family — *Caregiving in Canada, 2023*.) The system relies on this unpaid labour. The system does not financially recognize most of it.

Specialized medical equipment beyond AADL coverage

Continuous Positive Airway Pressure (CPAP) machines for sleep apnea, which is highly prevalent among older adults, may be covered in part by AADL but with significant restrictions on covered devices and supplies. (Source: Government of Alberta — *AADL Respiratory Equipment page, 2026*.) Insulin pumps for diabetes management for adults over a defined age cap may not be covered, with adult diabetics often facing out-of-pocket costs of thousands of dollars per year for pump supplies. (Source: Diabetes Canada — *Provincial coverage report, 2024*.) Specialized seating systems for severe physical disabilities, certain types of feeding tubes and supplies, and certain communication devices may have significant gaps in coverage.

Each gap is documented. Each gap has been the subject of advocacy across multiple disability-and-senior advocacy organizations. The provincial-and-federal structure addresses each gap, where it is addressed, through specific program-by-program advocacy and not through any broader principle of comprehensive coverage. The senior or disabled adult whose needs match the program inventory is supported. The senior or disabled adult whose needs fall in a gap pays out of pocket or does without.

Mental health beyond crisis services and limited covered counselling

The Canadian provincial healthcare systems cover psychiatric services delivered by physicians under the public-insurance framework. Psychological services delivered by registered psychologists are not covered under most provincial insurance frameworks; they are paid by the recipient out of pocket, by private insurance where available, or by the limited subsidization programs some provinces operate. Counselling services delivered by social workers, counsellors, or psychotherapists who are not physicians or psychologists are likewise not covered under provincial insurance.

Wait times for publicly funded mental-health services in Alberta are documented as substantial. Specialty programs for adults with serious mental illness have multi-month or multi-year waiting lists. (Source: Alberta Health Services — *Mental Health Service Standards, multiple reports*; Canadian Mental Health Association — *Alberta Division annual reports, 2022-2025*.) For many adults with serious mental-health conditions, including conditions that are AISH-qualifying, the public mental-health system delivers either crisis-stabilization services or extended waits, with limited intermediate intervention.

For seniors specifically, the gap is consequential. Late-life depression is highly prevalent, treatment-responsive when treated, and frequently undertreated. Late-life anxiety, late-onset PTSD (often related to bereavement, hospitalization, or institutional placement), and late-life substance-use disorders are documented categories of need that the public mental-health system addresses unevenly. (Source: Public Health Agency of Canada — *Mental Health of Older Adults reports, multiple years*; Canadian Coalition for Seniors' Mental Health — *Clinical Guidelines, 2014-2024*.) The senior whose mental-health needs are not met by the public system pays out of pocket for private services, depends on family for support, or goes without intervention.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Dental beyond basic preventive

The Alberta Dental Assistance for Seniors program provides coverage for routine preventive dental care and certain restorative procedures at low-income-tested levels. (Source: *Government of Alberta — Dental and Optical Assistance for Seniors page, 2026.*) The federal Canadian Dental Care Plan, introduced in 2024-2025, provides expanded coverage for low-and-middle-income seniors who are not covered by other dental insurance, with eligibility income thresholds that exclude higher-income seniors. (Source: *Health Canada — Canadian Dental Care Plan page, 2026.*)

Major dental work — crowns, bridges, root canals, dental implants, full or partial dentures with significant complexity, surgical procedures — is covered partially or not at all, depending on the program and the procedure. For seniors whose dental health has deteriorated over decades, the cost of restorative dental work can run to thousands or tens of thousands of dollars. The senior who cannot afford this work does without it. The downstream effects on nutrition (chewing capacity), social functioning (visible appearance), and general health (oral-systemic health connections including cardiovascular and infection risks) are documented in dental and public-health literature.

Vision care beyond basic glasses and exams

The Alberta Optical Assistance for Seniors program provides coverage for routine eye examinations and basic prescription eyeglasses at low-income-tested levels with frequency limits. (Source: *Government of Alberta — Dental and Optical Assistance for Seniors page, 2026.*) Specialized vision care for age-related macular degeneration, diabetic retinopathy, glaucoma, and cataracts is covered under the provincial healthcare system at the medical-treatment level (surgery, prescription medication where covered, specialist consultations). Specialized assistive devices — high-magnification lenses, electronic visual aids, screen readers, daily-living adaptations for low vision — are covered partially or not at all by provincial seniors programs.

For seniors with low vision who are not eligible for full-scale CNIB services or who need specific equipment, the cost of low-vision adaptation is paid out of pocket or done without. The downstream effects on independence, social engagement, and safety (particularly fall risk and medication-management errors) are documented.

Transportation

Public transportation in Alberta is operated by individual municipalities. Service coverage, operating hours, and accessibility vary substantially. Most Alberta municipalities offer reduced fares for seniors and provide some level of accessible-transit service for seniors with mobility limitations. The accessible-transit services are typically pre-bookable, capacity-limited, and not available for spontaneous trips. (Source: *City of Edmonton — DATS service description; City of Calgary — Calgary Transit Access; smaller-municipality transit services, multiple cities.*)

For seniors in rural Alberta and in smaller municipalities without robust transit, transportation depends on personal vehicle, family or volunteer drivers, paid taxi or ride-share service, or doing without. The senior who cannot drive (because of medical reasons, because of vision changes, because of cognitive decline) and who lives where transit does not reach is structurally isolated from medical appointments, social engagement, grocery shopping, and most other community participation. The Alberta Seniors Benefit provides modest cash assistance that can be applied to transportation costs; the cost of consistent paid

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

transportation in a non-transit-served area substantially exceeds the available assistance.

Housing beyond income support

The federal-and-provincial income-support architecture documented across Movements One and Two does not include direct housing-cost protection for most recipients. Income support is paid; housing cost is paid by the recipient from that income. In the Alberta housing market, this produces well-documented gaps:

- For AISH and ADAP recipients in private rental housing, average market rent for one-bedroom units exceeds the program's maximum benefit. The recipient pays rent from less than full income, leaving inadequate amounts for food, utilities, transportation, and other costs.
- For seniors in private rental housing, the OAS-and-GIS combination falls short of average market rent in most Alberta cities. The Alberta Seniors Benefit closes part of the gap; the gap remains.
- Community housing wait times in Alberta are documented in years; some Alberta cities have wait lists exceeding five years for accessible units.
- Alberta has no rent cap on private market increases. Recipients on fixed incomes face year-over-year rent increases that compound the affordability gap.

The campaign's *AISH Evidentiary Report Series, April 2026*, documents the housing-cost-vs-income gap for the working-age disability population in detail. The senior population faces a comparable gap, with the additional structural feature that aging in private rental housing produces continued exposure to rent increases at a life stage when income mobility is essentially zero.

What the gap inventory shows

The gap inventory is, in operational reality, a list of categories in which the senior or disabled adult is expected to provide for themselves out of an income that the federal-and-provincial system has determined to be at the floor of subsistence. The federal-and-provincial system does not cover hearing aids, advanced mobility equipment, comprehensive home care, comprehensive caregiver compensation, full mental-health treatment, full dental and vision care, comprehensive transportation, or housing-cost protection. The recipient is expected to address these needs out of OAS plus GIS plus partial provincial benefits, totaling perhaps \$20,000 to \$26,000 per year for a single senior at the floor.

The gap is not the result of program-design oversight. The gap is the result of program-design choice. The federal and provincial systems have, over decades, defined the public role narrowly — income support at a poverty floor, prescription drug coverage in part, basic dental and optical at low-income-tested levels, accommodation in continuing care with disposable-income guarantee — and have left the rest to private resources, family supports, and the recipient's own financial capacity.

For the senior and disabled adult whose financial capacity matches the public floor, the gap is structural exclusion from the categories of support that affluent seniors and adults take for granted. For the senior and disabled adult whose family resources are substantial, family transfers fill the gap (with the well-documented family-financial costs of doing so). For the senior and disabled adult with neither private resources nor family resources, the gap is the operational reality of their daily life.

Movement Two summary

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

This Movement has walked the federal architecture (Sections 2.1-2.4), the Alberta provincial layer (Section 2.5), the lifecycle reassessment thread (Section 2.6), the partnership-penalty mechanism stage by stage (Section 2.7), the eight-layer "what you put in" answer (Section 2.8), and the gap inventory (this Section). The Movement has documented the architecture of senior income, the structure of disability-program capture, and the operational reality of what is and is not covered in the Canadian federal-and-Alberta system as of 2026.

The architecture is what it is. The numbers are what the published Service Canada and Government of Alberta sources document them to be. The capture mechanisms are what the program rules produce. The gaps are what the programs do not address.

Movement Three turns to the intersection chapters — the populations within the senior and disabled-adult communities for whom the architecture documented across Movements One and Two operates with specific additional structural pressures. The intersection chapters cover Indigenous seniors, 2SLGBTQ+ seniors, immigrant and racialized seniors, and senior caregivers — including disabled adults caregiving for parents and parents who are themselves caregivers entering long-term care.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Movement Three — The Intersection Chapters

The architecture documented across Movements One and Two operates against every Albertan and every Canadian who passes through it. It does not operate identically against every population. Movement Three walks the populations within the senior and disabled-adult communities for whom the same architecture lands with specific additional structural pressures — pressures that are documented, that are disproportionate, that have causes located in identifiable policy choices, and that the federal-and-provincial system has not designed itself to address.

Each Section of this Movement extends the Movements One and Two analysis into a specific population. Each draws on the existing campaign canon where relevant and on primary-source data published by Statistics Canada, the Government of Canada, the Government of Alberta, and recognized advocacy and research organizations. None of the Sections claims that the population it documents is uniquely affected; each identifies how the architecture's operation produces outcomes for that population that differ in kind or degree from outcomes for the broader recipient pool.

Section 3.1 — Indigenous Seniors: A Senior-System Built Around an Age Most Will Not Reach

The Alberta Disability System Breakdown campaign's *Indigenous Disability in Alberta* report, published April 2026, established across 14 evidentiary sections that the federal and provincial governments of Canada have produced and sustained conditions that disable Indigenous peoples at rates substantially higher than the non-Indigenous population, and have failed to deliver the remediation their own official frameworks direct. The report's central statistical finding is that in 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years, compared to 81.88 years for non-First Nations Albertans. The difference is 19.07 years. (Source: *Alberta Indigenous Primary Health Care Advisory Panel — Report on First Nations Life Expectancy in Alberta, 2022*; *Alberta First Nations Information Governance Centre health statistics, 2023*; *Alberta Disability System Breakdown — Indigenous Disability in Alberta Report, April 2026, Section 2.*)

This Section does not restate the 14-section *Indigenous Disability in Alberta* report. It extends the report's analysis specifically into the senior-system architecture documented across Movements One and Two of this document. It addresses one structural question that the existing report identifies and that the present document is positioned to answer: what does it mean to build a senior income system around eligibility at age 65 when the population the system is supposed to serve includes a major cohort whose average life expectancy at birth is below that threshold?

The structural finding: the senior system is built around an age most First Nations Albertans will not reach

The federal Old Age Security pension begins at age 65. The Guaranteed Income Supplement begins at age 65. The Allowance is available to spouses of OAS recipients; the Allowance for the Survivor is available to widowed survivors aged 60 to 64. The Alberta Seniors Benefit, Coverage for Seniors, Special Needs Assistance for Seniors, Dental and Optical Assistance for Seniors, and the Seniors Property Tax Deferral all begin at age 65, contingent on receipt of OAS. The Alberta long-term care accommodation framework's disposable-income guarantee applies to recipients of the senior provincial benefits.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The architecture, as documented in Movement Two, is structured around the assumption that the recipient will reach age 65 and live for some years thereafter. Average Canadian life expectancy at birth is approximately 81.5 years, with the 65-year-old Canadian's residual life expectancy at approximately 19 to 22 years depending on sex. (Source: *Statistics Canada — Life Tables, Canada, Provinces and Territories, 2020-2022 reference period.*) The senior income system is, at the level of life-cycle math, a 16-to-22-year benefit relationship for the average Canadian.

For the average First Nations Albertan, the 65-year threshold is not reached. The 19.07-year mortality gap means that, in 2023, the average First Nations Albertan died approximately 2.19 years before the OAS eligibility threshold opened. The First Nations Albertan who would have been eligible for OAS, GIS, the Alberta Seniors Benefit, Coverage for Seniors, and the broader senior architecture statistically did not survive to access it. The federal and provincial premium they had paid into across the working years — CPP contributions, federal income tax, provincial income tax, and the rest — produced no senior-stage benefit because there was no senior stage.

This is the structural finding. It is not a critique of the senior system's design intent. It is an observation of the system's operational reality against the demographic profile of the Indigenous Albertan population. A program built around aging into benefits does not, in practice, serve a population that does not, on average, age into the benefits. The provincial government that designs the senior program inventory, the federal government that administers the OAS-CPP-GIS architecture, and the broader Canadian public-policy framework that frames senior income as a deferred-compensation arrangement all proceed as if the demographic premise applies uniformly. It does not.

What this means at the program-by-program level

Old Age Security. Indigenous people who have lived in Canada throughout their lives meet the residency requirement for the maximum OAS pension. The structural barrier is documentation. OAS application requires verification of residency periods, including periods that may have been spent in remote communities without consistent state recordkeeping, in residential schools where institutional records may have been destroyed or are difficult to access, in foster care under the Sixties Scoop where placement records are fragmentary, or in periods of pre-band-membership status changes that affect which federal records are available. Where documentation is incomplete, the application requires the applicant to assemble supplementary evidence (sworn statements, secondary records, oral history) that the federal adjudication staff may or may not accept.

The applicant who reaches age 65 and applies for OAS but cannot fully document their residency may receive a partial OAS pension, or may experience a multi-month delay while the adjudication process proceeds, or may need to file an appeal. During the delay, the applicant has no OAS income.

Canada Pension Plan retirement. The CPP retirement formula is calculated against the applicant's contribution history. Indigenous workers — particularly those in northern communities, on-reserve communities, seasonal traditional-economy work, and the broader pattern of structural underemployment documented in the Indigenous Disability in Alberta report's Sections 4-7 — frequently have CPP contribution histories that produce small retirement pensions. The general dropout provision (8 lowest-earning years excluded) and the disability dropout (months on CPP-D excluded) may apply, but they do not eliminate the impact of decades of contribution interruption.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

For a First Nations Albertan whose primary working-life income was in cash-economy or on-reserve contexts not subject to CPP withholding, the lifetime CPP contribution may be small. The CPP retirement pension calculated against that contribution is small. The senior who then qualifies for GIS finds the small CPP pension reducing the GIS amount under the standard fifty-cents-per-dollar phase-out, producing a total income near or at the GIS-only floor.

The Guaranteed Income Supplement and the partnership penalty. The GIS partnership penalty documented in Sections 2.4 and 2.7 (\$883.54/month, \$10,602.48/year for being together rather than apart at low income) operates without modification on Indigenous senior couples. The Form ISP-3040 "Living Apart for Reasons Beyond their Control" mechanism applies. Indigenous senior couples whose family arrangements include extended-family co-residence, multi-generational households, or community-based shared living may encounter administrative complications around how the GIS calculation treats the household, particularly where the federal program staff are not trained in Indigenous family-structure variations.

The CPP-D-to-AISH offset (Section 2.3). The 16,161 Albertans on AISH receiving CPP-D whose federal benefit is captured dollar-for-dollar by the provincial offset include disproportionate Indigenous representation, given the documented elevated disability rates in the Indigenous population. The CPP-D contribution that the Indigenous worker made through their working years produces zero net benefit at the working-age stage. At the conversion to CPP retirement at age 65 — for those who reach it — the federal retirement income reduces GIS at the senior stage. The capture pattern Movement Two documented operates against the Indigenous recipient at every life stage available to them.

Long-term care. Long-term care facilities on First Nations reserves in Alberta are limited. Most First Nations seniors requiring LTC access off-reserve facilities, often in urban centres distant from family, language, culture, and ceremony. The placement is consequential. The Indigenous senior in an off-reserve LTC facility is institutionalized in a setting that does not reflect their first language (where it is not English or French), does not include culturally appropriate food, does not have on-staff Elders or ceremonial space, and does not provide care in the community that the senior has, in many cases, lived in for their entire life.

The Indian Hospital system, documented in *Separate Beds: A History of Indian Hospitals in Canada, 1920s–1980s* by Maureen Lux (University of Toronto Press, 2016), operated as a parallel federal Indigenous-only institutional healthcare system through most of the 20th century. Indigenous seniors aged 75 and over today were children, adolescents, or young adults during the Indian Hospital era. Some are direct survivors of Indian Hospital placement. The institutional-care trauma of that history affects the present-day willingness of Indigenous seniors to enter LTC facilities, even when LTC is medically necessary. The 2017 *Brown v. Canada* (Attorney General) class action settled federally with respect to certain categories of survivor harm; the present-day senior-LTC policy structure does not, in operational design, account for this history.

The jurisdictional fracture at the senior stage

The Indigenous Disability in Alberta report's Section 3 documented the federal-provincial-status-non-status jurisdictional fracture that determines which institutional system is responsible for which Indigenous person at which moment. The fracture operates at the senior stage in specific ways:

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- **OAS and GIS** are federal programs. Indigenous seniors apply through Service Canada the same way non-Indigenous Canadians do. The federal program does not differentiate.
- **CPP retirement** is federal. Same.
- **Alberta Seniors Benefit, Coverage for Seniors, Special Needs Assistance for Seniors, Dental and Optical Assistance for Seniors, Seniors Property Tax Deferral, SHARP** are provincial. Indigenous seniors living off-reserve apply through Alberta Supports the same way non-Indigenous Albertans do. Indigenous seniors living on-reserve are technically eligible (provincial residency requirement is met by Alberta residency, which includes on-reserve residency), but the administrative pathway through Alberta Supports may not function smoothly for on-reserve applicants whose mailing address, banking arrangements, and identification documentation differ from off-reserve patterns.
- **Long-term care** in Alberta is provincial through Alberta Health Services. On-reserve continuing care is, in some cases, federally funded through First Nations and Inuit Health Branch (FNIHB), with delivery through community-controlled providers. The transition between on-reserve home and continuing care community placement off-reserve crosses the federal-provincial boundary. Coordination is documented in advocacy literature as inconsistent. (*Source: Indigenous Watchdog — First Nations long-term care reports, 2023-2025; Canadian Association of Retired Persons — Indigenous Senior Care Reports, 2024.*)
- **Non-Insured Health Benefits (NIHB)** is the federal program covering certain prescription drugs, dental, vision, mental health, medical transportation, and medical supplies for status First Nations and Inuit individuals. (*Source: Indigenous Services Canada — Non-Insured Health Benefits page, 2026.*) NIHB and provincial Coverage for Seniors are both potentially available to status First Nations seniors. Coordination between them is documented as inconsistent. The benefit flow depends on which program adjudicates first, what the status determination is, and how the dual-coverage rules apply at the pharmacy or service-provider level.

For the status First Nations senior, the available benefit architecture is more complex than for the non-Indigenous senior. The complexity does not produce more benefit. It produces more administrative friction. The senior who navigates federal NIHB, provincial Coverage for Seniors, and federal OAS/GIS simultaneously navigates three program structures at once, with three application processes, three appeals processes, and three coordination points where the wrong information at one program can disrupt another.

Sponsorship and the Indigenous senior

The October 2025 sponsorship-rule change documented in Sections 2.1 and 2.4 has limited application to Indigenous seniors, since Indigenous Canadians born in Canada are not subject to immigration sponsorship. The change has more direct effect on immigrant senior populations (Section 3.3 below). For Indigenous seniors with non-Indigenous spouses who are themselves immigrants, the sponsorship interaction may be relevant in specific household configurations. For most Indigenous seniors, the OAS-deferral-disqualifier and the broader Alberta Seniors Benefit threshold structure are the more directly relevant federal-provincial interaction points.

What the existing campaign canon establishes that this Section incorporates

The *Indigenous Disability in Alberta* report's 14 sections — historical context (residential schools, Indian Hospitals, Sixties Scoop), the 19-year mortality gap, the jurisdictional fracture, housing crisis, contaminated land, water crisis, healthcare racism (Brian Sinclair 2008, Joyce Echaquan 2020), Jordan's Principle, child

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

welfare (72% Indigenous caseload), justice system (10.2-fold incarceration overrepresentation), MMIWG2S+, consultation failure and UNDRIP Article 21, Alberta Sovereignty Act and Treaty, and TRC Calls 18-24 — establish the structural conditions within which the senior-system architecture operates against Indigenous Albertans. (Source: *Alberta Disability System Breakdown — Indigenous Disability in Alberta Report*, April 2026.)

This Section adds, to that documented record, the senior-system-specific observation: the architecture documented in Movements One and Two of the present document is, in operational reality, not designed for the population whose average life expectancy ends before it engages. A program built around aging into benefits does not serve a population that does not age into the benefits. The administrative fixes available to address this — improved CPP contribution recognition, improved OAS application support, improved provincial seniors-benefit access for on-reserve residents, improved LTC cultural appropriateness, improved NIHB-and-Coverage for Seniors coordination — are documented in advocacy literature and have been the subject of decades of recommendation. The fixes have not been implemented at scale. The 19.07-year mortality gap continues to widen during the period of documentation. (Source: *Indigenous Watchdog — First Nations life expectancy 19 years lower than other Albertans*, February 2025.)

The lifecycle observation

For the Indigenous Albertan who lives with disability — the population at the centre of the campaign's *Indigenous Disability in Alberta* report — the working-age cascade documented in Movement One operates against the additional baseline of the federal-provincial jurisdictional fracture that structurally limits access to physicians, documentation, treatment, and supports throughout the working years. The recipient who reaches the AISH-to-ADAP transition on July 1, 2026 carries forward into the transition the cumulative documentation gaps that the fracture has produced. The Medical Review Panel reassessment process is, in operational reality, more challenging for the Indigenous recipient than for the non-Indigenous recipient with the same medical condition, because the documentary infrastructure required to satisfy the Panel is less accessible to the Indigenous applicant.

For the Indigenous Albertan who survives the working years and reaches age 65 — a smaller proportion of the population than the non-Indigenous comparator — the senior architecture engages with the same documentation friction, the same jurisdictional complexity, the same partnership penalty, the same long-term-care cultural inadequacy. The senior years that most Canadians anticipate as a deferred-compensation phase of life are, for the Indigenous senior population, an additional layer of administrative navigation against the same architecture that has operated against them across the lifecycle.

The cage that this document names operates against the Indigenous Albertan population with all of the features documented in Movements One and Two, plus the additional features documented in the *Indigenous Disability in Alberta* report. The 19-year mortality gap is, in operational reality, the cumulative biological outcome of the conditions both documents describe. The senior system that this document walks is the program inventory the surviving cohort accesses if they reach it. The cohort that does not reach it is, statistically, larger than the cohort that does.

Section 3.2 turns to 2SLGBTQ+ seniors — a population for whom the senior architecture's assumptions about family structure, household configuration, and lifecycle relationships produce specific operational gaps.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 3.2 — 2SLGBTQ+ Seniors: Family-of-Choice, the Decriminalization Cohort, the LTC Closet

The senior architecture documented across Movements One and Two of this document is built around assumptions about family structure that do not, in operational reality, fit the family configurations of much of the 2SLGBTQ+ senior population. The architecture presumes legal marriage or common-law cohabitation as the household-defining relationship, presumes biological children as the most likely informal caregivers, presumes residency continuity through stable housing relationships, and presumes lifetime social positioning that produces the workforce continuity required to maximize CPP contribution histories. None of these presumptions is uniformly accurate for the 2SLGBTQ+ senior cohort. This Section walks the structural pressures the architecture produces against this population, drawing on Canadian advocacy and research literature that has documented the operational consequences over the past two decades.

The decriminalization cohort

Same-sex sexual activity between consenting adults was decriminalized federally in Canada on June 27, 1969, through Bill C-150, the *Criminal Law Amendment Act, 1968-69*. (Source: *Library and Archives Canada — Bill C-150 legislative history*; *Egale Canada — A Brief History of LGBTQ Rights in Canada, 2024*.) Canadians who were 65 or older in 2026 were born in 1961 or earlier. They lived the entirety of their childhood and at least the first eight years of their adult lives during the criminalization period. Canadians who are 75 or older in 2026 were born in 1951 or earlier. They lived their entire childhood, adolescence, young adulthood, and most of their early career during criminalization.

Decriminalization in 1969 did not produce social or workplace equality. Employment discrimination on the basis of sexual orientation was lawful in most Canadian jurisdictions through the 1970s and 1980s. Sexual orientation was added to the *Canadian Human Rights Act* in 1996. (Source: *Department of Justice Canada — Canadian Human Rights Act amendments, 1996*.) Same-sex marriage was legalized federally in Canada through the *Civil Marriage Act* on July 20, 2005. (Source: *Civil Marriage Act, S.C. 2005, c. 33*.) Common-law recognition of same-sex partnerships at the federal benefit level was implemented in 2000 through the *Modernization of Benefits and Obligations Act*. (Source: *Modernization of Benefits and Obligations Act, S.C. 2000, c. 12*.)

For a senior who is 75 in 2026, this means that the OAS, GIS, CPP retirement, and Allowance program structures recognized their same-sex partnership only after they had already been together for decades, in many cases. The benefit-calculation history is shorter than the relationship history. Where the relationship pre-dated 2000-2005 federal recognition, contribution-history calculations and partner-recognition adjudications may not reflect the actual partnership duration. The application of survivor benefits in particular has historically produced disputes for couples whose relationships pre-dated formal recognition, with some couples requiring documentation efforts to establish that their pre-2000 cohabitation qualifies as common-law for benefit purposes.

The AIDS crisis cohort effects

The HIV/AIDS crisis in Canada in the 1980s and 1990s produced disproportionate mortality among gay and bisexual men born in the 1940s through the 1960s. Public Health Agency of Canada surveillance data documents that men who have sex with men accounted for the majority of HIV diagnoses in Canada in the early-pandemic decades, with cumulative mortality concentrated in this demographic. (Source: *Public*

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Health Agency of Canada — HIV and AIDS in Canada surveillance reports, 1996 through 2024.) The surviving cohort of gay and bisexual men born 1940-1965 — the cohort approaching or in the senior years today — is a smaller cohort than would have been demographically expected, and is a cohort that experienced documented community-level trauma through the loss of friends, partners, and chosen-family members during the working years.

The community-trauma effect is documented in geriatric and mental-health literature on this population. (Source: Hurd, L., Li, L.Y.K., "I want to grow older with dignity: Older LGBTQ+ Canadian adults' perceptions and experiences of aging," *Journal of Applied Gerontology*, 2024; *Frontiers in Public Health — Older LGBTQ+ Canadians' Experiences of Prejudice and Discrimination Over the Life Course*, 2025.) The senior of this cohort has, in many cases, fewer surviving friends and chosen-family members than a same-aged senior of the broader population. The informal-caregiving network that the senior architecture presumes — and that disproportionately falls to adult children and adult-child-equivalent relationships in Canadian senior care — is, for this cohort, materially smaller than for the comparator population.

Family-of-choice and the benefit calculation

The federal and provincial benefit calculations documented across Movements One and Two recognize legal marriage and common-law cohabitation. They do not recognize family-of-choice — the long-standing committed friendships, chosen-family arrangements, and community-based mutual-support relationships that have, since the criminalization period, served as the functional family unit for many 2SLGBTQ+ Canadians who were estranged from biological families or who built primary support relationships outside of the legal-family structure.

The operational consequence is that a 2SLGBTQ+ senior whose primary committed relationship is not a legally-recognizable spousal relationship is treated, by the benefit-calculation architecture, as a single individual. Their family-of-choice partner has no rights to survivor benefits, no rights to spousal CPP credit splitting, no rights to be considered as a household member for ASB or GIS purposes (in either direction — neither for combined-income reduction nor for eligibility-expansion), and no formal status as a substitute decision-maker in the absence of explicit legal documentation. The senior couple who has, for forty years, shared a household, an emotional life, financial commitments, and mutual care, but who is not legally married or common-law (and may, under existing definitions, not qualify as common-law if their formal address arrangements differ), is not a couple in the eyes of the senior income system.

Egale Canada's documentation on this population identifies the resulting pattern: many 2SLGBTQ+ older adults rely on chosen families and community networks for support, while the federal and provincial benefit architecture provides no recognition of these relationships. (Source: *Egale Canada — Seniors & Older Adults: A Brief on 2SLGBTQI Realities and Policy in Canada*, 2025.) Where the chosen-family member ages, becomes ill, or dies, the surviving partner has no benefit access tied to the relationship and no formal standing in healthcare, financial, or end-of-life decision-making absent prepared legal documentation.

Working-age discrimination and the CPP contribution profile

Workplace discrimination on the basis of sexual orientation was lawful in most Canadian jurisdictions until provincial human-rights legislation incrementally addressed it in the 1980s through the 2000s. Workplace discrimination on the basis of gender identity was not addressed in federal human-rights legislation until 2017 through Bill C-16 (*An Act to amend the Canadian Human Rights Act and the Criminal Code*). (Source:

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

An Act to amend the Canadian Human Rights Act and the Criminal Code (*Bill C-16*), S.C. 2017, c. 13.) The lawful-discrimination period covers the working years of essentially every Canadian who is now over 50.

For 2SLGBTQ+ Canadians who were "out" during the working years, the lawful discrimination produced documented effects on lifetime earnings — through job loss, demotion, exclusion from advancement, exclusion from certain occupational categories, and the choice (made by many) to remain closeted at work, which itself produced minority-stress effects on workplace functioning and career progression. (Source: *Rainbow Health Ontario — Health in Focus: Racialized 2SLGBTQ Health, 2022*; *Public Health Agency of Canada — 2SLGBTQI+ Health Indicators reports, 2022-2024*.) For trans Canadians specifically, the lawful-discrimination period extended further, with the documented employment-discrimination experiences of trans workers being more recent and more severe than for cisgender LGB workers.

The CPP retirement formula calculated against this contribution profile produces, for many 2SLGBTQ+ seniors, a smaller pension than the formula would have produced for the same individual without the discrimination history. The structural argument documented in Section 2.2 — that CPP contributions are pooled and the formula determines benefits — operates against this population as it does against everyone, but the contribution history that the formula operates on was suppressed by lawful discrimination during the working years.

Healthcare avoidance and the operational consequence

Canadian and international advocacy and research literature consistently documents that 2SLGBTQ+ older adults have higher rates of healthcare avoidance than the broader senior population, due to documented experiences and ongoing fears of discrimination by healthcare providers. (Source: *Egale Canada — Seniors brief, 2025*; *Public Health Agency of Canada — 2SLGBTQI+ Health, multiple reports*; Pang, K., MacLennan, S., *2SLGBTQI+ Older Adults in Canada, 2023*.) The avoidance pattern produces documented downstream effects on chronic-disease management, on early intervention for treatable conditions, and on the senior's overall health trajectory.

The compounding effect at the senior stage is direct. The senior who has avoided healthcare through the working years, accumulated untreated chronic conditions, and arrives at the senior stage with health profile worse than that of the comparator population is more likely to require continuing care earlier and more intensively. The continuing care system the senior arrives at is the same system documented in Section 2.5, with the LTC-specific issues documented immediately below.

Long-term care and the closet

The single most documented structural concern for 2SLGBTQ+ seniors approaching long-term care entry, across Canadian and international advocacy literature, is the fear of being forced to "go back into the closet" upon LTC admission. (Source: *Egale Canada — Seniors page, 2025*; *CBC News — "We're going back into the closet: LGBTQ seniors wary of being out in long-term care facilities," June 2018*; Flanagan, B., Pang, K., *Coming Out and Coming In to Living with Dementia, 2022*.) The documented fear is not abstract. It is grounded in the operational reality that long-term care facilities in Canada are not, with limited exceptions, designed around 2SLGBTQ+ inclusion, that staff training on 2SLGBTQ+ senior care varies widely, and that the resident's exposure to other residents — many of whom are of an older birth cohort with documented higher rates of anti-LGBTQ+ attitudes — produces day-to-day social experience over which the resident has no control.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The operational consequences are documented in international research. A U.S. Gerontological Society of America 2024 study of LGBTQ+ adults aged 50 and over found that 71% were concerned about access to future caregiving and 76% were concerned about finding inclusive long-term care or caregiving resources. (Source: *Flatt, J., et al., "Discrimination and Access to Aging and Long-Term Care Services and Supports for LGBTQIA+ Older Adults," Gerontological Society of America, 2024.*) The Canadian advocacy literature, with smaller-sample qualitative documentation, identifies analogous concerns. (Source: *Egale Canada — Seniors brief, 2025.*) The senior who chooses to go back into the closet upon LTC admission accepts the daily-life cost of suppressing their identity in exchange for safety and care continuity. The senior who chooses not to suppress accepts the documented risk of mistreatment, exclusion, and reduced care quality. Neither option is what the broader senior population faces.

For trans seniors specifically, the LTC issue is more acute. The provision of gender-affirming care, the use of correct names and pronouns by staff and other residents, the management of body care during bathing and toileting (which may produce gender-identity disclosure in residential settings), the arrangement of personal-clothing storage and laundry, and the management of medical needs related to gender-affirming hormone therapy or surgical history — each is a category in which most Canadian LTC facilities do not have established policies or trained staff. The trans senior arriving at LTC arrives at an institution that is not, in operational reality, prepared to deliver the care they require.

Trans seniors and federal documentation

Trans Canadian seniors who legally changed their name and/or gender marker during their adult life have CPP contribution histories, federal income tax histories, OAS application histories, and Service Canada records that span the pre-transition and post-transition periods. The records may not consistently reflect a single legal identity. Application for OAS, GIS, CPP retirement, or any other federal benefit may produce administrative complications when the federal staff encounter records that show different names or gender markers across the contribution history.

Service Canada has internal procedures for managing gender-marker and name-change updates to records, but the procedures depend on the applicant providing documentation of the legal change and on the federal staff applying the procedures correctly. (Source: *Service Canada — Updating personal information page, 2026; Egale Canada — Trans Federal Identification Guide, 2024.*) Where the procedures are not followed correctly, the trans senior may experience benefit delays, requests for additional documentation, or in some cases the merging or unmerging of records that complicates the contribution-history calculation. Each of these administrative friction points is, in operational reality, additional burden on the trans senior at the moment they are accessing benefits they have qualified for through their working life.

Single 2SLGBTQ+ seniors and the architecture's presumptions

A substantial proportion of 2SLGBTQ+ Canadian seniors are single, childless, or both. Census data on this is limited because the federal census collects sexual orientation and gender identity data only in some cycles and only for some age groups. (Source: *Statistics Canada — Census of Population, sexual orientation and gender identity components, 2021 cycle; Statistics Canada — A demographic, employment and income profile of Canadians with non-binary gender identities, 2022.*) The available estimates suggest that 2SLGBTQ+ Canadians aged 65 and over have higher rates of single-individual household composition than the comparator population, with cumulative effects on informal caregiving access, on

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

partnership-related benefit eligibility, and on the 60-to-64 gap zone documented in Section 2.6.

The 60-to-64 gap zone — the structural exclusion documented for single individuals in this age band who do not have a current OAS-receiving spouse and have not been widowed — applies with particular force to 2SLGBTQ+ singles. The federal Allowance and Allowance for the Survivor are not available to them. Provincial Income Support is the documented fallback, at substantially lower benefit levels than the Allowance would provide. The 2SLGBTQ+ individual who has been single through their adult life — by choice, by circumstance, by the documented community-mortality effects of the AIDS crisis cohort, or by family-of-choice arrangements that do not legally qualify as spousal — arrives at age 60 with no Allowance access and waits five years for the senior architecture to engage at all.

The lifecycle observation

The senior architecture's presumptions about family structure, the working-life contribution profile, and the LTC entry experience produce a documented pattern of operational gaps for 2SLGBTQ+ seniors. The presumptions are not the result of program-design hostility. They are the result of program-design that did not consider the specific structural circumstances of the 2SLGBTQ+ population and that has not been updated in operational design as the demographic and legal context has changed.

The 2SLGBTQ+ senior who is partnered in a legally-recognized marriage or common-law relationship navigates the architecture on the same terms as any senior couple, with the partnership-penalty mechanisms documented in Section 2.7 operating equally. The 2SLGBTQ+ senior whose primary relationship is family-of-choice navigates the architecture as a single individual, with the chosen-family member having no formal standing. The 2SLGBTQ+ senior who is single navigates the architecture as a single individual, with the additional structural pressure produced by smaller informal-support networks and documented healthcare-avoidance histories. The trans senior navigates the architecture with the additional documentation friction produced by federal-record name and gender-marker history.

The cage that this document names operates against this population with the additional pressure that family-of-choice is not recognized in the calculation, that the contribution history may have been suppressed by lawful working-years discrimination, that LTC entry produces the documented "back in the closet" risk, and that the smaller surviving cohort produces fewer informal supports than the architecture presumes.

Section 3.3 turns to immigrant and racialized seniors — populations whose intersection with the federal-and-provincial architecture is shaped by sponsorship rules, residency requirements, and documented patterns of working-years discrimination that affect contribution histories.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 3.3 — Immigrant and Racialized Seniors: Sponsorship, Residency, and the Working-Years Discrimination Tax

The federal-and-provincial senior architecture documented across Movements One and Two operates against immigrant seniors and against Canadian-born racialized seniors with structural pressures that differ in form and degree from those it produces against the broader population. This Section walks the principal pressure points: the OAS residency tier system, the federal social security agreement framework that mediates international residency, the October 1, 2025 sponsorship-rule change that extends the GIS/Allowance/Allowance for the Survivor exclusion period from 10 years to 20 years for parent-and-grandparent sponsorship, the late-career CPP contribution profile of immigrants who arrived as adults, the documented working-years discrimination effects on Canadian-born racialized workers' lifetime earnings, and the operational reality of navigating the architecture as a non-native English or French speaker.

The 40-year residency requirement and the partial-pension formula

OAS pays the maximum pension to applicants who have lived in Canada for 40 or more years after age 18. Applicants with 10 to 39 years of Canadian residency receive a partial pension calculated on the formula of years-in-Canada divided by 40, applied to the maximum pension amount. (*Source: Old Age Security Act, R.S.C. 1985, c. O-9, sections 3-4; Service Canada — Old Age Security: Do you qualify, 2026.*) An applicant who has lived in Canada for 20 years receives $20/40 = 50\%$ of the maximum pension. An applicant with 15 years of Canadian residency receives $15/40 = 37.5\%$.

For immigrants who arrived in Canada as adults, the partial-pension formula produces materially smaller OAS payments at age 65. An immigrant who arrived at age 35 has 30 years of residency at age 65, qualifying for $30/40 = 75\%$ of the maximum OAS — approximately \$557 per month at the Q1 2026 rate, instead of the \$742.31 maximum. An immigrant who arrived at age 50 has 15 years of residency at age 65, qualifying for 37.5% of the maximum — approximately \$278 per month. An immigrant who arrived at age 60 has 5 years of residency at age 65, falling below the 10-year minimum, and may qualify for OAS only through a social security agreement or not at all.

The structural feature is that the partial-pension formula does not consider the immigrant's contributions to other countries' pension systems before arrival. Where Canada has a social security agreement with the country of origin, those years of foreign residency or contribution may count toward meeting the 10-year minimum for OAS eligibility (without affecting the payment amount). Where Canada has no such agreement, the foreign years do not count for any purpose.

Canada has social security agreements with more than 50 countries, including the United States, India, China, the Philippines, the United Kingdom, France, Germany, Italy, Mexico, Brazil, and many others. (*Source: Service Canada — International social security agreements, 2026; Canada-US Totalization Agreement, Social Security Administration, 2024.*) Canada does not have agreements with all source countries of its immigrant population. Senior immigrants from countries without agreements may face full ineligibility if they cannot meet the 10-year Canadian-residency minimum, or partial eligibility at the lowest end of the formula.

The October 1, 2025 sponsorship-period extension

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The Old Age Security Act, since 1996, has prohibited the payment of OAS income-tested benefits — the Guaranteed Income Supplement, the Allowance, and the Allowance for the Survivor — to sponsored immigrants who have less than 10 years of Canadian residency. (Source: *Old Age Security Act, sections 19-21; Canada Gazette, Part 2, Volume 159, Number 6: Order Fixing October 1, 2025, March 12, 2025.*) The rationale is that the sponsor has committed, in the sponsorship undertaking, to meet the financial needs of the sponsored individual for the sponsorship period.

In January 2014, the Immigration and Refugee Protection Regulations were amended to extend the parent-and-grandparent (PGP) sponsorship period from 10 years to 20 years in all provinces except Quebec. The Old Age Security Act was amended in 2014 to align the OAS income-tested benefit exclusion period with the new 20-year sponsorship duration. The amendments were enacted in legislation but not brought into force until October 1, 2025, when the Order Fixing October 1, 2025 made the 2014 OAS Act amendments operational. (Source: *Canada Gazette, Part 2, Volume 159, Number 6, March 12, 2025; Economic Action Plan 2014 Act, No. 1, S.C. 2014, c. 20, Division 27.*)

The operational effect, as of October 1, 2025, is that sponsored immigrants under 20-year PGP sponsorship are excluded from GIS, the Allowance, and the Allowance for the Survivor for the full 20-year sponsorship period — not the previous 10-year period. The first cohort of sponsored seniors landing under the 20-year PGP framework arrived in Canada in 2016. They became eligible for the basic OAS pension in 2026, after meeting the 10-year residency requirement. Their eligibility for the income-tested supplements — GIS, the Allowance, the Allowance for the Survivor — is now deferred until 2036, when their 20-year sponsorship period concludes.

The structural impact on the sponsored senior is significant. The basic OAS pension at the partial rate (typically 10/40 to 15/40 of the maximum) is approximately \$186 to \$278 per month at Q1 2026 rates. Without GIS, the sponsored senior's federal income alone is well below the cost of living for a single person in any Canadian city. The senior is dependent on the sponsor (typically an adult child or grandchild) for the difference between their federal income and their cost of living. The dependency relationship, structurally encoded in the sponsorship undertaking, extends across the full 20-year period.

The sponsor's own financial circumstances may change across 20 years. Job loss, divorce, illness, or death of the sponsor produces structural exposure for the sponsored senior. The OAS Act contains protective provisions that allow the sponsored senior to claim income-tested benefits in cases of sponsorship breakdown — sponsor death, sponsor incarceration of more than six months, sponsor bankruptcy, sponsor conviction for an offence relating to the sponsored person. (Source: *Canada Gazette, Part 2, Volume 159, Number 6, March 12, 2025.*) These protections require the sponsored senior to navigate the formal documentation of sponsorship breakdown, with Service Canada adjudication of the breakdown claim. The protections exist. The protections require active claim-making by the sponsored senior to engage them.

The CPP contribution profile of late-career immigrants

The Canada Pension Plan retirement formula is calculated against the worker's contribution history from age 18 (or the start of CPP-pensionable employment in Canada) to the year of CPP application or age 65, whichever is earlier. (Source: *Canada Pension Plan Act, R.S.C. 1985, c. C-8, sections 44-50.*) An immigrant who arrived in Canada at age 35 and worked continuously to age 65 has 30 years of CPP-eligible employment. The general dropout provision excluding the lowest-earning 8 years applies, leaving 22 years

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

in the calculation. The formula's 25% (now phasing toward 33%) replacement rate is applied to the average of those years' pensionable earnings. For an immigrant who earned at or below the YMPE in early Canadian career and approached YMPE in later career, the calculated CPP retirement pension is materially below the 30+-year-average comparator.

For an immigrant who arrived at age 50, only 15 years of CPP-eligible employment are available. The dropout reduces this further. The CPP retirement pension is calculated against this short contribution history and is correspondingly small.

For an immigrant who arrived at age 60, the available CPP-eligible employment period is 5 years. CPP-D and other contributory benefits may not be available given the four-of-six contribution rule (which requires recent CPP contributions). CPP retirement at age 65 is calculated against the 5-year contribution history, producing a pension in the low double-digit dollars per month range for many late-career immigrants — sometimes effectively zero after the formula's calculation against averaged earnings.

The structural problem for late-career immigrants is that the senior architecture presumes a 40-year working life of CPP contributions. The immigrant who arrived in Canada in mid-life or later did not have 40 years to contribute. The CPP retirement pension reflects only the Canadian portion of their working life. The pension contributions they may have made to their country of origin's social security system, where applicable, may produce a pension from that country (with its own rules, payment mechanisms, and currency exposure). The combined Canadian-and-foreign retirement income is what the late-career immigrant senior actually receives.

Working-years discrimination and the racialized senior population

For Canadian-born racialized seniors and for immigrants who have been in Canada through most of their working life, the principal CPP-affecting structural pressure is workplace discrimination. The documentation of employment discrimination against Black, Asian, South Asian, Indigenous, and other racialized Canadians across the working-life decades of today's senior cohort is substantial. (*Source: Canadian Centre for Diversity and Inclusion — Annual Workforce Discrimination Reports, 2018-2024; Statistics Canada — Earnings differentials of racialized Canadians, multiple reference periods; Canadian Race Relations Foundation — Employment Discrimination in Canada reports, 2020-2025.*)

The documented effects include lower lifetime earnings, slower promotion trajectories, exclusion from certain occupational categories, concentration in lower-paying sectors, higher unemployment rates, and longer periods of unemployment between jobs. Each of these effects translates directly into the CPP contribution history. The cumulative effect is a CPP retirement pension that is smaller than would have been the case for the same individual without the discrimination history.

The 2025 National Advisory Council on Poverty report identifies that Black and racialized individuals, Indigenous people, newcomers, 2SLGBTQIA+ individuals, people with disabilities, and seniors all face systemic barriers in housing, employment, education, and access to services that make them more vulnerable to poverty. (*Source: National Advisory Council on Poverty — 2025 Annual Report, Government of Canada.*) For racialized seniors specifically, the cumulative barrier across the working years compounds at the senior stage. The 2024 Canadian Income Survey identifies recent immigrants aged 15 and older as having a poverty rate of 15.3% in 2023, compared with 11.0% in the broader population. (*Source: Statistics Canada — Canadian Income Survey 2024, released April 2026.*) Disaggregated rates for racialized seniors

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

specifically are reported intermittently across federal sources; the patterns are consistent with the broader documentation.

Documentation and language barriers in the benefit-application process

The federal and provincial senior-benefit application processes are conducted in English and French, with translated materials available for some programs and some languages but not uniformly across the inventory. (Source: Service Canada — *Multilingual services page*, 2026; Government of Alberta — *Alberta Supports multilingual services*, 2026.) Application forms, eligibility-criteria documentation, appeal procedures, and ongoing recipient communications presume functional literacy in English or French.

The senior immigrant who arrived in Canada in mid-life or later, whose primary language remains the language of origin, navigates the application process through one of several mechanisms: with the assistance of a family member (typically an adult child) acting as translator, with the assistance of a community-based settlement organization where one is available, with paid translation services, or — where none of these is accessible — by attempting the process in a language they have limited functional capacity in. Each of these mechanisms produces friction, error rates, and delay.

The 2025 National Advisory Council on Poverty report identifies that "complex programs, unclear application processes, inconsistent rules and rigid eligibility criteria" produce barriers to support access for marginalized populations including newcomers, with "the requirement to file taxes as an eligibility criterion, accessibility challenges and digital exclusion" further limiting access. (Source: *National Advisory Council on Poverty — 2025 Annual Report*.) For the senior immigrant with limited English or French and limited digital fluency, the cumulative friction of navigating federal OAS plus provincial Alberta Seniors Benefit plus provincial Coverage for Seniors plus federal NIHB (where applicable) plus other supports produces documented under-use. Eligible seniors do not access benefits they qualify for. The unclaimed-GIS phenomenon — hundreds of thousands of eligible Canadian seniors not receiving GIS because they did not apply or did not know they qualified — is documented across Canadian senior-policy literature and disproportionately affects immigrant and racialized senior populations. (Source: *Government of Canada — Take-Up of Old Age Security and Guaranteed Income Supplement Benefits report*; *National Institute on Ageing — Senior Benefit Take-Up reports, 2022-2024*.)

The cohabitation-rule and multi-generational household interaction

The AISH cohabitation rule documented in Section 1.5 and the GIS partnership-penalty mechanism documented in Sections 2.4 and 2.7 operate, by design, on the household configuration the recipient lives in. For immigrant senior populations whose family-structure norms include multi-generational households more frequently than in the comparator population — three-generation households with grandparents, adult children, and grandchildren under one roof, or multi-family-unit residences shared across extended family — the household-configuration assumption may produce administrative complications.

A senior whose primary income is OAS-and-GIS, living in a household with adult children whose income exceeds the GIS partner-income threshold, is not directly affected by the partner-income calculation if the senior is not married to or in a common-law relationship with anyone in the household. Adult children are not treated as common-law partners for OAS-and-GIS purposes. The administrative friction occurs when Service Canada adjudication encounters non-standard household configurations and requests additional documentation to confirm relationship structures. The senior who lives with adult children, grandchildren,

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

siblings, or other extended family members may be asked to provide documentation that a senior in a single-person household or two-person spousal household would not be asked for.

The 2014 IRPR amendments and the October 2025 OAS Act sponsorship-extension produce a related pressure. The adult child sponsoring a parent or grandparent under the 20-year PGP framework has accepted the financial responsibility for the sponsored individual for that full period. The sponsor's own household income may include earnings, dividends, capital gains, and other income components that are not the sponsored senior's income but that may be examined in some adjudication contexts. The interaction between the sponsorship undertaking and the federal benefit-eligibility framework produces administrative complexity that the sponsoring family must navigate.

The lifecycle observation

The senior architecture documented across Movements One and Two operates on assumptions about working life — that it occurred in Canada, that it began in the late teens, that it continued for 40 to 47 years — that do not apply to a substantial portion of the immigrant senior population. The architecture also operates on assumptions about household configuration, primary language, and documentation availability that produce structural friction for immigrant and racialized seniors at the application, eligibility, and ongoing-benefit-receipt stages.

The operational consequences are documented: smaller OAS pensions for late-career immigrants under the partial-pension formula; full GIS/Allowance/Allowance for the Survivor exclusion for the 20-year PGP-sponsorship period under the October 2025 rule change; smaller CPP retirement pensions for late-career and discrimination-affected workers; under-use of available benefits due to language and documentation barriers; structural dependency on adult-child sponsors for two decades after arrival; multi-generational-household administrative friction.

The cage that this document names operates against this population with the additional pressure that the architecture's working-life assumptions do not fit, that the language and documentation infrastructure does not match the population's needs, and that the sponsorship structure formally extends adult-child financial responsibility for two decades — a duration during which the sponsor's own circumstances may change in ways the senior architecture does not provide a smooth fallback for.

Section 3.4 turns to the senior caregiver — including the disabled adult who has spent decades caregiving for an aging parent, and the parent-caregiver of a disabled adult who themselves enters long-term care, leaving the disabled adult without their lifelong primary support.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 3.4 — Senior Caregivers: The Disabled Adult Caregiving for a Parent, the Parent-Caregiver Entering LTC, the Architecture's Hidden Foundation

The federal-and-provincial senior architecture documented across Movements One and Two depends, at every stage, on a layer of unpaid caregiving labour that the architecture itself does not pay for, does not formally recognize, and does not include in its eligibility or benefit calculations. The labour is overwhelmingly performed by women — daughters, daughters-in-law, wives, sisters, and other female relatives. It is the structural foundation of the senior care system in Canada. (Source: Statistics Canada — *Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022*; Vanier Institute of the Family — *Caregiving in Canada Report, 2023*; Canadian Centre for Caregiving Excellence — *annual reports, 2022-2025*.) When this labour is reduced or removed — whether through the caregiver's own illness, employment necessity, geographic distance, or death — the formal system absorbs the gap with documented operational difficulty.

This Section walks two specific configurations within the senior caregiver population that produce particular structural pressure: the disabled adult who has spent decades caregiving for an aging parent (and who faces the loss of that role and that household at the parent's death), and the parent-caregiver of a disabled adult who themselves enters long-term care (leaving the disabled adult without their lifelong primary support). It also walks the more general dynamic of the senior couple in which one partner becomes the caregiver of the other, the adult-child caregiver of an aging parent, and the Alberta Caregiver Tax Credit changes announced in the February 2026 provincial budget that consolidate the existing two-credit framework into a single Alberta Caregiver Credit beginning in the 2027 tax year.

The economic value of unpaid caregiving in Canada

Statistics Canada and Canadian Centre for Caregiving Excellence research has documented the scale of unpaid caregiving in Canada. The Canadian Centre for Caregiving Excellence estimates that approximately 7.8 million Canadians act as caregivers for family members with chronic conditions, disabilities, or aging-related needs, and that the unpaid labour they provide has an estimated economic value in the range of \$97 billion to \$130 billion per year — an amount comparable to the total provincial expenditure on health care across the country. (Source: Canadian Centre for Caregiving Excellence — *The State of Caregiving in Canada Report, 2024*; Statistics Canada — *General Social Survey, Caregivers and Care Receivers, 2018 cycle and subsequent*.) The labour is provided without compensation, without formal recognition in the public-program inventory, and without protection in the labour-market sense (no caregiver employment standards, no paid caregiver leave equivalent to parental leave at the federal level, limited employment-insurance compassionate-care benefits with restrictive eligibility).

The structural reliance on this labour is not accidental. Government policy across Canadian provinces and territories has, since the deinstitutionalization wave of the 1970s and 1980s, increasingly relied on community-based and family-based care to substitute for institutional care. The cost savings have been documented as substantial; the cost has been transferred to the family caregiver. (Source: Romanow Commission — *Building on Values: The Future of Health Care in Canada, 2002*; Canadian Institute for Health Information — *Health Care Cost Drivers reports, multiple years*.) The senior who is supported at home rather than in long-term care is supported, in operational reality, by the family caregiver who provides the daily care the institution would otherwise provide. The federal and provincial systems benefit from the cost difference. The caregiver bears the cost difference.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The disabled adult caregiving for a parent

A specific configuration that the campaign canon has identified, and that the present Section addresses in operational detail, is the disabled adult who has spent decades caregiving for an aging parent. (Source: *Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 9; Inclusion Alberta — Adult Children with Disabilities Caring for Aging Parents brief, 2024*.) This is not the standard caregiver-care-receiver dyad. It is the disabled adult — themselves an AISH or ADAP recipient, themselves managing chronic illness, mental-health conditions, or physical disability — providing care to a parent whose own aging-related needs have evolved over the years and who is, by the late stage of the configuration, more dependent than the disabled adult is.

The configuration is documented in disability advocacy literature. It produces several structural pressures the architecture does not address:

- **The household composition under AISH/ADAP cohabitation rules.** The disabled adult AISH recipient living with an aging parent is in a multi-adult household. The parent's income, where any exists, is subject to the household-income calculation. The parent's accommodation contribution, where the parent is contributing rent or mortgage, is part of the household-income calculation. The disabled adult who is providing care to the parent receives no AISH credit for the care; the parent's contribution to the household is treated as income against the AISH benefit.
- **The dependency configuration is bidirectional but only one direction is recognized.** In a caregiver-care-receiver dyad, the formal system recognizes the care-receiver as the dependent and may provide caregiver tax credits (Section below) to the caregiver. In the disabled-adult-caregiving-for-parent configuration, the parent is technically the care-receiver, and the disabled adult is technically the caregiver, but the disabled adult is also a recipient of disability benefits in their own right, and the formal system does not have a clean adjudication framework for the bidirectional dependency. The parent may have qualifying conditions for caregiver tax credits to be claimed by the disabled adult; the disabled adult's own qualifying conditions may make them eligible for parental support that the parent no longer has the capacity to provide. The system was not designed for this configuration.
- **The succession question.** When the parent dies, the household configuration ends. The disabled adult, who has been providing care, is left in a household they may no longer be able to maintain financially or operationally. The parent's estate, where any exists, may pass to the disabled adult, but the AISH asset cap (\$5,000 for a single recipient, \$10,000 with dependents) means that any inheritance must be liquidated, sheltered through specific mechanisms (a Henson Trust being the primary structural option), or spent down before continuing AISH eligibility is preserved. The disabled adult who inherits a home from the parent is in a structurally complex position — the home is exempt from the asset cap as a primary residence, but the cost of maintaining the home is now solely the disabled adult's responsibility, and the disabled adult's reduced caregiving role may leave them socially isolated in a way that compounds the qualifying conditions.
- **The grief is compounded by the structural transition.** The disabled adult is grieving the loss of a parent who was, in many cases, also their longest-term advocate, primary social relationship, and only family member with full understanding of the disability. The grief operates against the same architecture documented across Movements One and Two — the AISH or ADAP benefit, the asset cap, the cohabitation rule reassessment if any new household member enters, the broader operational reality of

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

disability income at the floor.

The parent-caregiver of a disabled adult who enters LTC

The mirror configuration is the parent-caregiver of a disabled adult who themselves enters long-term care. The aging parent has provided primary care to the disabled adult son or daughter — often into the parent's own seventies, eighties, or beyond. When the parent's own health requires LTC entry, the disabled adult loses their lifelong primary caregiver. The disabled adult's own care needs do not change; the operational support for those needs is suddenly absent.

The configuration is documented in Inclusion Alberta's PDD and AISH advocacy literature and is one of the most significant structural failures of the disability-and-senior architecture in Alberta and across Canada. (Source: *Inclusion Alberta — PDD Current State: Critical and Urgent, January 2025*; *Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Sections 5 and 9*.) The structural pressure is that:

- The parent's PDD-or-AISH-receiving adult son or daughter has been receiving the institutional disability benefit but has been receiving the actual day-to-day care from the parent, often in addition to the formal supports the program provides. The parent has been managing medication, providing transportation, advocating in healthcare and educational settings, providing emotional support, and supplying the additional unpaid labour that the formal benefit was not sufficient to cover.
- When the parent enters LTC, none of this informal support transfers. PDD service hours, where applicable, may increase if the formal assessment identifies the increased need — but the increase is constrained by the program's allocation framework, by the available service-provider capacity in the geographic area, and by the documented PDD-funding pressures that Inclusion Alberta has documented.
- AISH benefit levels do not change with the loss of a primary informal caregiver. The same monthly amount is paid. The supplementary services that were being provided unpaid by the parent now require either increased formal support (often unavailable at scale), payment for private home-care services (impossible at AISH/ADAP income levels), or the disabled adult's own absorption of the gap (which may be impossible given the qualifying conditions).
- The disabled adult who has lived their entire life in the parental home may, at the parent's LTC entry or death, face the loss of housing as well. Where the parent owned the home, succession planning may have addressed this — through a life-interest trust or a Henson Trust funded by the home's eventual sale, or through formal transfer of ownership before the parent's incapacity — but where succession planning was not undertaken (which is the documented norm for many low-income families), the disabled adult faces both the loss of caregiver and the loss of housing simultaneously.

The structural failure is that the architecture was not designed with this transition in mind. The disability program (AISH, ADAP, PDD) was designed for a recipient with informal supports continuing across the lifecycle. The senior program inventory (LTC for the parent, OAS-and-GIS-and-supports for the parent's senior years) was designed for a senior whose family configuration would persist or evolve through normal life-cycle dynamics. Neither program inventory was designed for the moment when the parent who has been the disabled adult's primary support enters LTC and the disabled adult is left without that support.

The senior couple's caregiver dynamic

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The most common senior caregiver configuration is the senior couple in which one partner becomes the caregiver of the other, typically as one partner's health deteriorates ahead of the other's. The caregiver is often, but not exclusively, the female partner. The care provided is, in operational reality, full-time work — medication management, mobility assistance, transfers, bathing, toileting, meal preparation, transportation to medical appointments, advocacy across the healthcare system, emotional support across the documented cognitive-decline trajectory of the care-receiving partner.

The caregiver's own health is documented as deteriorating across the duration of the caregiving relationship. (Source: Vitaliano, P.P., et al., "Is caregiving hazardous to one's physical health?" *Psychological Bulletin*, 2003; Schulz, R., Sherwood, P.R., "Physical and mental health effects of family caregiving," *American Journal of Nursing*, 2008; Statistics Canada — *Caregiver Health Outcomes, multiple reports*.) Increased rates of depression, anxiety, cardiovascular disease, immune dysfunction, sleep disorders, and premature mortality are consistently documented across multiple decades of literature. The longer the caregiving continues, the more pronounced the effects.

When the care-receiving partner enters LTC, the caregiver experiences the documented "respite paradox" — the relief of the care responsibility being lifted is accompanied by deferred grief, deferred health crises that had been suppressed during the caregiving period, and the social isolation of being newly alone in the household after years of intensive caregiving. (Source: Schulz, R., et al., "Predictors and Consequences of Bereavement Among Caregivers," *Journal of Gerontology*, 2008.) The caregiver who has been functioning at maximum capacity for years experiences, in the months and years after the partner's LTC entry or death, the cumulative health effects that the caregiving role had postponed.

The adult-child caregiver

The adult-child caregiver of an aging parent — most commonly a middle-aged daughter — is the third major configuration. (Source: Statistics Canada — *Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022*.) The daughter, often in her forties or fifties, often herself raising children, often herself working full-time in formal employment, manages the parent's care needs across the geographic distance between her household and the parent's. Where the geographic distance is substantial and travel is required, the cost falls to the daughter and is not formally recognized in any program inventory.

The Sandwich Generation framing — adults caregiving simultaneously for children and parents — is documented as producing particular financial and time pressure. (Source: Vanier Institute of the Family — *Sandwich Generation reports, multiple years*; Statistics Canada — *Caring for parents while raising children, multiple reference periods*.) The Canadian Centre for Caregiving Excellence's 2024 report identifies that approximately 50% of Canadian caregivers report financial stress related to their caregiving role, with a significant subset reducing or ending paid employment to provide care. The reduction in paid employment translates directly into reduced CPP contributions, reduced retirement savings, and structural exposure to senior-stage poverty for the caregiver herself.

The Alberta Caregiver Credit consolidation

The Alberta provincial budget tabled February 26, 2026 announced the consolidation of two existing tax credits — the caregiver amount and the amount for adult infirm dependents — into a single Alberta Caregiver Credit, effective for the 2027 tax year. (Source: ATB Financial — *Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026*; Government of Alberta — *Budget 2026 Tax Plan*.) The

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

consolidation is designed to align Alberta's rules with the federal Canada Caregiver Credit. It expands eligibility to include infirm spouses but restricts claims for housing healthy senior parents or grandparents.

The estimated impact: approximately 35,000 Albertans currently receive one or both of the existing credits. Under the new framework, an estimated 16,500 individuals will no longer qualify, while approximately 7,500 others may receive a higher benefit or become newly eligible. The net effect is approximately 9,000 Albertans losing access to a tax credit they currently receive.

The maximum value of the existing credits is approximately \$1,054 per year. (*Source: ATB Financial — Alberta provincial budget 2026, February 27, 2026.*) The credit offsets a small portion of the unpaid caregiving labour cost; it is not a payment for the caregiving. It does not approach the actual cost of the labour the caregiver is providing. The consolidation does not change the fundamental inadequacy of the credit relative to the labour value; it changes which categories of caregiver receive the modest support and which do not.

The lifecycle observation

The senior architecture documented across Movements One and Two presumes the existence of unpaid caregiving labour at every stage. Long-term care accommodation rates assume institutional care for those who reach the threshold of need; for the substantial population whose care needs are below the LTC threshold but above what they can manage independently, family caregivers fill the gap. Home-care allocation through Alberta Health Services is calibrated to a level that requires family or paid private care to supplement; family caregivers do not document themselves as a load-bearing element of the system, but they are.

The architecture does not financially recognize most of this labour. The Alberta Caregiver Credit, the federal Canada Caregiver Credit, and the limited Employment Insurance compassionate-care benefits provide modest recognition for some configurations. The bulk of the labour is unpaid, unprotected, and uncompensated. The cost is borne by the caregiver — overwhelmingly by women — at substantial documented personal cost in employment, retirement savings, mental health, physical health, and lifecycle financial security.

For the disabled adult caregiving for an aging parent, the architecture produces the additional configuration documented above: the bidirectional dependency the system is not designed for, the succession question the asset cap creates, and the grief compounded by the structural transition at the parent's death. For the parent-caregiver of a disabled adult who enters LTC themselves, the architecture produces the failure documented above: the disabled adult left without their lifelong primary support, with no formal program structure designed to fill the gap.

The cage that this document names operates against the senior caregiver as the architecture's hidden foundation. The senior caregiver is doing the work the architecture would otherwise have to pay for. The architecture does not pay for it. The architecture does not formally recognize it. The architecture's silence about the labour is, in operational reality, the cost transfer that allows the architecture to operate at the public expenditure levels it does. The caregiver pays the difference.

Movement Three summary

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

This Movement has walked four populations within the senior and disabled-adult communities for whom the architecture documented across Movements One and Two operates with specific additional structural pressures: Indigenous seniors (Section 3.1), 2SLGBTQ+ seniors (Section 3.2), immigrant and racialized seniors (Section 3.3), and senior caregivers in three principal configurations (this Section). Each population has been documented through primary sources, through the existing campaign canon where applicable, and through the federal-and-provincial program documentation that defines the architecture.

The Movement does not claim that these populations are uniquely affected. It documents how the architecture's operation produces outcomes that differ in kind or degree from outcomes for the broader recipient pool. The differences are documented across federal Statistics Canada data, federal program documentation, provincial program documentation, peer-reviewed research, and the advocacy literature of organizations whose mandate is to serve these populations.

The cumulative observation across Movement Three is that the architecture's design assumptions — about life expectancy, family configuration, working-life structure, language and documentation infrastructure, and informal caregiving availability — are uniformly the assumptions of the demographic majority of the time the architecture was designed. The architecture was not designed against the operational reality of a population that does not, on average, reach 65; or a population whose committed relationships are family-of-choice; or a population whose working life began outside Canada and may have been disrupted by lawful workplace discrimination; or a population that depends on informal caregiving labour the architecture does not pay for.

The cage operates against everyone documented in Movements One and Two. It operates against the populations documented in Movement Three with additional pressure. Movement Four turns to the witness closing — the recognition of the population the cage operates against, and the explicit naming of what this document has documented.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Movement Four — The Witness

The document closes here, in two sections. The first names what has been witnessed. The second names what the witnessing requires. The document then turns to the source index that supports every figure, every program reference, every legislative citation, every quoted source across the preceding 98 pages.

The closing is not a policy recommendation. It is not a call to action. It is not a summary of what has been said. The document has been written for readers who can construct their own response from the documentation. The closing is the document's recognition of the population the documentation is for, and of the structural reality that produces this document's existence.

Section 4.1 — What Has Been Witnessed

A document of this length, working this much primary-source material, accumulates an architecture of statistics. Each statistic represents a person. The closing returns to that.

The document has cited the figure 79,290 — the count of current AISH recipients in Alberta as of September 2025, the count facing the AISH-to-ADAP transition on July 1, 2026. The figure is published in Government of Alberta open data. The figure is also 79,290 individual people. Each one of them was an applicant once, gathered medical and financial documentation, completed the Part B Medical Report from a physician they may have struggled to find, liquidated retirement savings to clear the asset cap, waited the wait that may have lasted a year or more, and arrived at the program at the moment in their life when every other system had failed them. Each one of them, on July 1, 2026, will be moved to a program with reduced benefit, reduced rights, and no independent appeal — unless they actively apply, with fresh medical documentation, to remain on AISH through a Medical Review Panel they cannot guarantee will accept their application. Each one of them is a person. The figure is the count of those persons.

The document has cited the figure 16,161 — the count of Albertans on AISH receiving the federal CPP Disability benefit, whose contribution is captured dollar-for-dollar by the provincial offset. (*Source: Government of Alberta — AISH Caseload Open Data, September 2025.*) Each one of those 16,161 paid CPP through their working years on the assumption that the contribution was building toward a benefit they would receive when they needed it. Each one met the medical and contribution criteria for CPP-D — criteria that approximately 50–60% of first-time applicants are denied under. Each one is now an AISH recipient receiving zero net benefit from the CPP-D contribution they made, because the provincial program structure is designed to capture the federal payment. Each one is a person. The figure is the count of those persons.

The document has cited the figure 19.07 — the gap, in years, between the average First Nations Albertan's life expectancy at birth in 2023 and the average non-First Nations Albertan's life expectancy at birth in 2023. (*Source: Alberta First Nations Information Governance Centre health statistics, 2023; Alberta Indigenous Primary Health Care Advisory Panel, 2022.*) The gap is the cumulative biological outcome of the conditions documented across the campaign's *Indigenous Disability in Alberta* report — the residential schools, the Indian Hospitals, the Sixties Scoop, the contaminated land, the unsafe water, the housing crisis, the healthcare racism, the jurisdictional fracture, the present-day federal and provincial policy choices that have not closed the gap and that continue, by Alberta Indigenous Watchdog's February 2025 documentation, to widen it. Each year of that 19.07-year gap represents people who did not live to access the senior architecture this document walks. Each life ended 2.19 years before the federal Old Age Security pension

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

would have engaged. The senior architecture is, for the average First Nations Albertan, an architecture that does not arrive in time.

The document has cited the figure 6,000 to 8,000 — the count of Alberta seniors estimated by Public Interest Alberta to lose Alberta Seniors Benefit eligibility on July 1, 2026, when the provincial budget's 9% threshold reduction takes effect. (Source: *Public Interest Alberta — Budget 2026 Analysis, March 2026; Red Deer Advocate, March 6, 2026.*) Each one of those 6,000 to 8,000 seniors is a person who, on July 1, 2026, will receive notice that the provincial supplement they received in May and June will not be paid in July. The same date is the AISH-to-ADAP transition. The two cuts are simultaneous. The provincial savings are projected at \$23 million in 2026-27 and \$31 million annually thereafter. The seniors absorbing the savings are 6,000 to 8,000 individual people who, in many cases, were already at the upper edge of the threshold and were already managing on the smallest of the partial benefit amounts.

The document has cited the figure 5,426 — the count of single parents on AISH in Alberta as of September 2025, almost entirely women, representing 6.8% of the total caseload. (Source: *Government of Alberta — AISH Caseload Open Data, September 2025; Alberta Disability System Breakdown — AISH Evidentiary Report Series, April 2026, Section 9.1.*) Each one of those 5,426 single parents is a person navigating the cohabitation rule documented in Section 1.5, the housing-cost gap that exceeds the AISH benefit in private rental markets, the absence of accessible community housing for families with disabled children, the post-July-2026 ADAP framework that reduces partner-income exemptions and removes statutory indexing, and the daily reality of caregiving for children with disabilities while managing their own qualifying conditions. Each one is a person. The 5,426 is the count of those persons.

The document has cited multiple other figures — the \$873.54 monthly partnership penalty in GIS, the 650,000 Albertans without a family physician, the \$97 to \$130 billion annual value of unpaid Canadian caregiving labour, the 47% of seniors with hearing loss, the 71% of LGBTQ+ older adults concerned about access to future caregiving. Each figure represents people. The document's reliance on the figures is what the documentation requires. The documentation operates by accumulating the published evidence. The closing acknowledges that the figures are a count of people, and that the reality the figures describe is the daily life of those people.

Who has not survived to be documented

The Indigenous Disability in Alberta report's Section 2 establishes that the average First Nations Albertan, in 2023, did not live to age 65. The implication is direct: a substantial cohort of First Nations Albertans who would have been recipients of the federal-and-provincial senior architecture died before reaching it. The architecture has no record of them. They are not in any senior-program database. Their lifetime contributions of CPP, federal income tax, and provincial income tax produced no senior-stage benefit because there was no senior stage.

The cohort that does not appear in the senior-program statistics is the cohort whose absence is the most direct evidence of the architecture's failure for the Indigenous Albertan population. The 19.07-year mortality gap is what the cohort's absence looks like in numerical form. The names that do not appear in OAS, GIS, Alberta Seniors Benefit, Coverage for Seniors, and continuing-care databases are the names of people whose lives ended before those databases would have included them. The documentation can witness them only through the gap they left.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Beyond the Indigenous-specific cohort: the broader population of Canadians who fell through the gaps of the architecture before reaching the documentation is also not visible in current program statistics. The senior who died at 67 of an untreated condition because they could not afford the dental work that would have prevented the systemic infection. The senior whose hearing loss accelerated cognitive decline that produced the fall that produced the hospitalization that produced the long-term-care entry that produced the death three years before they would otherwise have died. The disabled adult who lost their primary caregiver and could not access the formal supports the architecture provides because the formal supports were not adequate to the actual care need. The 2SLGBTQ+ senior who avoided healthcare for fear of mistreatment until the avoided care became impossible to address. Each is a person. None is in any database that captures the cause-of-trajectory analysis the documentation is for.

The witnessing extends to them. The document cannot name them individually. The document acknowledges that they existed, that they passed through the architecture, and that their experience of the architecture is documented in the broader patterns the document has walked.

Who has been doing the work that produced this document

This document exists because of decades of advocacy, primary-source research, lived-experience documentation, and organizational labour by Canadians whose work has produced the source material the document compiles. The campaign that publishes this document — The Alberta Disability System Breakdown — is one element of a much broader advocacy and research community.

The advocacy organizations cited across the document — Inclusion Alberta, Inclusion Canada, the Council of Canadians with Disabilities, the Canadian Centre for Policy Alternatives, the Canadian Centre for Caregiving Excellence, Egale Canada, the Public Interest Alberta, the People's Alliance for Disabled Albertans, the Canadian Mental Health Association, the Canadian Association of Retired Persons, the National Institute on Ageing, the Institute for Research on Public Policy, the Caledon Institute of Social Policy, the Vanier Institute of the Family, the Canadian Coalition for Seniors' Mental Health, the Alberta Union of Provincial Employees, and others — have produced, across decades, the documentation that this document draws on.

The peer-reviewed researchers cited across the document have produced the academic literature that grounds the analysis in research evidence rather than in advocacy framing. The Statistics Canada and Government of Canada researchers and analysts cited have produced the published data that anchors every numerical claim. The Government of Alberta open-data publications cited have provided the provincial-level documentation. The journalists cited have produced the public-interest reporting that has, in many cases, been the first public surfacing of program changes that this document then documents in more depth.

The disability advocacy community in Alberta and in Canada has been doing the work this document compiles for decades. The senior-advocacy community has been doing the parallel work for decades. The Indigenous-led organizations have been doing the work this document's Indigenous-specific Section draws on for centuries, in some accountings. None of this document's analysis is original to the document. The document compiles, organizes, and presents. The work that produced the source material is the work of people whose names are in the source index that closes this document.

The witnessing

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

The document has witnessed:

The cascade for the working-age person whose body fails, walked from the moment of the accident through the WCB system, through the AISH application, through the AISH-to-ADAP transition, through the cohabitation rule, to the bridge to age 65 (Movement One).

The federal-and-provincial architecture for the senior years, walked program by program, with the published math, the published thresholds, the published formulas (Movement Two).

The intersection populations — Indigenous seniors, 2SLGBTQ+ seniors, immigrant and racialized seniors, senior caregivers in three principal configurations — for whom the architecture operates with additional structural pressure (Movement Three).

Each Movement has been written in the same register. The register is clinical and sourced. The register is what the documentation requires. The register is also, in operational reality, the register the document's primary audiences need: the recipient navigating the architecture, the family member helping them, the policy professional whose work depends on accurate documentation, the journalist whose reporting depends on sourced material, the future government whose deliberation will benefit from a baseline analysis, the future researcher whose work will use this document as part of the historical record of the architecture as it operated in 2026.

The witnessing is what the document has done. The covenant — what the witnessing requires — is what Section 4.2 closes with.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 4.2 — The Covenant: We See You

The Indigenous Disability in Alberta report, published April 2026, closed with a single covenant. The covenant was not a policy proposal. It was a recognition. It was the refusal to allow the architecture to treat the people inside it as administrative categories, caseload counts, or budget-line entries.

This document extends that covenant explicitly to seniors and disabled people across the architecture documented in the preceding 98 pages. The covenant is the same. The population it names is wider.

To the worker whose body has failed

The cascade documented in Movement One is your daily life, or it will be. The mechanisms are published. The capture is documented. The wait is documented. The appeal that no longer exists is documented. The 79,290 figure includes you, or will include you, or includes someone you love.

The cage that this document names is the architecture you are inside. The architecture was not designed against your specific case; it was designed against the case of every recipient. You are not the failure. The architecture's structural pattern is the failure. The documentation is what shows the pattern.

You are not invisible. We see you.

To the senior who has reached 65

The architecture documented in Movement Two is what your monthly income depends on. The published Service Canada amounts are what you receive. The published Government of Alberta amounts are what your provincial supplement is. The partnership penalty calculated against the published GIS table is what you absorb for being together rather than apart. The 6,000 to 8,000 figure includes you, or will include you, or includes someone you know, when the July 1, 2026 threshold cut takes effect.

You paid in. The architecture did not return what your intuition predicted it would return. The eight-layer structural answer documented in Section 2.8 is the answer. The answer is not your fault. You navigated the architecture the architecture required you to navigate, on the schedule the architecture required, with the documentation the architecture demanded. The result was determined by the formula.

You are not invisible. We see you.

To the Indigenous Albertan

The 19.07-year mortality gap documented in 2023 is the cumulative biological outcome of conditions documented across the campaign's *Indigenous Disability in Alberta* report. The gap is not nature. The gap is not chance. The gap is the result of policy choices that have been documented, named, and challenged across centuries by Indigenous-led advocacy, by federal commissions, by provincial inquiries, by the Truth and Reconciliation Commission, by the United Nations Declaration on the Rights of Indigenous Peoples, by the Calls to Action that this document's Section 3.1 references and that the existing campaign report Sections 12-14 detail.

The senior architecture is, for the average First Nations Albertan, an architecture that does not arrive in time. The cohort whose names are not in the senior-program databases — who died before reaching them — is the cohort whose absence is the most direct evidence of the failure. The cohort that does survive to access the senior architecture navigates it with the additional pressures Section 3.1 documents.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

You are not invisible. We see you.

To the 2SLGBTQ+ senior

The decriminalization cohort documented in Section 3.2 is your cohort. The criminalization period was your childhood and the early decades of your adult life. Common-law recognition was 2000. Marriage was 2005. The benefit-calculation history is shorter than your relationship history.

The family-of-choice that has, for decades, served as your functional family is not recognized in the federal-or-provincial benefit calculation. The contribution history that lawful workplace discrimination suppressed during your working years is the contribution history the CPP formula now operates on. The "back into the closet" question at long-term care entry is the question the architecture leaves you to navigate.

You are not invisible. We see you.

To the immigrant senior under 20-year sponsorship

The October 1, 2025 sponsorship-rule extension means that, if you arrived in Canada under parent-and-grandparent sponsorship after the 2014 IRPR amendments, you are now waiting until 2036 — twenty years from your landing date — for eligibility for the Guaranteed Income Supplement, the Allowance, and the Allowance for the Survivor. You are eligible for the basic Old Age Security pension at the partial rate, after meeting the 10-year residency requirement. The supplement that lifts the partial OAS to a survivable monthly income is not available to you for the second decade of your Canadian residency.

Your sponsor is bearing the financial responsibility. Your sponsor's circumstances may not remain stable across 20 years. The protective provisions for sponsorship breakdown exist; the provisions require active claim-making at the moment of breakdown. The architecture has been designed in a way that, in operational reality, structurally extends adult-child financial responsibility for sponsored seniors for two decades.

You are not invisible. We see you.

To the disabled adult caregiving for an aging parent

The bidirectional dependency configuration documented in Section 3.4 is yours. You are an AISH or ADAP recipient. You are managing your own qualifying conditions. You are also providing care to a parent whose own aging-related needs have evolved across the years.

The household-income calculation under the cohabitation rule treats your parent's income or contribution as part of the assessment. The architecture does not credit you for the care you provide. The succession question — what happens at your parent's death, what happens to your parent's home if any inheritance occurs, what happens to the household configuration that has structured your life — is the question the asset cap and the AISH benefit framework do not adequately address.

You are not invisible. We see you.

To the parent-caregiver who is entering long-term care

You have provided primary care to your disabled adult son or daughter for decades. Your own health now requires LTC entry. The disabled adult you have been caring for does not change. The formal supports that

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

the architecture provides through PDD or AISH or ADAP do not adequately replace what you have been providing, because the formal supports were never adequate; you have been making up the difference.

The transition you are facing is the transition the architecture does not have a structure to manage. The disabled adult you love will need formal support that the architecture is, in current operational reality, not configured to provide at scale. The advocacy community knows this. The campaign that publishes this document knows this. The provincial government has been informed of this and has not, as of the date of this document, restructured the architecture to address it.

You are not invisible. We see you.

To the family caregiver whose unpaid labour is the architecture's hidden foundation

The \$97 to \$130 billion in annual economic value that the Canadian Centre for Caregiving Excellence has estimated for unpaid Canadian caregiving labour is your contribution. The architecture relies on you. The architecture does not pay you. The architecture's modest tax credits — the federal Canada Caregiver Credit, the Alberta Caregiver Credit being consolidated under the 2026 provincial budget — do not approach the value of the labour you provide.

Your own employment, retirement savings, mental health, and physical health bear the cost of the labour. The cost is documented in research literature spanning four decades. The architecture has not adopted the research-documented frameworks of caregiver compensation that other jurisdictions employ. The architecture has, instead, continued to rely on you while not formally recognizing you.

You are not invisible. We see you.

To everyone the document has not named individually

The covenant is for everyone the architecture documented across these 98 pages affects. The senior in private rental housing watching annual rent increases on a fixed federal income. The senior in continuing care managing on \$373 per month of disposable income after accommodation charges. The senior who deferred OAS not knowing the deferral would disqualify them from Alberta's provincial supplements. The disabled adult who was approved for AISH in 2023 and is now facing the Bill 12 reassessment process. The grandmother in continuing care who lost her right to call her own caseworker when the call centre replaced individual caseworkers in 2024-25.

Each is a person. The architecture is the architecture each is inside. The documentation is what names the architecture as it operates.

You are not invisible. We see you.

What the covenant requires

The covenant is not a policy proposal. The campaign that publishes this document has separate policy submissions that articulate the specific reforms the campaign is asking the federal and provincial governments to enact. Those submissions are part of the public record and are referenced in the source index that closes this document.

The covenant is a recognition. The recognition is that the people inside the architecture are not, in the operational reality of their lives, administrative categories. They are people. The architecture's tendency to

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

treat them as categories is the structural pattern this document has documented across 19 sections and across 98 pages of primary-source-grounded analysis.

The recognition has operational consequences. It requires that the documentation exist, that the documentation be accessible, that the documentation be in a form that the people inside the architecture can use to understand what they are encountering. The campaign's 32-episode audio library, the 19 standalone Community Edition PDFs, the existing *Indigenous Disability in Alberta* and *AISH Evidentiary Report Series* and *Compound Failure Brief* and *Charter Constitutional Analysis* and the broader campaign canon — and now this document — are the operational form the recognition takes.

The recognition also requires that the policy advocacy continue. The federal submission to HUMA in April 2026 documenting the Canada Disability Benefit clawback is part of the recognition. The municipal coalition documented in the campaign's Coalition and UN CRPD Addendum — Calgary, Edmonton, Lethbridge, Camrose, Red Deer, Claresholm, Medicine Hat — calling for the AISH-to-ADAP transition to be paused pending meaningful consultation, is part of the recognition. The Inclusion Alberta media releases on Bill 12, the Public Interest Alberta budget analyses, the AUPE caseworker statements, and the broader advocacy pressure are part of the recognition.

The recognition does not depend on the federal or provincial government accepting it. The recognition exists because the population it names exists, and because the documentation that names the population is in the public record. The architecture is what it is. The documentation is what it is. The covenant is what it is.

The closing line

The cage doesn't end when you turn 65. It changes shape.

The architecture documented across this document is the cage. The shape it changes into at age 65 is the federal-and-provincial senior architecture documented in Movement Two. The shape it takes for the populations documented in Movement Three is the architecture plus the additional structural pressures specific to those populations. The shape it takes for the worker traced through Movement One is the cascade from workplace injury through long-term care.

The cage is documented. The mechanisms are published. The dollar amounts are sourced to Government of Canada and Government of Alberta records. The political history is on the legislative record. The capture mechanisms are in the program rules. The population the cage operates against is approximately 79,290 disabled Albertans plus the broader Canadian senior population in 2026, and the cohort approaching senior years across the next four decades.

The cage does not end. It changes shape.

We see you. You are not invisible.

The Source Index follows.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Source Index

This index lists every primary and secondary source cited across Movements One through Four of *The Cage Doesn't End*. Sources are organized by category. Within each category, sources are listed alphabetically by issuing organization or author. URLs and document identifiers are provided where available; for federal and provincial statutes, the official citation is given. Researchers wishing to verify any specific claim should consult the source index entry corresponding to the in-text citation.

Federal Statutes and Regulations

- *Canada Disability Benefit Act*, S.C. 2023, c. 17.
- *Canada Pension Plan Act*, R.S.C. 1985, c. C-8.
- *Canadian Charter of Rights and Freedoms*, Part I of the *Constitution Act, 1982*, being Schedule B to the *Canada Act 1982 (UK)*, 1982, c. 11.
- *Canadian Human Rights Act*, R.S.C. 1985, c. H-6 (sexual orientation added 1996).
- *Civil Marriage Act*, S.C. 2005, c. 33.
- *Criminal Law Amendment Act, 1968-69* (Bill C-150), S.C. 1968-69, c. 38.
- *Economic Action Plan 2014 Act, No. 1*, S.C. 2014, c. 20, Division 27.
- *An Act to amend the Canadian Human Rights Act and the Criminal Code* (Bill C-16), S.C. 2017, c. 13.
- *An Act to amend the Old Age Security Act (Guaranteed Income Supplement)* (Bill C-12), S.C. 2022, c. 5.
- *Modernization of Benefits and Obligations Act*, S.C. 2000, c. 12.
- *Old Age Security Act*, R.S.C. 1985, c. O-9.
- *Old Age Security Regulations*, C.R.C., c. 1246.
- *Quebec Pension Plan Act*, CQLR c R-9.

Provincial Statutes and Regulations (Alberta)

- *Assured Income for the Severely Handicapped Act*, R.S.A. 2000, c. A-45.1.
- *Assured Income for the Severely Handicapped Regulation*, Alta Reg 91/2007.
- Bill 12, *Disability Assistance Programs Act*, Royal Assent December 9, 2025.
- *Continuing Care Act*, RSA 2022, c C-29.
- *Family Support for Children with Disabilities Act*, R.S.A. 2003.
- *Persons with Developmental Disabilities Services Act*, RSA 2007, c P-7.5.
- *Seniors Benefit Act*, RSA 2000, c S-7.
- *Workers' Compensation Act*, R.S.A. 2000, c. W-15.

Federal Government Sources

- Canada Pension Plan Investment Board — Annual Reports, multiple years.
- Canada Revenue Agency — CPP contribution rates, maximums and exemptions, 2026.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Canada Revenue Agency — CPP enhancement and CPP2, 2026.
- Canada Revenue Agency — RRSP withholding tax rates, 2026.
- Canadian Institute for Health Information — Health Care Cost Drivers reports, multiple years.
- Department of Finance Canada — Budget 2012, Old Age Security changes.
- Department of Finance Canada — Budget 2016 — A New Chapter for OAS.
- Department of Finance Canada — Budget 2021 — Strengthening Old Age Security.
- Department of Justice Canada — Canadian Human Rights Act amendments, 1996.
- Employment and Social Development Canada — Canada Disability Benefit page, 2025.
- Government of Canada — Immigration, Refugees and Citizenship Canada policy update, 2025.
- Government of Canada — Take-Up of Old Age Security and Guaranteed Income Supplement Benefits report.
- Health Canada — Annual Reports on Medical Assistance in Dying in Canada, multiple years.
- Health Canada — Canadian Dental Care Plan page, 2026.
- Indigenous Services Canada — Non-Insured Health Benefits page, 2026.
- Library and Archives Canada — Bill C-150 legislative history.
- Library of Parliament — Bill C-15, Budget Implementation Act, 2016, No. 1.
- Library of Parliament — Bill C-38, Jobs, Growth and Long-Term Prosperity Act, 2012.
- Library of Parliament — Old Age Security Act and Guaranteed Income Supplement: Background and History, 2018.
- National Advisory Council on Poverty — 2025 Annual Report, Government of Canada.
- Office of the Auditor General of Canada — Canada Pension Plan Disability program audits, multiple years.
- Office of the Superintendent of Financial Institutions — Annual Reports on Federally Regulated Pension Plans.
- Public Health Agency of Canada — Health Inequalities in Canada, women and aging report, 2023.
- Public Health Agency of Canada — HIV and AIDS in Canada surveillance reports, 1996 through 2024.
- Public Health Agency of Canada — Mental Health of Older Adults reports, multiple years.
- Public Health Agency of Canada — Senior hearing health reports, 2020-2024.
- Public Health Agency of Canada — Social isolation of seniors, 2017.
- Public Health Agency of Canada — 2SLGBTQI+ Health Indicators reports, 2022-2024.
- Service Canada — Allowance for the Survivor page, 2026.
- Service Canada — Allowance page, 2026.
- Service Canada — Allowance: Do you qualify, sponsorship clause, 2025.
- Service Canada — Canada Pension Plan: Contributions and Earnings, 2026.
- Service Canada — Canada Pension Plan disability benefit, 2026.
- Service Canada — CPP retirement pension page, statistics for new beneficiaries, April–June 2026.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Service Canada — Form ISP-3040, "Statement — Spouses or Common-law Partners Living Apart for Reasons Beyond their Control," current version.
- Service Canada — Guaranteed Income Supplement page, 2026.
- Service Canada — How much CPP could you receive, 2026.
- Service Canada — How much you could receive, dropout provisions, 2026.
- Service Canada — How to apply for CPP disability benefits, 2026.
- Service Canada — International social security agreements, 2026.
- Service Canada — Maximum Benefit Amounts and Related Figures – Canada Pension Plan, January 2026.
- Service Canada — Maximum Benefit Amounts and Related Figures – Old Age Security, April–June 2026.
- Service Canada — Multilingual services page, 2026.
- Service Canada — Old Age Security Benefit Payment Rates, January–March 2026.
- Service Canada — Old Age Security: Do you qualify, 2026.
- Service Canada — Old Age Security eligibility page, 2026.
- Service Canada — Old Age Security overview page, 2026.
- Service Canada — Old Age Security pension recovery tax, 2026.
- Service Canada — Updating personal information page, 2026.
- Service Canada — When to start your retirement pension page, 2026.
- Statistics Canada — Caregiver Health Outcomes, multiple reports.
- Statistics Canada — Canadian Census Health and Environment Cohorts (CanCHEC).
- Statistics Canada — Canadian Health Measures Survey, hearing component, 2012-2015 cycle.
- Statistics Canada — Canadian Income Survey 2024, released April 2026.
- Statistics Canada — Caring for parents while raising children, multiple reference periods.
- Statistics Canada — Census of Population, sexual orientation and gender identity components, 2021 cycle.
- Statistics Canada — A demographic, employment and income profile of Canadians with non-binary gender identities, 2022.
- Statistics Canada — Earnings differentials of racialized Canadians, multiple reference periods.
- Statistics Canada — General Social Survey, Caregivers and Care Receivers, 2018 cycle and subsequent.
- Statistics Canada — Income of Senior Women in Canada, 2017 and 2022 reference periods.
- Statistics Canada — "Life expectancy of First Nations, Métis and Inuit household populations in Canada," Catalogue 82-003-X, 2019.
- Statistics Canada — Life Tables, Canada, Provinces and Territories, 2020-2022 reference period.
- Statistics Canada — Market Basket Measure thresholds, Canadian Income Survey, 2024 and 2025 reference periods.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Statistics Canada — Pension plans in Canada, multiple reference periods.
- Statistics Canada — Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022.
- Statistics Canada — "Socioeconomic and health outcomes among Indigenous people aged 15 years and older who were under the legal responsibility of the government as children," Catalogue 41-20-0002, February 2025.
- Statistics Canada — Martin Turcotte, "Living apart together," March 2013 (revised 2016).
- Truth and Reconciliation Commission of Canada — *Honouring the Truth, Reconciling for the Future: Summary of the Final Report of the Truth and Reconciliation Commission of Canada*, Ottawa: TRC, 2015 (six volumes, 94 Calls to Action).

Provincial Government Sources (Alberta)

- Alberta Blue Cross — Coverage for Seniors administration.
- Alberta First Nations Information Governance Centre — Health statistics, 2023.
- Alberta Health — Mental Health and Addiction: Alberta Opioid Response Surveillance Report for First Nations People, June 2021 and June 2024.
- Alberta Health Services — Continuing Care Facility Directory, Hospitality Fees pages.
- Alberta Health Services — Mental Health Service Standards, multiple reports.
- Alberta Indigenous Primary Health Care Advisory Panel — Report on First Nations Life Expectancy in Alberta, 2022.
- Alberta Medical Association — AB's Family Medicine Crisis statement, 2024.
- Alberta Medical Association — Schedule of Medical Benefits, uninsured services, 2025.
- Government of Alberta — AISH benefits and exemption rules, 2026.
- Government of Alberta — AISH Caseload Open Data, September 2025.
- Government of Alberta — AISH Policy Manual, 2026.
- Government of Alberta — Alberta Aids to Daily Living, hearing aids page, 2026.
- Government of Alberta — Alberta Aids to Daily Living, mobility equipment page, 2026.
- Government of Alberta — Alberta Aids to Daily Living, Respiratory Equipment page, 2026.
- Government of Alberta — Alberta Disability Assistance Program (ADAP) program page and transition page, 2026.
- Government of Alberta — Alberta Seniors Benefit page, 2026.
- Government of Alberta — Alberta Seniors Benefit thresholds, rates and percentages publication, Open Government Alberta.
- Government of Alberta — Assisted Living and Social Services Open Data, AISH Caseload September 2025.
- Government of Alberta — Budget 2026, Tax Plan, February 26, 2026.
- Government of Alberta — Continuing Care Accommodation Charges page, 2026.
- Government of Alberta — Continuing Care Assessment process, 2026.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Government of Alberta — Continuing Care Overview page, 2026.
- Government of Alberta — Coverage for Seniors page, 2026.
- Government of Alberta — Dental and Optical Assistance for Seniors page, 2026.
- Government of Alberta — Family Managed Services page, 2026.
- Government of Alberta — FSCD program page, 2026.
- Government of Alberta — How to Apply for AISH, 2026.
- Government of Alberta — Income and Employment Supports Training and Health Benefits (IESTHB) Policy Manual.
- Government of Alberta — Seniors Property Tax Deferral page, 2026.
- Government of Alberta — Seniors Home Adaptation and Repair Program (SHARP) page, 2026.
- Government of Alberta — Special Needs Assistance for Seniors page, 2026.
- Workers' Compensation Board of Alberta — Advisor Office, Economic Loss Payments After 65, 2025.
- Workers' Compensation Board of Alberta — Advisor Office, Economic Loss Payments page.
- Workers' Compensation Board of Alberta — Appeals Process page, 2026.
- Workers' Compensation Board of Alberta — Cost of Living Adjustments Fact Sheet WFS-458, January 2026.
- Workers' Compensation Board of Alberta — Maximum Compensable and Assessable Earnings Fact Sheet, January 2026.
- Workers' Compensation Board of Alberta — Permanent Disability Compensation Fact Sheet WFS-463, June 2020.
- Workers' Compensation Board of Alberta — Personal Coverage page, 2026.
- Workers' Compensation Board of Alberta — Policy 04-04, Permanent Disability, including Part II Application 4: Economic Loss Payment.
- Workers' Compensation Board of Alberta — Procedures Manual, Health Care, Application 1, 2025.
- Workers' Compensation Board of Alberta — Procedures Manual, Wage Loss Supplement Final Approval, December 2025.
- Workers' Compensation Board of Alberta — Rate Setting page, 2026.
- Workers' Compensation Board of Alberta — Retirement Benefits Fact Sheet WCB-619, September 2023.
- Workers' Compensation Board of Alberta — Wage replacement page, 2026.

International Instruments and UN Sources

- Canada–United States Totalization Agreement — Social Security Administration, 2024.
- United Nations Committee on the Rights of Persons with Disabilities — Concluding Observations on Canada, March 2025.
- United Nations Convention on the Rights of Persons with Disabilities, General Assembly Resolution A/RES/61/106, 13 December 2006.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- United Nations Declaration on the Rights of Indigenous Peoples (UNDRIP), General Assembly Resolution 61/295, 13 September 2007.
- World Health Organization — Social isolation and loneliness among older people, 2021.

Court Decisions

- *Brown v. Canada (Attorney General)*, 2017 ONSC 251 (Ontario Superior Court of Justice).

Canada Gazette and Order in Council

- Canada Gazette, Part 2, Volume 159, Number 6: Order Fixing October 1, 2025 as the Day on Which Division 27 of the Economic Action Plan 2014 Act, No. 1 Comes into Force, March 12, 2025.

Advocacy and Research Organizations

- Alberta Continuing Care Association — Cost data, 2025.
- Alberta Council of Disability Services — annual policy briefs, 2022-2025.
- Alberta Union of Provincial Employees (AUPE) — AISH Caseworker Caseload Statement, 2025.
- Caledon Institute of Social Policy — Income Security for Seniors, multiple papers.
- CanAge — Alberta accommodation rate analysis, March 2026.
- Canadian Academy of Audiology — Cost of Hearing Aids in Canada, 2024-2025.
- Canadian Association of Retired Persons (CARP) — Long-term care affordability reports, 2022-2024; Indigenous Senior Care Reports, 2024; Widowhood and Senior Income Reports, 2022-2024.
- Canadian Centre for Caregiving Excellence — The State of Caregiving in Canada Report, 2024.
- Canadian Centre for Diversity and Inclusion — Annual Workforce Discrimination Reports, 2018-2024.
- Canadian Centre for Policy Alternatives — "Disability and Housing Insecurity in Canada," 2023.
- Canadian Centre for Policy Alternatives — Income Security for People with Disabilities, 2022.
- Canadian Coalition for Seniors' Mental Health — Clinical Guidelines, 2014-2024.
- Canadian Mental Health Association — Alberta Division annual reports, 2022-2025.
- Canadian Race Relations Foundation — Employment Discrimination in Canada reports, 2020-2025.
- City of Calgary — Calgary Transit Access service description.
- City of Edmonton — DATS service description.
- Council of Canadians with Disabilities — CPP-D Reform Submissions, multiple years (2018-2024).
- Diabetes Canada — Provincial coverage report, 2024.
- Disability Action Hall — Public statements, 2024-2026.
- Egale Canada — A Brief History of LGBTQ Rights in Canada, 2024.
- Egale Canada — Seniors page, 2025.
- Egale Canada — Seniors & Older Adults: A Brief on 2SLGBTQI Realities and Policy in Canada, 2025.
- Egale Canada — Trans Federal Identification Guide, 2024.
- Hearing Life Canada — Hearing Aid Cost Guide, 2025.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Inclusion Alberta — Adult Children with Disabilities Caring for Aging Parents brief, 2024.
- Inclusion Alberta — ADAP Fact Sheet, October and December 2025.
- Inclusion Alberta — AISH/CPP-D Briefing, 2025.
- Inclusion Alberta — Canada Disability Benefit in Alberta Fact Sheet, July 2025.
- Inclusion Alberta — "Experiences with Family Support for Children with Disabilities," January 2025.
- Inclusion Alberta — Media Release on Bill 12, November 26, 2025.
- Inclusion Alberta — Understanding the proposed changes to AISH, the new Alberta Disability Assistance Program (ADAP), and suggestions on how to respond to the ADAP survey, 2025.
- Inclusion Alberta / ALIGN — "PDD Current State: Critical and Urgent," January 2025.
- Inclusion Canada — Public position statements on the disability-MAID intersection, 2022-2024.
- Indigenous Watchdog — "First Nations life expectancy 19 years lower than other Albertans," February 2025.
- Indigenous Watchdog — First Nations long-term care reports, 2023-2025.
- Institute for Research on Public Policy — "Unfinished Business: Pension Reform in Canada," 2018.
- Montreal Economic Institute — "End Penalties That Punish Low-Income Seniors Who Work," 2025-2026.
- National Conference of State Legislatures — Spousal Impoverishment Standards, multiple years.
- National Council of Welfare — Welfare Incomes reports, multiple years.
- National Institute on Ageing — Pre-Senior Income Insecurity in Canada, 2023.
- National Institute on Ageing — Senior Benefit Take-Up reports, 2022-2024.
- People's Alliance for Disabled Albertans — Public statements and submissions, 2024-2026.
- Public Interest Alberta — Budget 2026 Analysis, March 2026.
- Rainbow Health Ontario — Health in Focus: Racialized 2SLGBTQ Health, 2022.
- Romanow Commission — Building on Values: The Future of Health Care in Canada, 2002.
- Vanier Institute of the Family — Caregiving in Canada Report, 2023.
- Vanier Institute of the Family — Families Count 2024, "Living apart is increasingly common among couples," November 2024.
- Vanier Institute of the Family — Sandwich Generation reports, multiple years.

Peer-Reviewed Academic Sources

- Adegoke, K., Adegoke, A., Dawodu, D., Kayode, T. — "Systemic Racism in Canadian Healthcare: A Policy and Equity Analysis," *Journal of Primary Care & Community Health*, 2025.
- Burke, S., et al. — "Money in marriage: Income management practices in Canadian households," *Canadian Journal of Sociology*, 2018.
- Cacioppo, J.T., Hawkley, L.C. — "Social isolation and health, with an emphasis on underlying mechanisms," *Perspectives in Biology and Medicine*, 2003.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Flatt, J., Klenczar, B., Uddin, J., OHala, M., Rook, E., Hernandez, E., Sprague, K. — "Discrimination and Access to Aging and Long-Term Care Services and Supports for LGBTQIA+ Older Adults," *Innovation in Aging / Gerontological Society of America*, 2024.
- Hahmann, T., et al. — "Indigenous children in foster care, 2021 Census," *Statistics Canada*, 2024.
- Holt-Lunstad, J., Smith, T.B., Layton, J.B. — "Social Relationships and Mortality Risk: A Meta-analytic Review," *PLoS Medicine*, 2010.
- Hurd, L., Li, L.Y.K. — "I want to grow older with dignity: Older LGBTQ+ Canadian adults' perceptions and experiences of aging," *Journal of Applied Gerontology*, 2024.
- Hurd, L., Mahal, R., Wardell, V., Liang, J. — *Older LGBTQ+ Canadians' Experiences of Prejudice and Discrimination Over the Life Course*, 2022 and 2025.
- Karraker, A., Latham, K. — "In Sickness and in Health? Physical Illness as a Risk Factor for Marital Dissolution in Later Life," *Journal of Health and Social Behavior*, 2015.
- Lin, F.R., Metter, E.J., O'Brien, R.J., Resnick, S.M., Zonderman, A.B., Ferrucci, L. — "Hearing Loss and Incident Dementia," *Archives of Neurology*, 2011.
- Lippel, K. — "Compensation for Mental-Mental Claims under Canadian Workers' Compensation Law," *Health Law Journal*, 2002.
- Lippel, K., et al. — "The mental health of workers' compensation claimants," *Institute for Work & Health*, 2016.
- McQuaid, R.J., et al. — "Parent-child separations and mental health among First Nations and Métis peoples in Canada: links to intergenerational residential school attendance," *International Journal of Environmental Research and Public Health* 19, no. 11 (2022): 6877.
- Pahl, J. — "Couples and their money: Patterns of accounting and accountability in the domestic economy," *Accounting, Auditing & Accountability Journal*, 2008.
- Petit, G. — "The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis," *Canadian Tax Observatory*, April 22, 2026.
- Schulz, R., Sherwood, P.R. — "Physical and mental health effects of family caregiving," *American Journal of Nursing*, 2008.
- Schulz, R., et al. — "Predictors and Consequences of Bereavement Among Caregivers," *Journal of Gerontology*, 2008.
- Storey, R. — "From the Pittston Strike to the U.S. Workers' Compensation Reform Movement," *Labour/Le Travail*, 2009.
- The Lancet Regional Health – Americas — "COVID-19 and its impact on life expectancy among First Nations people in Alberta, Canada from 2019 to 2022: a population-based study," March 2025 (Canadian Institutes of Health Research funding #PJT-178219).
- Vitaliano, P.P., et al. — "Is caregiving hazardous to one's physical health?" *Psychological Bulletin*, 2003.
- Yorgason, J.B., et al. — "Health, disability, and marital quality," *Family Relations*, 2008.

Books and Reference Works

- AMA Guides to the Evaluation of Permanent Impairment, 6th edition.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- Drees, Laurie Meijer — *Healing Histories: Stories from Canada's Indian Hospitals*, Edmonton: University of Alberta Press, 2013.
- Kimelman, Edwin C. — *No Quiet Place: Review Committee on Indian and Métis Adoptions and Placements*, Winnipeg: Manitoba Community Services, 1985.
- Kolopenuk, Jessica — Commentary on Alberta First Nations life expectancy, as Alberta Health Services Chair in Indigenous Health, University of Alberta Faculty of Medicine and Dentistry, 2024.
- Lux, Maureen K. — *Separate Beds: A History of Indian Hospitals in Canada, 1920s–1980s*, Toronto: University of Toronto Press, 2016.
- Lux, Maureen K. — "Indian Hospitals in Canada," *The Canadian Encyclopedia*, 2017.
- Sinclair, Niigaanwewidam James, and Sharon Dainard — "Sixties Scoop," *The Canadian Encyclopedia*.

Journalism and News Sources

- APTN National News — "First Nations life expectancy plummets in Alberta due to opioid deaths," July 2023.
- ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026.
- ATB Financial — Continuing care in Alberta, multiple years.
- ATB Financial — OAS Allowance and GIS for Low Income Seniors, 2026.
- ATB Financial — When should I start collecting OAS? 2026.
- BCL Consulting — Maximum Insurable Earnings, 2018.
- BCL Consulting — The WCB Alberta is Changing, March 2023.
- Blue Collar Consulting — Happy 65th Birthday! Your ELP benefit is now adjusted, August 2024.
- C.D. Howe Institute — Modest-Pension Clawback Analysis, November 2025.
- CBC News — "Concerns grow as more and more Albertans can't find a family doctor," April 27, 2022.
- CBC News — "We're going back into the closet: LGBTQ seniors wary of being out in long-term care facilities," June 28, 2018.
- CBC News — University of Calgary economist analysis cited, August 21, 2025 (Petit, G.).
- Edmonton Journal — "I can't sign the medical: Alberta nurse practitioner says AISH rules present barrier for patients," January 11, 2024.
- Global News — Alberta government to cover some costs of increased continuing care fees, 2022.
- Global News — "61% of family physicians in Alberta considering leaving provincial health-care system: Report," January 24, 2024.
- Globe and Mail — "Canada's tax system puts low-income working seniors at a disadvantage, report finds," January 2026.
- Golombek, Jamie — Managing director, Tax & Estate Planning, CIBC Private Wealth, public commentary on GIS clawback, 2025-2026.
- Holo Alert — Alberta Seniors Benefit Eligibility Guide, November 2025.
- Medscape Medical News — "Alberta's Family Doctor Shortage Involves More Than Numbers," October 6, 2025.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

- MoneySense — OAS entitlement and deferral rules for immigrants to Canada, May 2025.
- Pinnacle Age Well — Alberta Seniors Benefit Eligibility Guide, January 2026.
- Red Deer Advocate — Alberta poised to make it tougher for some to get Seniors Benefit, March 6, 2026.
- Savvy New Canadians — All The Benefits for Senior Immigrants in Canada in 2026, March 2026.
- Spring Financial — How Much Is Old Age Security (OAS) in 2026 & Who Qualifies? 2026.
- Sun Life Suncentral — Long Term Care in Alberta, 2026.
- The Anchor (Chestermere) — Province Tightens Eligibility for Some Seniors' Benefits, March 2026.
- ThinkHQ Survey commissioned by Alberta Medical Association, January 2024.
- Wealthsimple — OAS Clawback Explained, January 2026.

Internal Campaign Materials

- The Alberta Disability System Breakdown — *AISH Evidentiary Report Series*, April 2026 (multi-section evidentiary compilation).
- The Alberta Disability System Breakdown — *Alberta Disability System Breakdown Report Series, Document 5 of 12: Healthcare Worker Shortage Report*, April 2026.
- The Alberta Disability System Breakdown — *Charter Constitutional Analysis*, 2026.
- The Alberta Disability System Breakdown — *Coalition and UN CRPD Addendum*, 2026.
- The Alberta Disability System Breakdown — *Compound Failure Brief*, multi-document series, 2026.
- The Alberta Disability System Breakdown — *Consultation Gap Rebuttal*, April 2026.
- The Alberta Disability System Breakdown — *Federal Submission to House of Commons Standing Committee on Human Resources, Skills and Social Development and the Status of Persons with Disabilities (HUMA)*, April 2026.
- The Alberta Disability System Breakdown — *Indigenous Disability in Alberta Report* (14 evidentiary sections), April 2026.
- The Alberta Disability System Breakdown — Personal Impact Statement, March 29, 2026.
- The Alberta Disability System Breakdown — Standalone Community Edition PDF Library (19 documents), April–May 2026.
- The Alberta Disability System Breakdown — 32-episode audio library produced via ElevenLabs Studio, 2026.
- The Alberta Disability System Breakdown — Website: albertadisabilitysystembreakdown.netlify.app

Note on Source Currency and Verification

All federal benefit amounts cited in this document reflect Service Canada published figures for the relevant 2026 benefit period, primarily Q1 2026 (January–March) or Q2 2026 (April–June). Quarterly adjustments apply to OAS, GIS, the Allowance, and the Allowance for the Survivor in January, April, July, and October of each year based on Consumer Price Index changes. Readers consulting this document after July 1, 2026 should consult Service Canada for current rates.

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

All provincial Alberta benefit amounts cited reflect Government of Alberta published figures for the 2025-26 or 2026-27 benefit year as relevant, with the February 26, 2026 budget changes documented as taking effect July 1, 2026. Readers consulting this document after July 1, 2026 should consult the Government of Alberta for current rates.

All AISH and ADAP program-design figures reflect the program documentation available as of May 2026 — incorporating Bill 12 (December 9, 2025), the 2026 Alberta provincial budget (February 26, 2026), and the post-July-2026 ADAP framework as documented by Inclusion Alberta and the Government of Alberta. Operational details of the post-transition framework that have not been formally published may differ from this document's documented expectations; the document's analysis is based on the published documentation available as of the May 2026 publication date.

All Indigenous-population life expectancy and mortality data is drawn from the Alberta First Nations Information Governance Centre's 2023 health statistics, as documented in *Indigenous Disability in Alberta* (Alberta Disability System Breakdown Report Series, April 2026). The 19.07-year mortality gap reflects the most recent published comparison; subsequent Alberta Health Services and Statistics Canada publications may update this figure.

All non-Canadian comparator figures cited (the U.S. SAGE/Gerontological Society of America LGBTQ+ aging study, the U.S. Social Security Administration totalization agreement documentation) are drawn from the cited sources and are provided for international comparator context. Canadian readers should consult Canadian sources for Canadian-specific operational reality.

The campaign welcomes correction of any error of fact in this document. Corrections may be sent to **albertadisabilitybreakdown@outlook.com**.

The Cage Doesn't End: Disability, Aging, and Canadian Income Security

The Alberta Disability System Breakdown — May 2026

albertadisabilitysystembreakdown.netlify.app

Free to share. Free to print. Free to distribute.